Reducing alcohol-related harm

SAFER implementation handbook

SAFER
A WORLD FREE FROM ALCOHOL RELATED HARMS

World Health Organization

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Foreword

Alcohol was responsible for an estimated 2.6 million deaths in 2019, and 400 million people were living with alcohol use disorders. Behind these figures lie road traffic injuries, cancers, cardiovascular disease, liver disease and violence. Beyond the toll on health lies social and economic harm to families and communities, including family disruption, child maltreatment and household poverty. The burden falls hardest on those with the fewest resources: for the same level of consumption, people in disadvantaged communities suffer more harm. Alcohol is not only a health issue; it is also an equity issue.

None of this is inevitable. We know what works. The most effective and cost-effective measures are set out in existing WHO recommendations, including the "best buys" for the prevention and control of noncommunicable diseases. The international mandate is equally clear. It rests on the Global strategy to reduce the harmful use of alcohol, the Global alcohol action plan 2022–2030 and target 3.5 of the Sustainable Development Goals. Yet progress has been uneven. What has been missing in many countries is not evidence or a mandate, but practical support for implementation.

The SAFER initiative, launched by WHO and international partners in 2018, was created to close that gap. It concentrates effort on five high-impact interventions: strengthening restrictions on alcohol availability; advancing and enforcing drink–driving countermeasures; facilitating access to screening, brief interventions and treatment; enforcing bans or comprehensive restrictions on alcohol advertising, sponsorship and promotion; and raising alcohol prices through excise taxes and pricing policies. The question this handbook answers is not whether these interventions work; it is how to put them to work. This handbook takes the reader through the full implementation pathway: assessing the national landscape, conducting joint assessments, developing roadmaps, monitoring progress and using economic evidence to secure sustainable financing. It extends that pathway to the settings in which policy meets daily life: municipalities, communities and workplaces. It draws on implementation experience from across WHO regions. It is also candid about the political realities of alcohol policy and provides practical tools to protect public health decisions from commercial interests that can delay or dilute them.

I encourage governments, subnational authorities, civil society and our own WHO offices to use this handbook as intended: by adapting it to national and local contexts and putting its guidance into practice. Every Member State, whatever its starting point, can take the next step, and WHO stands ready to support each one.

I thank the many contributors within and beyond WHO whose experience shaped these pages. The measures described here are among the most effective instruments available to public health. This handbook is an invitation to use them.



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Abbreviations and acronyms

APC	Alcohol per capita consumption
AUD	Alcohol use disorder
AUDIT	Alcohol Use Disorders Identification Test
BAC	Blood alcohol concentration
CHE	Current health expenditure
COI	Conflict of interest
CSO	Civil-society organization
CSR	Corporate social responsibility
DALY	Disability-adjusted life year
DSM-5	Diagnostic and Statistical Manual of Mental Disorders, fifth edition
GDP	Gross domestic product
GISAH	Global Information System on Alcohol and Health (WHO)
I-CAAN	Irish Community Action on Alcohol Network
ICD-11	International Classification of Diseases, 11th revision
ILO	International Labour Organization
MCAP	Multi-City Action Plan on Alcohol
MDA	Ministry, department or agency
M&E	Monitoring and evaluation
mhGAP	Mental Health Gap Action Programme
NCD	Noncommunicable disease
NGO	Nongovernmental organization
OECD/DAC	Organisation for Economic Co-operation and Development, Development Assistance Committee
ROI	Return on investment

SAFER	Strengthen availability restrictions; Advance drink–driving countermeasures; Facilitate access to screening, brief interventions and treatment; Enforce advertising restrictions; Raise alcohol prices
SAOR	Support, Ask & Assess, Offer Assistance, Referral (Ireland's screening-and-brief-intervention model)
SAPRO	Social-aspects/public-relations organization
SBI	Screening and brief intervention
SBIRT	Screening, brief intervention and referral to treatment
SDG	Sustainable Development Goal
SES	Socioeconomic status
UN	United Nations
UNDP	United Nations Development Programme
UNIATF	United Nations Inter-Agency Task Force on the Prevention and Control of NCDs
UNICEF	United Nations Children's Fund
WHO	World Health Organization

Glossary of key terms

The definitions below apply throughout this handbook. Many are drawn from, or adapted from, WHO technical documents, the evaluation glossary of the Organisation for Economic Co-operation and Development's Development Assistance Committee (OECD/DAC) and the WHO *Global alcohol action plan 2022–2030*. Cross-references within the definitions point to the Part of this handbook in which a fuller treatment can be found.

Term	Definition
Alcohol industry	Manufacturers of alcoholic beverages, wholesale distributors, major retailers and importers, together with business associations and non-State actors representing them. The full operational definition used in this handbook is set out in Part 4. See also Economic operators.
Alcohol policy	All aspects of public policies relating to alcohol, health and social well-being, including policy development, adoption, implementation, monitoring and evaluation.
Alcohol policy interference	Alcohol industry efforts – intentional or unintentional – to stop, delay or weaken progress towards public health goals in alcohol policy development and implementation.
Best buys	A WHO-defined subset of cost-effective NCD interventions with cost-effectiveness of ≤ I$ 100 (international dollars) per disability-adjusted life year (DALY) averted in low- and lower-middle-income countries. Several SAFER interventions qualify as best buys.
Community	In this handbook, an umbrella term for any subnational level – provincial, district, municipal or submunicipal – at which SAFER actions can be planned, implemented, monitored and protected. See Part 5, section 5.2 for the full operational typology.
Conflict of interest (COI)	A situation in which the independence or objectivity of professional judgement or action on a primary interest (for example, public health) is unduly influenced, or may reasonably be perceived to be so, by a secondary interest (for example, commercial interest).
Corporate social responsibility (CSR)	Voluntary business activities framed as contributions to society; in the alcohol sector, widely associated with reputational and public-relations strategies that can support policy interference.
Country SAFER Landscape Report	A country-level analytical report that describes the social, economic, political and policy context affecting alcohol policy, used to inform SAFER implementation. See Part 1, Module 1.

Term	Definition
Economic operators	The term used in the *Global strategy to reduce the harmful use of alcohol* and the *Global alcohol action plan 2022–2030* for commercial actors in alcohol production and trade, in their roles as developers, producers, distributors, marketers and sellers of alcoholic beverages. The action plan addresses a statement to economic operators in each of its six action areas; the operative content is a set of refrain clauses describing industry conduct that is incompatible with protected policy-making. It overlaps with, but is not identical to, the alcohol industry (see the entry above), which in this handbook also covers business associations and non-State actors representing these operators. See Part 4, sections 4.3, 4.5.1 and 4.11.
Evaluation	Systematic and objective assessment of a planned, ongoing or completed intervention – including its design, implementation and results – against the criteria of relevance, coherence, effectiveness, efficiency, impact and sustainability.
Harmful use of alcohol	Drinking that causes detrimental health and social consequences for the drinker, people around the drinker and society at large, as well as patterns of drinking associated with increased risk of adverse health outcomes. The phrase is negotiated consensus language in global deliberations: it entered governing-body usage through the process that produced resolution WHA58.26, adopted by the Fifty-eighth World Health Assembly in 2005, and it is defined for policy purposes in the Global strategy to reduce the harmful use of alcohol (2010), from which the definition above is drawn. In this usage, "harmful" refers only to the public health effects of alcohol consumption, without prejudice to religious beliefs and cultural norms. The concept concerns ethyl alcohol consumed as a beverage and does not encompass its non-beverage uses, such as industrial, pharmaceutical, cosmetic or fuel applications.
Impact	Higher-level long-term changes (positive or negative, intended or unintended) resulting from an intervention's outcomes.
Indicator	A quantitative or qualitative variable used to track changes and performance of an intervention or its context.
Inputs	Financial, human, material, institutional, technological and information resources deployed for an intervention.
Joint SAFER Assessment	A jointly led technical country visit, convened by the ministry of health, WHO and SAFER partners, to assess the alcohol policy situation and inform the SAFER Roadmap. See Part 1, Module 2.
Monitoring	Continuous, systematic collection of data on specified indicators to provide stakeholders with information on implementation progress, achievement of results and use of resources.
Outcomes	Short- and medium-term effects of an intervention's outputs.
Outputs	Products, goods and services (including changes in knowledge, skills or abilities) resulting directly from an intervention's activities.
SAFER initiative	The WHO-led initiative launched in 2018 that promotes five high-impact, cost-effective alcohol policy interventions (Strengthen availability; Advance drink–driving countermeasures; Facilitate access to screening, brief interventions and treatment; Enforce advertising restrictions; Raise prices).

Term	Definition
SAFER Roadmap	A country-level strategic implementation plan covering 12–24 months that translates SAFER landscape-report and assessment-report recommendations into prioritized interventions, activities, responsibilities and timelines. See Part 1, Module 3.
Screening and brief intervention (SBI)	A two-component clinical procedure: use of a validated screening instrument, such as AUDIT, to identify hazardous or harmful drinking, followed by a short structured conversation delivered in a non-specialist setting such as primary care. Efficacious for hazardous and harmful drinking; not the indicated response for alcohol dependence. See Part 1, section 1.1.5.5.
Screening, brief intervention and referral to treatment (SBIRT)	Screening and brief intervention together with a specified pathway to assessment and treatment for people whose screening result indicates probable dependence, for whom brief intervention alone is not the indicated response. See Part 1, section 1.1.5.5.
Screening, brief interventions and treatment	The third SAFER intervention area (F). Wider than either SBI or SBIRT: the policy and health-system commitment to facilitate access to screening, to brief interventions and to treatment for alcohol use disorders, the last of these as a service in its own right and not only as a destination for referral. See Part 1, section 1.1.5.5 and Part 5, section 5.4.3.
Social-aspects/public-relations organization (SAPRO)	Industry-funded body whose stated purposes are public information, self-regulation or CSR, but which is widely understood to perform policy-interference functions.
Transnational alcohol corporation	Large alcohol producer or retailer operating across multiple countries, typically controlling a substantial share of the global or regional market.

Executive summary

Alcohol is among the leading preventable causes of death, disease and social harm worldwide. It causes millions of deaths each year and a substantial share of the global burden of disease, spanning noncommunicable diseases (NCDs) such as cardiovascular disease and several cancers, injuries and communicable diseases, with a large population living with alcohol use disorders. The harm is unequally distributed: low-income countries carry a disproportionate share of the per-litre health burden; men account for the great majority of alcohol-attributable deaths; and young adults account for a disproportionate share of alcohol-attributable deaths. On current trajectories, countries are unlikely to achieve the 20% relative reduction target set by the WHO *Global alcohol action plan 2022–2030* without accelerated, protected and well-coordinated action.

In 2018 the World Health Organization (WHO), in collaboration with international partners, launched the SAFER initiative as the operational backbone of national alcohol policy efforts. SAFER is an acronym for the five high-impact, cost-effective interventions to reduce alcohol-related harm: Strengthen restrictions on alcohol availability; Advance and enforce drink–driving countermeasures; Facilitate access to screening, brief interventions and treatment; Enforce bans or comprehensive restrictions on alcohol advertising, sponsorship and promotion; and Raise alcohol prices through excise taxes and pricing policies. The *Global alcohol action plan 2022–2030*, endorsed by the Seventy-fifth World Health Assembly in May 2022, makes SAFER central to its first action area on the implementation of high-impact strategies and interventions, and surrounds those interventions with five further action areas on advocacy, partnership, technical support, knowledge production and resource mobilization.

This handbook operationalizes the SAFER package across three levels: the national (Parts 1 to 4), the community (Part 5) and the workplace and institutional (Part 6). It is designed for ministries of health, other government ministries, departments and agencies, subnational authorities at provincial, municipal and local-council level, WHO country and regional offices, United Nations (UN) partners, civil-society organizations (CSOs), academic and research institutions, and people with lived experience of alcohol harm. Part 1 covers the three sequenced national instruments by which a country builds the foundation for SAFER implementation: a Country SAFER Landscape Report, a Joint SAFER Assessment and a SAFER Roadmap. Part 2 provides the framework for monitoring, evaluation and learning across the SAFER cycle. Part 3 sets out how to select and present the economic evidence for alcohol policy, including the structure and methodology for an investment case, and the arrangements for sustainable financing. Part 4 provides practical guidance for protecting SAFER interventions from conflicts of interest and alcohol industry policy interference. Part 5 translates the SAFER package into action at the community level, covering all subnational tiers from provinces and states through districts and municipalities to submunicipal and local councils. Part 6 provides an adaptable workplace alcohol guidance template for governments, community authorities, United Nations entities and other large public-sector employers, addressing both prevention and protection from industry interference.

Each Part is self-contained and concludes with its own list of references, but the parts are designed to be used together as a coherent implementation pathway. A typical country adopting SAFER for the first

time will move through Part 1 in sequence (Module 1 ➜ Module 2 ➜ Module 3), apply Part 2 throughout, and embed the Part 4 protections from the outset. It will develop the Part 3 economic assessment alongside the Roadmap, as an investment case where that is the appropriate vehicle. It will then translate the result into Part 5 community-level action, where the work meets the lived experience of citizens. A country that already has a landscape analysis, assessment report or action plan may enter the cycle at the relevant point and use this handbook as a diagnostic against which to benchmark and strengthen existing processes.

Three framing choices deserve emphasis. First, the handbook treats alcohol harm as strongly shaped by structural determinants: physical and economic availability, marketing exposure, the price environment, and the strength of public-policy protection from commercial interests. Individual behaviour and choice matter in their own right, but the environment determines how easy or hard the healthier choice is to make. The implications run through every Part: the SAFER actions with the strongest population-level evidence are those that change the regulated environment, working together with education, awareness and support for individual change. Second, this handbook takes seriously the proposition that national policy and community-level implementation are mutually constitutive. National laws shape what communities can do; community delivery largely determines whether national laws take effect; and the evidence and political momentum generated by communities are among the principal forces shaping national policy reform. Part 5 is therefore not an appendix to the national handbook but an equal partner.

Third, this handbook is written government-first, because governments carry the primary mandate for formulating and implementing alcohol policy. It is not written government-only: policy is also made and applied at other levels, more often through regulatory and administrative instruments than through statute – by municipalities and communities, by workplaces and institutions (Part 6), and within the governance of CSOs. The handbook treats these as part of the policy architecture rather than as its audience. It also treats the relationship between government and the people it serves as an operational condition of success, not a courtesy. The communities in which policy takes effect, the CSOs that monitor and sustain it, and the people affected by alcohol harms are the source of much of the legitimacy, evidence and delivery on which the measures in this handbook depend. Alcohol policy that is technically sound but loses the trust and ownership of the people it serves struggles to be implemented and rarely endures.

The practical guidance in this handbook operates across levels, and the dynamic between them is its core. Global normative instruments – the Global strategy, the Global alcohol action plan 2022–2030, the SAFER technical package and the WHO best-buy list – set the framework within which national action is designed. Direct country engagement gives that framework operational form, through country SAFER landscape reports, joint SAFER assessments and national roadmaps supported by WHO and partners across all six WHO regions. Country experience feeds back into the guidance, including the Uganda and Nepal experiences documented in the WHO progress report on the SAFER initiative. Intercountry learning, convened through WHO regional offices, regional networks and the United Nations Inter-Agency Task Force on the Prevention and Control of NCDs, carries lessons between countries. Within that architecture, two contemporary reference points inform the community-level guidance in particular. The first is the Building SAFER Communities project in Ireland (2024–2026), led by Well Ireland (formerly Alcohol Forum Ireland) and the Irish Community Action on Alcohol Network; its 15 community-level actions, two-tier governance, evaluation architecture and i-Mark approach to industry-interference protection inform Part 5 throughout. The second is the WHO City-level monitoring guidance for the prevention and control of noncommunicable diseases and injuries (2023), whose 2024 assessment instrument and 20-city pilot provide the operational scaffold for community-level monitoring and for alignment between community, national and global indicator frameworks.

How to use this handbook

This handbook is a menu, not a checklist. It describes the full set of practices that contribute to durable, protected, evidence-based national and community alcohol policy. No country is expected to do all of it at once.

Three observations frame how to use it:

Alcohol policy development rarely follows a linear sequence. Countries enter the cycle opportunistically – through a window of political opportunity on tax, a road-safety reform, a public concern about youth drinking, a request from a parliamentary committee, a new government, a crisis. The handbook is written as a sequence for ease of reference but should not be read as one.

Capacity varies. A ministry of health with a 30-person alcohol unit will use the handbook differently from a ministry of health with one focal point and no protected budget. Both are legitimate starting positions. The handbook is intended to serve both.

There are no shortcuts on the institutional foundations. The handbook is permissive about scope, scale and sequencing. It is not permissive about governance, conflict-of-interest safeguards or monitoring. Alcohol policy gains secured without those foundations have frequently been reversed, often within an electoral cycle. The minimum pathway described below is a smaller entry point into the full alcohol policy architecture, not a scaled-down parallel version.

Entry profiles

Profile A. Country starting from limited capacity

A country with limited dedicated alcohol policy capacity should not attempt the full Module 1 ➜ Module 2 ➜ Module 3 sequence as the first step. The minimum pathway (see Box 1) is the recommended entry point. It produces a small but real set of outputs in 6–12 months that can be built on in subsequent cycles: a named focal point, a stakeholder and harm map, a conflict-of-interest protocol, one or two priority actions, a small indicator set, and a visible link to the national SAFER architecture.

Priority reading: Introduction; this page; Box 1 (below); Part 4 (in summary, then in full); Part 5 section 5.7.4.

Profile B. Country with a partial alcohol policy plan already in place

A country with an existing alcohol policy or strategy that pre-dates this handbook should use the handbook as a diagnostic, not as a fresh start. The relevant question is what is missing from the existing plan, not what an idealized plan would look like. Common gaps detected by such a diagnostic include weak monitoring infrastructure (Part 2), absence of a costed economic case for investment (Part 3), weak protection from industry interference (Part 4), and weak community-level translation (Part 5).

Priority reading: this page; Part 2; Part 4; Part 5; selected sections of Parts 1 and 3 as identified by the diagnostic.

Profile C. Country preparing for a full SAFER implementation cycle

A country with adequate capacity and resources, political momentum and partner support can use the handbook as a full implementation cycle: Module 1 (Landscape Report) ➜ Module 2 (Joint SAFER Assessment) ➜ Module 3 (SAFER Roadmap), with Parts 2, 3, 4 and 5 applied in parallel. The recommended sequence is set out in the Introduction. Even in this case, the institutional foundations should be in place before high-profile single actions are pursued.

Priority reading: the full handbook, with the Introduction setting the working order.

Profile D. Community or municipal authority

A community, municipality or other subnational authority can use the handbook without waiting for a completed national process, or to reinforce national policies that are already strong. Part 5 is written for this reader: it translates the five SAFER intervention areas into community-level action menus (Tables 27 to 31), sets out a five-step pathway for building a Community SAFER Plan (Table 34), and pairs both with monitoring guidance (section 5.5) and fill-in tools (Annexes 8 to 11). The conflict-of-interest safeguards in Part 4 apply at community scale from day one, and the minimum pathway in Box 1 is as valid for a municipality as for a ministry. Where a national SAFER process exists, community work should link to it visibly; where it does not yet exist, documented community action is one of the strongest arguments for starting one.

Priority reading: Part 5; Part 4; Box 1 (below); Annexes 8 to 11.

Profile E. Workplace or institution

Public agencies, employers, universities, health services and international organizations enter the handbook through Part 6, which addresses the settings in which policy-makers and implementers themselves work. Its premise is that institutions advocating alcohol policy change are more credible when they walk the talk: an internal alcohol policy, protection of staff health, and clear rules on alcohol at official functions and on interaction with alcohol industry actors. Annex 12 provides a complete workplace policy template that can be adapted to organizational context, including for United Nations system and development organizations.

Priority reading: Part 6; Annex 12; Part 4.

Profile F. Civil-society organization or network

Civil-society organizations (CSOs) and networks enter the handbook through Part 5 and Part 4. The action menus (Tables 27 to 31) identify where civic advocacy matters most, Part 2 supports evidence-based argument, and Part 4 is essential reading twice over: for safeguarding an organization's own independence from alcohol industry funding, and for holding public processes to the same standard. CSOs are a constituency of this handbook in their own right, not only implementation partners of government: they may initiate, convene and lead community SAFER programmes (section 5.9), mobilizing the statutory authorities that hold the regulatory, fiscal and enforcement powers, as well as providing public information and independent monitoring of implementation and of industry conduct (section 5.7.5). Global and regional civil-society networks play these roles at larger scale, channelling evidence, technical support and solidarity towards national and community action.

Priority reading: Part 5; Part 4; Part 2.

Box 1. The minimum pathway: a smaller entry point into the full alcohol policy architecture

Countries with limited capacity, and communities with limited capacity, can begin SAFER with a small set of foundational steps that do not require the full Module 1 ➜ Module 2 ➜ Module 3 sequence:

- appoint a named focal point with a clear mandate and a protected, even if small, budget;
- map the principal alcohol-related harms, the principal stakeholders and the principal alcohol industry actors using available data and rapid key-informant work – the formal Landscape Report comes later;
- adopt a written conflict-of-interest protocol from day one (Part 4); this applies regardless of scale;
- select one or two priority SAFER actions feasible with current legal powers and capacity;
- define a small indicator set drawn from the WHO city-level monitoring guidance and aligned with the *Global alcohol action plan 2022–2030* (Part 2 and Part 5 section 5.5);
- report progress to the relevant elected or administrative authority on a defined cycle;
- link the work visibly to the national SAFER architecture so that the minimum pathway is recognized as part of the national plan, not separate from it.

The minimum pathway is a real entry point, not a lower standard of ambition. It is built on the same institutional foundations – governance, conflict-of-interest safeguards, monitoring – as the full implementation cycle. The foundations apply at every scale. What scales with capacity is scope, not architecture.

Introduction to the handbook

Setting the scene

Alcohol is a psychoactive and toxic substance with dependence-producing properties. It is causally linked to more than 200 disease and injury conditions. These include cancers of the lip, oral cavity, pharynx, oesophagus, liver, colon, rectum, larynx and breast; cardiovascular disease, liver cirrhosis, pancreatitis and alcohol use disorders; communicable diseases such as tuberculosis, HIV/AIDS and lower respiratory infections; unintentional injuries such as road traffic injuries, poisonings and drownings; and intentional injuries such as self-harm, suicide and interpersonal violence *(1)*. Alcohol is classified as carcinogenic to humans, and reducing or ceasing consumption lowers the risk of some alcohol-related cancers *(2)*. Beyond its direct effects on health, alcohol drives substantial social and economic harm: reduced workforce productivity, absenteeism, unemployment, crime and delinquency, intimate-partner and gender-based violence, family disruption, child maltreatment and household poverty *(1, 3)*. These harms cluster within and across societies in particular age groups, particular neighbourhoods and particular settings, and they are amplified by alcohol marketing and by environments that normalize heavy consumption.

The scale of the global problem is large and persistent. The 2024 World Health Organization (WHO) *Global status report on alcohol and health and treatment of substance use disorders* documents that, although age-standardized alcohol-attributable death rates fell by approximately 20% between 2010 and 2019, the absolute burden remains very high *(1)*. The evidence summarized below is not simply background context; it forms the case for sustained national and community action that animates every Part of this handbook. The global toll is summarized in Box 2.

Box 2. Global burden of alcohol-related harm

- Alcohol is among the leading preventable causes of death and disability worldwide, contributing to more than 200 disease and injury conditions and causing an estimated 2.6 million deaths in 2019 *(1)*.
- Much of this burden falls on people in the prime of life. Younger adults account for a disproportionate share of alcohol-attributable deaths, and a large share of it is avoidable *(1)*.
- Per-litre risk of death and disability is highest in low-income countries and low-income groups *(1)*.
- The burden spans noncommunicable diseases (NCDs) such as cardiovascular disease and cancer, injuries and communicable diseases *(1)*.
- In 2019, an estimated 400 million people worldwide were living with alcohol use disorders, including many with alcohol dependence *(1)*.

The 2024 status report finds that, in 2019, only 56% of 124 reporting countries had a written national alcohol policy in place; only 38% of them required warning labels on alcohol products; and more than half of countries with excise taxes did not adjust them for inflation *(1)*. Almost half of reporting countries also recorded interference from the alcohol industry in policy development – and countries making greater policy progress may face stronger commercial opposition *(1)*.

Across the WHO regions, the picture is uneven. Global adult per capita consumption fell slightly, from 5.7 litres of pure alcohol in 2010 to 5.5 litres in 2019, with the highest levels in 2019 recorded in the European Region (9.2 litres) and the Region of the Americas (7.5 litres) *(1)*. More recent estimates from the WHO Global Information System on Alcohol and Health indicate a continued decline, to 4.9 litres in 2024 (three-year average) *(4)*. The number of drinkers fell across all regions, but the patterns of drinking among those who do consume have, in many places, intensified.

The Global alcohol action plan 2022–2030

To address this burden, the global community has adopted a coherent set of instruments. The WHO *Global strategy to reduce the harmful use of alcohol* (2010) provided the original ten-area framework for action *(5)*. The WHO *Global alcohol action plan 2022–2030*, endorsed by the Seventy-fifth World Health Assembly in May 2022, strengthened that framework into an operational plan. It sets a clear goal, at least a 20% relative reduction in the harmful use of alcohol by 2030 compared with 2010, and organizes proposed actions into six action areas *(3)*. Those six action areas frame the structure of this handbook:

1. Implementation of high-impact strategies and interventions – operationalized through the *SAFER technical package*.

2. Advocacy, awareness and commitment – addressed in Parts 2 and 4 and integrated into community-level work in Part 5.

3. Partnership, dialogue and coordination – addressed across all parts, with particular emphasis in Part 1 (joint SAFER assessments) and Part 5 (community-level steering groups).

4. Technical support and capacity-building – implicit in every Part of the handbook, made explicit in Part 5's treatment of training and accreditation.

5. Knowledge production and information systems – addressed through the national monitoring, evaluation and learning framework in Part 2, complemented at community level by Part 5's use of the WHO city-level monitoring guidance.

6. Resource mobilization – addressed in Part 3 (economic evidence and sustainable financing) and Part 5 (community-level resource mobilization).

The *SAFER technical package* is the operational backbone of Action Area 1, but it pervades the other five. The measures and interventions in SAFER therefore form the core of the action plan's implementation at every level – global, regional, national and community *(3, 6)*.

Introduction to the SAFER initiative

WHO launched the SAFER initiative in September 2018 at the third United Nations (UN) High-level Meeting on the Prevention and Control of NCDs, in collaboration with international partners *(7)*. SAFER supports Member States to reduce the harmful use of alcohol and the resulting health, social and economic impact through adoption and implementation of the Global strategy (2010), the *Global alcohol action plan 2022–2030* and the WHO Global action plan for the prevention and control of noncommunicable diseases (2013–2020, extended to 2030) *(8)*. It also supports the United Nations Sustainable Development Goals, particularly target 3.5 on strengthening the prevention and treatment of substance abuse, including narcotic drug abuse and harmful use of alcohol *(6, 9)*.

The five letters of SAFER each name an evidence-based, cost-effective intervention area. Several of these interventions are recognized as WHO "best buys" – interventions with cost-effectiveness ratios of I$ 100 or less per disability-adjusted life year (DALY) averted in low- and lower-middle-income countries *(10)*. The five intervention areas are summarized in Table 1, with the typical national-level levers; the community-level translation of each area is set out in Part 5, section 5.4.

Table 1. The SAFER package: five high-impact interventions and their typical focus at national level

Letter	Intervention area	Focus at national level
S	**Strengthen** restrictions on alcohol availability	Licensing of production, sale and service; outlet density; days and hours of sale; minimum legal purchase age; restrictions on alcohol use in public places.
A	**Advance** and enforce drink–driving countermeasures	Blood alcohol concentration (BAC) limits; sobriety checkpoints; random breath testing; graduated licensing; ignition interlocks; mass-media campaigns.
F	**Facilitate** access to screening, brief interventions and treatment	Health-system capacity for early identification and management of hazardous drinking and alcohol use disorders, including in primary care and emergency services.
E	**Enforce** bans or comprehensive restrictions on alcohol advertising, sponsorship and promotion	Statutory bans, co-regulation, surveillance of marketing, enforcement against infringements, elimination of marketing to minors, regulation of digital and social-media marketing.
R	**Raise** alcohol prices through excise taxes and pricing policies	Specific excise taxes; regular inflation-based adjustments; bans on volume and price promotions; minimum unit pricing; removal of subsidies.

Source: adapted from the WHO SAFER technical package *(6)*.

SAFER is coordinated by WHO headquarters, regional and country offices together with the United Nations Inter-Agency Task Force on the Prevention and Control of NCDs (UNIATF), the United Nations Development Programme (UNDP) and a group of international non-State partners – including the Global Alcohol Policy Alliance, Movendi International, the NCD Alliance and Vital Strategies *(7, 11)*. SAFER rests on three strategic functions, set out in the WHO technical package: Implement, Monitor and Protect *(6)*.

Those three functions are not separate phases of work but parallel and continuous strategies. Their treatment in this handbook reflects this:

- **Implement** is addressed at national level in Part 1 (Modules 1 to 3) and at community level in Part 5, section 5.4.
- **Monitor** is addressed at national level in Part 2 and at community level in Part 5, section 5.5, anchored on the WHO City-level monitoring guidance for noncommunicable diseases and injuries *(12)*.
- **Protect** is addressed at national level in Part 4, at community level in Part 5 section 5.6, and at the workplace and institutional level in Part 6 of this handbook, with workplace alcohol guidance currently under discussion among United Nations agencies.[1]

The menu of SAFER interventions

Table 2 summarizes the evidence-based activities that can be drawn on for the Roadmap matrix. It is adapted from the WHO *SAFER technical package* and reflects the consolidated WHO best-buy list *(6, 10)*. Countries may add activities outside this menu provided they are contextualized and evidence-informed. SAFER is defined by impact, not by cost-effectiveness alone: the best-buy list identifies interventions that are highly cost-effective, but SAFER brings together high-impact measures across all five areas, several of which extend beyond the best-buy subset. There is now direct evidence that well-designed alcohol policies reduce alcohol consumption at population level *(13)*.

Table 2. Menu of SAFER interventions

SAFER intervention area	Activities
S **Strengthen** restrictions on alcohol availability	Implement licensing systems for production, wholesale, sale (including delivery) and service; regulate outlet number, density and location; regulate hours and days of sale; set and enforce a national minimum legal age for purchase and consumption; restrict alcohol use in public places.
A **Advance** and enforce drink–driving countermeasures	Set and enforce blood alcohol concentration (BAC) limits (with lower limits for novice and professional drivers); sobriety checkpoints; random breath testing; administrative licence suspension; graduated licensing; ignition interlocks; driver education; alternative transport; counselling and, as appropriate, treatment for repeat offenders; planned, high-intensity mass-media campaigns.
F **Facilitate** access to screening, brief interventions and treatment	Build health- and social-system capacity for prevention, treatment and care of alcohol use disorders and related conditions, including family support; support screening and brief interventions in primary care and other settings (with special attention to pregnant women); integrate prevention, treatment and care of alcohol use disorders with comorbid conditions (drug-use disorders, depression, suicide, HIV/AIDS, tuberculosis); ensure universal access to affordable treatment services including for low-socioeconomic-status groups.
E **Enforce** bans or comprehensive restrictions on alcohol advertising, sponsorship and promotion	Statutory or co-regulatory bans or restrictions on exposure to alcohol advertising; supplementary self-regulation consistent with statute; elimination of marketing and advertising of alcohol products to minors and in digital environments; surveillance of alcohol marketing by public or independent bodies; administrative and deterrent systems for infringements.

[1] Workplace alcohol guidance has been discussed among United Nations agencies but has not been published as an agreed instrument; it is referred to here in general terms only.

SAFER intervention area	Activities
R **Raise** alcohol prices through excise taxes and pricing policies	Establish a system of specific domestic taxation (with effective enforcement) that may reflect the alcohol content of the beverage; regularly adjust taxes and prices for inflation and income; restrict or prohibit direct and indirect price promotions, discount sales, sales below cost and flat-rate unlimited alcohol offers; establish minimum prices where applicable; price incentives for non-alcoholic beverages; reduce or eliminate subsidies to alcohol operators.
Overarching actions	Monitoring and evaluation (M&E) systems (Part 2); economic evidence and investment cases for alcohol policy (Part 3); advocacy and awareness-raising on alcohol harms; protection of policy from alcohol industry interference (Part 4); community-level translation (Part 5).

Source: adapted from the WHO SAFER technical package *(6)*.

Current implementation of SAFER

Since its launch in 2018, SAFER has been adopted as the flagship operational framework for alcohol policy in the Global alcohol action plan. WHO and partners have supported countries across all six WHO regions to conduct SAFER country landscape analyses and joint assessments; to develop and cost national alcohol policy action plans and investment cases; to strengthen drink–driving legislation, advertising restrictions and alcohol taxation; and to integrate screening and brief interventions into primary health care *(1, 6, 7, 11)*. Implementation progress across regions and countries, and the partnership architecture supporting it, are documented in the WHO progress report on the SAFER initiative *(14)*.

Progress has been uneven. The 2024 WHO *Global status report on alcohol and health and treatment of substance use disorders* provides the most recent comprehensive global assessment *(1)*. On the policy side, the report documents persistent gaps in national alcohol policy, warning labels, advertising restrictions and inflation-indexed taxation, with the deepest gaps in low-income countries. On the implementation side, almost half of reporting countries reported alcohol industry interference in policy development. On current trajectories, countries are unlikely to achieve the 20% relative reduction target set in the action plan without accelerated and protected national action – and countries advancing stronger policy measures may also encounter increased commercial opposition *(1, 3)*.

The international architecture has also evolved. Multi-country technical-assistance initiatives are supporting national governments in low- and middle-income countries (primarily in Latin America, Africa and Asia) to implement the SAFER policy actions on taxation, availability and marketing. These include the RESET Alcohol initiative, led by Vital Strategies in partnership with the Global Alcohol Policy Alliance, Movendi International, the NCD Alliance and Johns Hopkins University Tobacconomics *(15)*. At the community level, the Building SAFER Communities project in Ireland (2024–2026) has been described as one of the first efforts at national scale, across multiple sites, to translate the SAFER package into a coherent, evaluated community-level demonstration *(16, 17)*. At the monitoring level, the WHO City-level monitoring guidance (2023) and its 2024 assessment instrument have created a standardized global tool for tracking community-level alcohol policy progress alongside seven other policy areas *(12, 18)*. This handbook draws all three of those developments into the SAFER architecture.

Purpose, audience and scope

The purpose of this handbook is to provide technical guidance to Member States and their partners on how to:

- prepare a Country SAFER Landscape Report (Part 1, Module 1);
- plan and conduct a Joint SAFER Assessment (Part 1, Module 2);
- develop and implement a SAFER Roadmap (Part 1, Module 3);
- establish a monitoring, evaluation and learning system for SAFER (Part 2);
- select the economic assessment that fits the decision to be informed and develop an alcohol policy investment case where appropriate (Part 3);
- protect SAFER interventions from conflicts of interest (COI) and policy interference (Part 4);
- translate SAFER into community-level action across all subnational tiers (Part 5); and
- establish workplace alcohol guidance in community authorities, United Nations entities and other large employers (Part 6).

The handbook is written primarily for: government officials at national, subnational and local-council level with policy or operational responsibility for alcohol-related matters; ministries of health and other relevant ministries, departments and agencies; WHO country, regional and headquarters staff and other UN agencies; CSOs, academia, professional associations and national non-State actors engaged in alcohol policy advocacy, research or service delivery; parliamentarians, legislative drafters, law-enforcement agencies and regulators; and people with lived experience of alcohol harm whose participation in policy and programme design is increasingly recognized as essential.

Independent CSOs are a primary audience for this handbook. The handbook is designed to support not only government and statutory partners but also CSOs operating in monitoring, advocacy, service-delivery and watchdog roles. Civil society's independence – from alcohol industry funding and from government direction – is structurally important to the protection architecture of SAFER and is treated in detail at Part 5 section 5.7.5 and at Part 4 section 4.7. The same logic runs in the other direction: while government is the prime formulator and implementer of alcohol policy, the trust and participation of the communities it serves, of civil society and of people affected by alcohol harms are an operational requirement for durable policy, not a consultation formality.

Box 3 places SAFER's design in the context of a long-standing debate in global health between vertical and horizontal approaches.

Box 3. Vertical or horizontal? An old debate returns

The tension between dedicated programmes and integrated approaches is as old as WHO itself. The terms "vertical" and "horizontal" were used by Carlos L. González, whose 1965 WHO monograph drew the lessons of the single-purpose campaigns of the 1950s against yaws, malaria and smallpox *(19)*. His conclusion was that the approaches are not mutually exclusive: dedicated campaigns achieved what general services could not, but their gains endured only where the handover to general health services was deliberately designed rather than assumed. Later analyses reframed the question: integration is a spectrum rather than a binary, and the useful question is not whether a programme integrates but which of its functions integrate, and under what conditions *(20, 21)*.

The debate has returned with force. The Lusaka Agenda (2023) calls for a shift away from fragmented, vertical, donor-driven programming towards country-led, integrated primary health care *(22)*, and the 2025 Political declaration on noncommunicable diseases and mental health – adopted at a high-level meeting held under the theme "Equity and integration" – commits Member States to a fully integrated response *(23)*. At the same time, contractions in development assistance for health have driven integration by necessity rather than design, the very failure mode González identified at the handover stage.

SAFER is built for this environment. It is vertical only where alcohol policy has historically been diluted within broader agendas: a named package, a defined target, dedicated monitoring, and explicit protection from alcohol industry interference. It is horizontal in delivery: screening and brief interventions within primary health care, taxation within general fiscal administration, drink–driving countermeasures within road safety. Countries implementing SAFER are not building a parallel structure; they are protecting the elements of alcohol policy that inclusion within broader frameworks has not, by itself, been sufficient to advance *(1, 24)*.

How this handbook was developed

This handbook consolidates eight years of implementation experience gained through the SAFER initiative since its launch at the third United Nations General Assembly High-level Meeting on noncommunicable diseases in 2018. It is an implementation tool intended to support the application of existing WHO recommendations and policy instruments. It does not issue new WHO recommendations and was not developed as a WHO guideline involving formal assessment or grading of evidence.

Normative and evidence base

The five SAFER intervention areas rest on existing WHO normative and technical products: the Global strategy to reduce the harmful use of alcohol; Appendix 3 of the Global action plan for the prevention and control of noncommunicable diseases 2013–2030; the SAFER technical package: five areas of intervention at national and subnational levels; the WHO "best buys" and other recommended interventions for the prevention and control of noncommunicable diseases; and the Global alcohol action plan 2022–2030, which established SAFER as an operational mechanism for implementing evidence-based alcohol policy. Where the handbook describes an intervention as effective or cost-effective, it does so by reference to these sources.

Relevant WHO and United Nations materials were reviewed for specific subjects, including alcohol taxation and pricing, availability, marketing, drink–driving, screening, brief interventions and treatment, monitoring and evaluation, investment cases, engagement with non-State actors and management of conflicts of interest. Peer-reviewed research, official national sources and documented implementation experience were used where relevant and are cited in the corresponding parts of the handbook.

The identification of sources was purposive and directed towards the operational requirements of the handbook. It was not a systematic review. No protocol-based literature search, formal risk-of-bias assessment or grading of the certainty of evidence was undertaken specifically for the handbook. Statements about effectiveness rely on existing WHO recommendations, technical products and evidence syntheses. Implementation experience was used to identify practical options, institutional requirements, barriers and considerations for adaptation, not as independent proof of intervention effectiveness.

Implementation experience

The practical content draws on the implementation record documented in the 2026 progress report on the SAFER initiative and on related materials generated through country, regional and global SAFER processes.

Detailed operational learning was drawn particularly from in-depth SAFER country engagement in Uganda and Nepal. This included landscape analysis; joint engagement by governments, WHO and other United Nations entities; prioritization and roadmap development; implementation support; investment case work; monitoring and progress review; and national and subnational action. These experiences informed the handbook's treatment of governance, multisectoral coordination, implementation capacity, enforcement, health-system integration, financing, monitoring, civil-society participation and protection from alcohol industry interference.

The handbook also draws on structured inter-country learning organized by WHO regional offices and partners. These processes brought multisectoral country teams together to review national policy landscapes, identify implementation gaps, exchange experience and develop or refine delivery plans. They included preparatory work, facilitated technical exchange and follow-up, rather than only stand-alone workshops. Regional capacity-building and cooperation through WHO regional offices, including the Pan American Health Organization, the WHO Regional Office for Europe and the WHO Regional Office for the Western Pacific, also informed the handbook.

Engagement through the United Nations Inter-Agency Task Force on the Prevention and Control of Noncommunicable Diseases and the United Nations Development Programme contributed experience concerning multisectoral country support, governance, public finance and integration of alcohol policy into wider development processes. Civil-society and technical partners contributed implementation experience, evidence translation and perspectives on advocacy, accountability and protection of alcohol policy from commercial interference.

Development of specific components

Methods for economic assessment of alcohol policy, monitoring, evaluation and learning, and management of conflicts of interest were developed and refined through dedicated technical working groups and workstreams. These groups focused on development of the underlying methods and approaches, not on development of the handbook itself. Relevant outputs and implementation experience were subsequently reviewed, synthesized and adapted for inclusion in the corresponding parts of the handbook.

The economic assessment component draws on WHO and United Nations approaches to economic evaluation and investment cases for noncommunicable diseases, and on country applications. The monitoring component was aligned with the Global alcohol action plan 2022–2030, the Global Information System on Alcohol and Health, the WHO Global Monitoring Framework for Noncommunicable Diseases as applied to the Global action plan for the prevention and control of noncommunicable diseases extended to 2030, Sustainable Development Goal target 3.5 and WHO city-level monitoring guidance. The conflict-of-interest component draws on relevant WHO policy instruments and on practical country experience of protecting alcohol policy from commercial interference.

The community-level component was informed particularly by collaboration with Well Ireland, formerly Alcohol Forum Ireland, and the Irish Community Action on Alcohol Network through the Building SAFER Communities initiative, together with experiences from Nepal and Uganda. Ireland was not treated as an in-depth SAFER country partnership equivalent to Uganda or Nepal. Its contribution was used as a contemporary example of localization of SAFER in a high-income setting, including community governance, action planning, training, evaluation and protection from industry influence. As the initiative was still being implemented, the handbook draws primarily on its design and implementation approach rather than on demonstrated population outcomes.

The part on workplaces and institutions was developed partly through collaboration with the United Nations Inter-Agency Task Force on the Prevention and Control of Noncommunicable Diseases and work with United Nations agencies towards a model policy on preventing alcohol-related harm and addressing alcohol industry interference. This was informed by United Nations experience with institutional measures on tobacco, adapted to the different normative context for alcohol, and by relevant country and institutional experience. At the time of preparation, the model alcohol policy had not been adopted as an agreed United Nations system policy. Part 6 and Annex 12 therefore provide WHO implementation guidance and an adaptable template; they do not reproduce or represent an approved United Nations system policy.

Country examples, consultation and review

Country examples were selected to illustrate implementation choices, enabling conditions, obstacles and responses in different governance, income and resource settings. Selection was influenced by the relevance and availability of sufficiently detailed documentation. The examples are illustrative and were not selected through systematic comparative sampling. They describe what was done and observed and do not attribute changes in alcohol consumption or harm to the measures described unless supported by cited evaluation evidence. Ongoing initiatives and early implementation outputs are distinguished from demonstrated outcomes.

The handbook was prepared by the Alcohol, Drugs and Addictive Behaviours team at WHO headquarters in collaboration with relevant WHO technical departments and regional and country offices. United Nations partners, external specialists, civil-society organizations and other contributors with relevant technical or implementation experience provided input to selected parts. Successive drafts were informed by written contributions, technical discussions and review comments. Country material was checked against available documentary sources and, where feasible, reviewed with relevant WHO offices and country or implementation counterparts.

Declarations of interest were managed in accordance with applicable WHO requirements at relevant stages of the work. Decisions on the content and formulation of the handbook remained with WHO. Contributors and reviewers are listed in the acknowledgements, and final responsibility for the handbook rests with WHO.

The development and publication of the handbook were funded by the Government of Norway and the Norwegian Agency for Development Cooperation (Norad), as recorded in the acknowledgements. The funder had no role in determining the content of this handbook. No funding, in-kind contribution or other support was accepted at any stage from alcohol industry actors, from bodies substantially funded by them, or from organizations acting on their behalf. Contributing non-State actors were subject to the same declaration-of-interest requirements as other contributors.

Adaptation and limitations

The processes, action menus, templates and checklists are intended for adaptation to national and subnational legal authority, governance arrangements, institutional capacity, available data and resources. Adaptation should preserve the evidence-based content and public health objectives of the five SAFER intervention areas.

The implementation experience reflects countries and institutions that engaged with WHO and partners through SAFER, rather than the state of alcohol policy worldwide. The depth of documentation varies among countries and regions, and the examples are not representative samples or a basis for comparing country performance. The evidence base is stronger for the effectiveness of the principal SAFER interventions than for every aspect of their operational delivery. Some operational guidance therefore reflects synthesis of documented implementation experience and expert judgement. Several initiatives described were ongoing during preparation of the handbook, and their activities or early outputs should not be interpreted as evidence of impact. Legal, policy and institutional arrangements may also change, and users should verify current national information before applying the guidance.

Translation raises one specific question: how to carry the SAFER name, which is both an acronym and an English word, into other languages. Annex 14 provides a short note on this for translators, communicators and designers.

Review, updating and monitoring of use

This handbook will be reviewed and updated in response to new evidence, revisions to the SAFER technical package and Member State experience, including feedback from SAFER inter-country learning workshops, and in conjunction with the final reporting on implementation of the Global alcohol action plan 2022–2030. Use of the handbook will be monitored through WHO publication metrics and through its application in country SAFER processes. Uptake of the high-impact interventions it supports is tracked through the action plan's monitoring framework, including the global target on implementation of high-impact policy options and interventions by 2030, and through the Global Information System on Alcohol and Health, and is reported periodically through the global status reports on alcohol and health.

Overview of the handbook

The handbook has six substantive parts, preceded by this introduction and followed by a set of annexes. Each Part is self-contained and concludes with its own list of references. Cross-references between parts are indicated in the body text and summarized in Table 3. Section numbering follows the numbering of the parts: the three modules of Part 1 are numbered 1.1 to 1.3, with their sections and subsections nested beneath them, and Parts 2 to 6 carry section numbers 2 to 6, so that a reference such as section 5.4 always denotes Part 5.

Table 3. Structure of the handbook

Part	What it covers	Key cross-references
Part 1 – landscape reports, joint assessments and roadmaps	Three sequenced national modules that guide countries from evidence-gathering (Module 1) through diagnostic country engagement (Module 2) to prioritized implementation planning (Module 3).	Part 2 for monitoring; Part 3 for the economic evidence that supports the Roadmap; Part 4 for COI safeguards integrated from the outset; Part 5 for community-level translation.

Part	What it covers	Key cross-references
Part 2 – Monitoring, evaluation and learning	A results-based framework, planning matrices and reporting templates for tracking SAFER implementation, evaluating results and learning from experience at national level.	Part 5 section 5.5 for community-level monitoring anchored on the WHO city-level monitoring guidance.
Part 3 – Economic evidence and sustainable financing	Guidance on selecting the economic assessment that fits the decision to be informed; a step-by-step process and structure for developing a country investment case where that is the appropriate vehicle; and options for sustainable financing of SAFER implementation.	Part 1 Module 1 for the underlying evidence base; Part 5 section 5.8 for community-level resource mobilization.
Part 4 – Protecting SAFER from conflict of interest	Principles, a five-step action pathway and practical checklists to safeguard alcohol policy processes from industry interference.	Part 5 section 5.6 for community-level COI management; Part 6 and Annex 12 for workplace alcohol guidance applicable to community authorities, the United Nations system and other large public-sector employers.
Part 5 – Community-level implementation of SAFER	Translation of the SAFER package into action at community level – provincial, district, municipal, submunicipal – using the three SAFER strategies (Implement, Monitor, Protect).	Builds on Parts 1 to 4; case study of Building SAFER Communities, Ireland.
Part 6 – Workplaces and institutions	An adaptable workplace alcohol guidance template for governments, community authorities, United Nations entities and other large employers, covering both prevention (staff health, occupational safety, support pathways) and protection (sponsorship, procurement, partnerships, conflict of interest).	Part 4 for COI principles; Part 5 section 5.6 for community-level COI; Annex 12 for the workplace template.
Annexes	Templates, checklists, example tables and worked examples for adaptation to national and community context. Annexes 1 to 7 support Parts 1 and 2 (Annex 3 is also relevant to Part 4); Annexes 8 to 11 support Part 5; Annex 12 supports Part 6; Annex 13 consolidates the fill-in templates from the main text; Annex 14 is a note on the SAFER name and how to carry it across languages, for translators, communicators and designers.	Referenced from Parts 1, 2, 3, 4, 5 and 6 as relevant.

Reading pathways

The handbook is intended to complement – not replace – the national public-policy process. The six entry profiles and reading pathways are set out in "How to use this handbook" at the front of this handbook; the recommended sequence for a country starting from scratch is:

1. Begin with Part 1, Module 1 – prepare a Landscape Report to build the country evidence base.

2. Use Part 1, Module 2 to plan and conduct a Joint SAFER Assessment that validates and deepens the landscape findings.

3. Use Part 1, Module 3 to translate those findings into a prioritized SAFER Roadmap covering 12–24 months.

4. Apply Part 2 in parallel from the outset – monitoring, evaluation and learning should be designed into the Roadmap rather than added afterwards.

5. Use Part 3 to select the economic assessment that fits the decision at hand, to develop an investment case where that is the right vehicle, and to plan sustainable financing for implementation.

6. Apply Part 4 throughout – conflict-of-interest management is a cross-cutting safeguard, not a separate phase.

7. Use Part 5 to extend the work into the community level – provinces, districts, municipalities and local councils.

8. Apply Part 6 to establish workplace alcohol guidance in the community authority itself, in United Nations entities and other multilateral organizations, and in other large public-sector employers.

Countries that already have a landscape analysis, an assessment report or an action plan in place may enter the cycle at the relevant point and use the handbook as a diagnostic against which to benchmark and strengthen existing processes. Countries piloting community-level SAFER implementation may wish to start with Part 5 and use Parts 1 to 4 as the framing for the national context within which the community work sits. Either route is fully supported by the handbook structure.

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PART 1

Country SAFER landscape reports, joint assessments and roadmaps for SAFER implementation

PART 1

Country SAFER landscape reports, joint assessments and roadmaps for SAFER implementation

Part 1 provides three sequenced, self-contained modules that together describe how a country builds the foundation for SAFER implementation:

- Module 1 – developing a Country SAFER Landscape Report;
- Module 2 – planning and conducting a Joint SAFER Assessment;
- Module 3 – developing and implementing a SAFER Roadmap.

Although each module can stand alone, the three are most effective when implemented as a sequence. The Landscape Report builds the evidence base; the Joint Assessment validates, deepens and contextualizes it through direct engagement; the Roadmap then translates the combined findings into a prioritized implementation plan covering 12–24 months with named leads, costed activities and an embedded monitoring framework.

Each module is presented in three parts: a discussion of why the instrument exists and what it is for; the practical process by which a country produces it; and the structure and content of the resulting product. Part 1 as a whole concludes with a single list of references to the sources cited across the three modules.

Cross-references
Part 1 should be read alongside Part 2, which provides the monitoring, evaluation and learning architecture that is built into the Roadmap from the outset; alongside Part 3, which provides the economic evidence, including the investment case methodology, that makes the financial argument for SAFER and the arrangements for financing it; alongside Part 4, which provides the conflict-of-interest safeguards that protect the entire process; and alongside Part 5, which translates the national plan into community-level action.

1.1 Module 1: Developing a Country SAFER Landscape Report

1.1.1 Introduction

The success of SAFER implementation depends on how well the SAFER interventions are integrated into the country's alcohol policy context. That context is many-layered – historical, cultural, economic, political, legal, institutional, epidemiological – and it differs sharply between, say, a high-income Member State with a long-established alcohol-licensing regime and a low-income Member State in which informal alcohol production accounts for a substantial share of consumption. A country adopting SAFER cannot rely on a generic implementation script; it must understand its own context first, in detail, and then design the SAFER interventions accordingly.

The Country SAFER Landscape Report is the instrument that builds that understanding in a structured form. It describes the social, economic, political, policy, legal, institutional, financial and epidemiological factors that shape alcohol policy and the adoption of SAFER interventions in the country. It builds a shared analytical baseline among the ministry of health, government partners, WHO country and regional offices, UN agencies and SAFER non-State partners. That shared baseline is what subsequently informs the Joint SAFER Assessment (Module 2) and the SAFER Roadmap (Module 3).

Cross-country experience suggests that the discipline of producing a Landscape Report – even more than the report itself – is what generates value. The report is the visible output. The invisible output is the inter-agency working relationship that the report-writing process forges between the ministry of health and the other government ministries, departments and agencies (MDAs), between the public sector and civil society, between national and subnational authorities, and between the country and its WHO and UN partners *(1–3)*.

1.1.2 Purpose and scope

The purpose of the Landscape Report is to describe and analyse the country context as it relates to alcohol policy and the implementation of SAFER. It identifies the public and non-State stakeholders to be engaged; provides a baseline for SAFER programming; and informs both the Joint Assessment and the Roadmap. Its scope is a structured review and analysis of the country's alcohol policy, legislative and development context, viewed through the lens of the five SAFER intervention areas and the three SAFER strategies *(4)*. It is not a research thesis: it is a practical analytical document, written to inform decisions, with realistic depth and a finite timeline.

1.1.3 Objectives

The objectives of a Landscape Report are to: deepen understanding of the social, economic, political and institutional context that influences alcohol policy and SAFER implementation; provide a baseline for the design of SAFER programmes and activities; identify needs, gaps and entry points for the implementation of SAFER interventions; and provide guidance to the ministry of health and other MDAs on how SAFER can be implemented in the prevailing context. A well-prepared report identifies what should be done, by whom, in what sequence, and which structural conditions must be addressed before any one SAFER intervention can be expected to deliver results.

1.1.4 Process of developing a Country SAFER Landscape Report

The process of producing a Landscape Report is iterative rather than linear. It typically takes six months from initiation to formal adoption. The 11 steps below describe the sequence used by a number of countries; their order may be adjusted to local circumstances, but each step needs to be completed.

1.1.4.1 Initiation

Preparation of the Landscape Report is initiated by the ministry of health with support from WHO and SAFER partners. Three preliminary considerations should be addressed before formal initiation: securing high-level buy-in, preferably from the senior leadership of the ministry of health; sharing a concept note and proposed process through early communication between the ministry of health, the WHO country and regional offices, and key SAFER partners; and aligning the schedule of the report with the government's fiscal and electoral cycles, so that the report is ready when its findings can be acted on.

1.1.4.2 Leadership and coordination

The ministry of health provides overall leadership and coordination. A Leadership and Coordination Team should be established at the outset, comprising representatives of the ministry of health; the

WHO country office; the WHO regional office and, where appropriate, WHO headquarters; UN partners engaged in the process; SAFER international non-State partners; and SAFER local partners. This team coordinates the overall process, mobilizes resources, takes strategic decisions and ensures successful delivery of the report. Its meetings should be minuted and its decisions documented; the team is the institutional memory of the process.

1.1.4.3 Technical Team

A separate Technical Team carries out the analytical work. It should include technical officers and experts from the ministry of health and other relevant MDAs (typically finance, trade, transport, communication, licensing, justice and law enforcement); the WHO country office and WHO headquarters; UN and SAFER partners; academia and research institutions; and subject-matter experts. The Technical Team prepares the report and advises the Leadership and Coordination Team on process matters; it may be supported by specialist consultants where required. The two teams should not be merged: the separation between strategic oversight and analytical drafting is what protects the integrity of the work.

1.1.4.4 Assignment launch

Once both teams are in place, an official launch meeting – led by the senior leadership of the ministry of health and attended by all partners – signals the start of the assignment. The launch is more than a ceremonial event: it is the public commitment that secures the partner contributions of staff time and access to data on which the rest of the process depends.

1.1.4.5 Timelines

The ministry of health and the Leadership and Coordination Team agree the timelines, typically over six months. The Technical Team prepares a detailed workplan against those timelines, including dependencies on data sources that may be slow to deliver.

1.1.4.6 Communication

Effective communication is maintained throughout – between the two teams, with the relevant MDAs that will provide information, and with stakeholders invited to focus groups and key-informant interviews. A short standing communication protocol covering meeting cadence, document-sharing and confidentiality is recommended at the outset.

1.1.4.7 Stakeholder mapping and engagement

All relevant government and non-State alcohol policy actors are mapped and engaged. The mapping is not a one-off exercise: it is updated through the life cycle of the report and used as a working list for the Joint Assessment (Module 2) and the Roadmap (Module 3). The actors typically engaged at this stage include:

- the ministry of health and the relevant ministry, department or agency for each SAFER intervention area;
- the ministry responsible for finance and the national treasury, and the national revenue authority;
- the ministry responsible for information and communication;
- national police and other law-enforcement agencies;
- the national bureau of statistics;
- the ministry responsible for transport and the road-safety agency;

- Parliament – relevant departmental committees and caucuses;
- subnational governments and local administrations (in many countries, the actors with the greatest day-to-day operational responsibility for licensing, enforcement and primary care – see Part 5 for the community-level treatment);
- alcohol-licensing agencies and regulators;
- the judiciary and the state law office or attorney-general's office;
- academic and research institutions;
- CSOs active on alcohol policy; and
- relevant professional bodies.

Political and ideological drivers beyond industry interference

Barriers to SAFER implementation include alcohol industry interference (Part 4), but they are not limited to it. Across multiple WHO regions, alcohol policy reform also encounters political and ideological positions that operate independently of commercial interests, even when they may align with them in practice.

Three patterns are visible in country experience and should be diagnosed in the Landscape Report.

- **Positions on the scope of government regulation of commerce.** Political parties and administrations that hold a general position against expansion of state regulation of commercial activity may oppose SAFER measures – particularly availability, advertising and pricing controls – on grounds that have little to do with the public health evidence, and that would not be reversed by stronger evidence.
- **Positions on individual responsibility versus collective protection.** Frames that emphasize individual responsibility – "responsible drinking", "informed adult choice" – over structural protection may align with industry framings but also exist independently within broader political traditions. The frames are not simply industry constructs; they are positions held in their own right by some political actors.
- **Positions on the legitimate scope of public health intervention.** Some political traditions hold that public health action should focus on communicable disease, clinical services and education, and should not extend to commercial-determinants regulation. Within this frame, SAFER may be characterized as overreach regardless of evidence.

These positions are not the same as industry interference and should not be conflated with it. They are political positions held in their own right, and they would not disappear if industry interference were fully managed. Successful implementation depends on understanding which of these positions are held by current decision-makers, what frames are most likely to resonate with each, and which cross-political concerns – child safety, road safety, productivity, fiscal benefit, family welfare, criminal-justice cost – provide common ground.

The landscape-report analysis should therefore include an explicit section on political and ideological drivers, distinguishing them from commercial interference and identifying the cross-political coalition that the SAFER Roadmap will need to build. This handbook takes no position on the merits of any political tradition; the analytical point is that successful implementation requires reading the political map accurately, not only the commercial map.

1.1.4.8 Data collection, review and analysis

The Landscape Report draws primarily on secondary data, complemented by key-informant interviews and focus-group discussions. The Technical Team conducts a structured desk review of national and subnational sources: census data, household surveys, economic and budget data, mortality and morbidity registries, police and judicial statistics, regulatory records and government databases. International sources include the WHO Global Information System on Alcohol and Health (GISAH), the WHO *Global status report on alcohol and health*, peer-reviewed literature and grey literature *(1, 5)*. Where data are unavailable or uncertain, the gap is explicitly flagged so that subsequent data-collection efforts can address it.

Data quality should also be explicitly assessed. The Technical Team may classify major data sources as strong, moderate or weak, depending on recency, representativeness, completeness and level of disaggregation. Identified gaps should be documented and carried forward into the SAFER Roadmap and monitoring framework.

1.1.4.9 Integrating Joint Assessment findings

Where a Joint SAFER Assessment has already been conducted (Module 2), its findings are reviewed and integrated. In many countries the Landscape Report and the Joint Assessment run in parallel, with iterative integration.

1.1.4.10 Report writing

The Technical Team prepares a comprehensive Landscape Report following the structure in section 1.1.5. The draft is submitted to the Leadership and Coordination Team for review.

1.1.4.11 Review and validation

The Leadership and Coordination Team reviews the draft against the required structure and convenes a validation meeting with the stakeholders identified in section 1.1.4.7. The validation meeting is the moment at which the analytical work is tested against the lived experience of the actors who will be expected to use it. Substantive changes coming out of validation should be welcomed rather than resisted: a report that survives validation unchanged has typically been written from too narrow a base.

Validation should be treated as a substantive review process rather than a formality. Stakeholder feedback should be used to correct gaps, improve contextual accuracy and strengthen ownership of the findings, while preserving the independence and integrity of the technical analysis.

1.1.5 Structure of a Country SAFER Landscape Report

The Landscape Report adopts the structure described below (Table 4), adapted to local context and language. Where information is unavailable, this should be explicitly noted so that data gaps can be addressed in subsequent cycles. As an indicative planning assumption, a Landscape Report may run to between 80 and 120 pages, organized in 12 sections.

1.1.5.1 Country-context indicators

The country-context section sets the scene (Table 5). Most of the indicators are routinely available from national statistical offices, the World Bank, UN agencies and WHO; the analytical work is in their interpretation, not their collection. Particular attention should be paid to disaggregation: alcohol harm is unevenly distributed by sex, age, geography and socioeconomic status, and aggregated national figures often conceal the patterns that matter most for SAFER implementation.

Table 4. High-level structure of the Country SAFER Landscape Report

Detailed indicator tables for sections 2 to 9 are provided below.

Section	Content and purpose
1. Summary	Two- to four-page summary of key findings and recommendations, written so that it can be read by senior decision-makers without reference to the rest of the report.
2. Country context	Geography, population (size, density, urban/rural, age, sex, ethnicity), administrative divisions, economic profile (gross domestic product, income per capita, World Bank income classification, poverty, employment, inequality), standards of living and governance system. See Table 5.
3. Alcohol consumption and harms	Alcohol-consumption patterns and trends; health consequences; social impacts; economic impacts; alcohol production and supply (including the unrecorded and informal sectors). See Table 6.
4. Policy and legislative context	Constitutional framework; international agreements; national and subnational alcohol laws and policies; notable legal cases; evidence of effectiveness. See Tables 7 and A13.1; Annex 1 gives worked examples.
5. Institutional and governance arrangements, capacities and resources	Multisectoral coordination mechanisms; government bodies with decision-making power; implementation capacities. See Table 8. Annex 5 provides a key-informant interview checklist; Annex 2 a worked example.
6. Health-system capacity for screening, brief intervention and treatment	Availability and use of screening, brief interventions and treatment; providers, costs and challenges. See Table 9.
7. Financing dimensions of alcohol policy	Health-expenditure trends, financial resources, earmarking and taxes, international assistance and donor funding. See Table 10. Cross-reference Part 3 for the investment case methodology.
8. Stakeholder mapping and political drivers	Government MDAs, CSOs and academia, lawmakers and committees, and local and international alcohol industry actors. See Table A13.2. Annex 3 gives a worked example.
9. Potential collaborators in SAFER implementation	Ministries, parliamentary committees, international organizations, CSOs, health-care associations, academia, think tanks, media, religious groups, disease associations, potential donors. See Table 11. Annex 4 gives a worked example.
10. Past alcohol policy efforts and challenges	Analysis of past successes and failures, barriers, critical success factors, observed industry interference.
11. Limitations on information availability	Data gaps and mitigation strategies.
12. Recommendations	Priority actions to strengthen alcohol policy, adopt SAFER interventions, strengthen health and governance systems and mobilize resources.

Table 5. Country-context indicators for the Landscape Report

Data-collection instrument for the country-context section of the Landscape Report; completed by the Technical Team from national statistics and international databases.

Indicator	Definition
Strategic factors	
Universal health coverage	Formal commitment to universal health coverage (all people access the full range of quality health services without financial hardship).
National health strategy	National strategy for health, with an NCD or alcohol policy subsection or subordinate strategy.
Sustainable Development Goals (SDGs)	Commitment to SDG 3.5 (strengthen the prevention and treatment of substance abuse, including harmful use of alcohol).
Demographic and regional factors	
WHO region	One of six WHO regions (African Region, Region of the Americas, South-East Asia Region, European Region, Eastern Mediterranean Region, Western Pacific Region).
Administrative divisions	Subnational divisions (provinces, states, regions, districts, municipalities).
Population size and density	Total population; population per unit area.
Urban/rural share	Proportion of the population in urban/rural settings.
Age, sex, ethnicity composition	Distribution across age cohorts, sex and ethnic groups.
Economic factors	
Income classification	Household economic capacity; World Bank income classification (low-income, lower-middle-income, upper-middle-income and high-income countries).
Gross domestic product (GDP) and GDP per capita	Total economic output and economic output per person.
Income inequality	Distribution of income (for example, Gini coefficient).
Poverty and employment	Headcount ratio below poverty line; unemployment rate; share of formal vs informal workforce.

1.1.5.2 Alcohol consumption and harms

This section provides a comprehensive understanding of the multifaceted impact of alcohol on health, society and the economy (Table 6). It covers consumption patterns and trends, health consequences, social and economic impacts, perceptions, and alcohol production and supply (including the unrecorded sector – typically home-brew, informal production, smuggled, surrogate or cross-border alcohol – which can contribute as much as half of total consumption in some settings). Indicators are drawn primarily from GISAH and the most recent WHO Global status report *(1, 5)*, complemented by national surveys and administrative data.

Table 6. Indicators of alcohol consumption and harms

Data-collection instrument for the consumption and harms section of the Landscape Report; sources include national surveys, registries and GISAH.

Indicator	Definition
Alcohol consumption – patterns and trends	
Alcohol per capita consumption (APC)	Average individual consumption of pure alcohol (litres) per adult (15+) per year.
Heavy episodic drinking	Proportion of individuals consuming ≥60 g pure alcohol on at least one occasion in the past 30 days.
Age, sex, geographic and socioeconomic patterns	Variation in prevalence, frequency and quantity by age, sex, urban/rural, subnational area, wealth and education.
Household expenditure on alcohol	Average household spending on alcohol per year.
Beverage-type patterns (beer, wine, spirits, traditional)	Distribution of consumption across beverage categories.
Unrecorded alcohol consumption	Alcohol consumed but not taxed or captured in official statistics.
Tourist alcohol consumption	Estimated litres of alcohol consumed by visitors aged 15+ in the country.
Health consequences	
Alcohol-attributable mortality and DALYs	Total deaths and DALYs attributable to alcohol; alcohol-attributable fraction of all-cause deaths; years of life lost.
Alcohol-related road traffic injuries	Fatal and non-fatal road traffic injuries with alcohol as contributing factor.
Alcohol use disorder (AUD) prevalence and incidence	Proportion of adults meeting AUD criteria under the Diagnostic and Statistical Manual of Mental Disorders, fifth edition (DSM-5) or the International Classification of Diseases, 11th revision (ICD-11); new cases per period.
Alcohol-related liver disease and cirrhosis	Prevalence and incidence.
Alcohol-related cancers	Prevalence, incidence and mortality for cancers causally related to alcohol (lip and oral cavity, pharynx, larynx, oesophagus, liver, colon/rectum, breast).
Alcohol dependence	Chronic condition of problematic use meeting diagnostic criteria.
Social impacts	
Alcohol-related gender-based and intimate-partner violence	Share of gender-based violence/intimate-partner violence incidents involving alcohol use.
Childhood development impacts	Effects of prenatal alcohol exposure (including fetal alcohol spectrum disorders) and parental alcohol use on physical, cognitive, emotional and social development.
Social-service use related to alcohol	Demand on counselling, rehabilitation, community programmes and homelessness services.
Criminal-justice costs	Costs of alcohol-related offences (public intoxication, driving under the influence, disorderly conduct).

Indicator	Definition
Costs of drink–driving enforcement	Enforcement, equipment, legal and health-care costs.
Environmental costs	Waste, littering and vandalism associated with alcohol use.
Economic impacts	
Health-care costs	Direct and indirect health-care costs of alcohol-related disease and injury.
Productivity losses	Absenteeism, presenteeism and premature death.
Alcohol production and supply	
Government monopolies on production/ distribution	Exclusive government control over parts of the alcohol supply chain.
Ownership of beverage companies (foreign/local)	Structure of ownership of producers and distributors.
Imported vs locally produced alcohol	Market share of imports and domestic production.
Beverage types available	Beer, wine, spirits, liqueurs, no- and low-alcohol beverages, surrogate alcohol, traditional beverages.
Informal production and supply	Production outside formal licensing and taxation.

1.1.5.3 Policy and legislative context

This section identifies and describes the policy and legal instruments that bear on alcohol-related issues (Table 7). It covers the constitutional framework; international agreements; national and subnational alcohol laws and policies; notable legal cases; and evidence on the effectiveness of those laws and policies. The objective is to map both the formal architecture of alcohol regulation and its observed implementation. Significant gaps between law-as-written and law-as-enforced are themselves a major analytical finding – and a typical entry point for SAFER intervention design.

Table 7. Categories of law and policy to review

Category of law or policy
Constitutional arrangements relevant to alcohol policy.
International agreements to which the country is party (trade, health, human rights, child protection).
NCD policies and strategies with an alcohol component.
Dedicated national and subnational alcohol policies.
Legislation on alcohol availability (licensing, minimum legal age, outlet density, place and hours of sale).
Legislation on alcohol advertising, promotion and sponsorship – including digital and social-media marketing.
Legislation on drink–driving and BAC limits.
Legislation on alcohol taxation and pricing.
Evaluation studies on the effectiveness of alcohol policies.

Category of law or policy
Past or ongoing legal cases on alcohol or alcohol policy.
Policies in related areas (road safety, gender-based violence, child protection, mental health, cancer, NCDs more broadly).

A fill-in template for this analysis is provided in Table A13.1, and Annex 1 gives a worked example.

1.1.5.4 Institutional and governance arrangements

This section maps who decides, who implements and what resources and capacities are available across the alcohol policy architecture, including conflicts, gaps or overlaps between national and subnational authorities (Table 8). Institutional arrangements are often the deepest determinant of whether a SAFER intervention is feasible: a country with a well-funded national licensing authority can move quickly on availability restrictions; a country in which licensing is fragmented across municipalities, with limited inspection capacity, will need to invest in institutional strengthening before the SAFER S-pillar can deliver. Annex 5 provides a detailed key-informant interview checklist for use in this section, and Annex 2 gives an illustrative completed example.

Table 8. Checklist of institutional and governance arrangements, capacities and resources

Checklist for assessing institutional and governance arrangements, capacities and resources; completed with the ministries, departments and agencies concerned.

Category	Components to assess
Multisectoral coordination mechanism	• Existence of coordination mechanisms for alcohol policy (committees, task forces). • Roles and responsibilities of coordinating entities. • Mechanisms for information-sharing between government and non-government actors. • How often, and how effectively, mechanisms operate. • Any formal or informal government interaction with the alcohol industry – to be reviewed against the COI principles in Part 4.
Government bodies with decision-making power	• Ministries/departments responsible for alcohol policy. • Roles in formulating policies and regulations. • Decision-making processes and mechanisms. • Engagement with other sectors (multisectoral approach). • Direct and indirect impact on alcohol policy.
Implementation capacities	• Existence of licensing boards and regulators. • Human resources and expertise within institutions. • Monitoring and surveillance systems for alcohol consumption and harms. • Communication and public-awareness strategies. • Collaboration with nongovernmental organizations (NGOs) and civil society in implementation.

1.1.5.5 Health-system capacity for screening, brief intervention and treatment

Of the five SAFER intervention areas, the F-pillar (access to screening, brief interventions and treatment) is the most directly dependent on existing health-system capacity. Legislative change may sometimes be achieved relatively quickly, whereas building primary-care brief-intervention capacity at scale generally

requires sustained investment over a longer period. The Landscape Report should therefore assess this dimension in some depth, drawing on key-informant interviews with providers, advocacy groups and professional associations as well as administrative data.

Three terms are current in this field and they are not synonyms. Screening and brief intervention (SBI) is a clinical procedure: a validated screening instrument followed by a short structured conversation, delivered in a non-specialist setting. Screening, brief intervention and referral to treatment (SBIRT) adds a specified pathway for those whose screening result indicates probable dependence, for whom brief intervention alone is not the indicated response and for whom effective joint working between general and specialist services is required *(6)*.

The SAFER intervention area is wider than both: it commits governments to facilitate access to screening, to brief interventions and to treatment for alcohol use disorders, the last of these as a service in its own right and not only as a destination for referral.

The distinction is operationally consequential. Screening identifies need, and a screening programme without a referral pathway therefore produces, by design, a group of identified people for whom the only available response is known in advance to be insufficient. That is not a shortfall in coverage but a fault in service design, and it has three practical implications. First, screening capacity should be assessed alongside referral and treatment capacity and scaled in step with it rather than ahead of it; Table 9 is structured accordingly. Second, an economic assessment that costs screening and brief intervention without costing treatment understates what the intervention area requires (Part 3). Third, coverage of screening is not on its own a sufficient indicator, because it can rise while the failure to treat worsens; it should be paired with indicators of referral and treatment coverage (Part 2 and Annex 7).

Alcohol harm per litre consumed is higher in lower socioeconomic populations, which typically also have the least access to treatment (Part 3, section 3.10.3). Screening scaled ahead of treatment capacity can therefore widen the gradient in alcohol harm rather than narrow it, by identifying need disproportionately among the people whom the health system is least equipped to serve.

Table 9. Health-system capacity for screening, brief intervention and treatment

Assessment instrument for health-system capacity for screening, brief intervention and treatment.

Component	Sub-components and sample key-informant questions
Availability and use of alcohol screening	• Are systematic screening guidelines available and adapted locally? • Are validated screening tools (for example, the Alcohol Use Disorders Identification Test, AUDIT) available in primary care, and how are results recorded? • Are screening programmes integrated with community outreach and existing health-care programmes? • Are geographic, financial and cultural barriers to screening identified and addressed?
Brief interventions for alcohol use	• Are brief interventions available in health-care settings? Are they linked with community supports? • Do providers receive pre-service and in-service training on identifying and managing alcohol use? • Is there a standard operating procedure with clear roles? • Are provider knowledge and competencies regularly assessed?

Component	Sub-components and sample key-informant questions
Treatment for alcohol use disorders	• Are treatments for AUD available, and at what levels of care? • What treatment modalities are provided? What referral pathways exist? • Are specialized addiction-treatment centres available and accessible? • Is mental-health care integrated with substance-use treatment? • Are services culturally and socially tailored?
Providers, costs and challenges	• Are provider roles and responsibilities clearly defined? • Are costs assessed? Are services covered by public insurance or out-of-pocket? • How are stigma and resource limitations addressed?

1.1.5.6 Financing dimensions of alcohol policy

This section describes the existing and potential financial and other resources available to support alcohol policy (Table 10). It draws on national health accounts, the WHO Global Health Expenditure Database, government budget data and key-informant interviews with the ministry of finance. The financing analysis here is the foundation for the economic assessment described in Part 3, and for the resource-mobilization work at community level described in Part 5 section 5.8.

Table 10. Financing-dimension indicators for alcohol policy

Data-collection instrument for the financing dimensions of alcohol policy; the foundation for the economic assessment (Part 3).

Indicator	Definition
Health-expenditure trends	
Current health expenditure (CHE) as % of GDP	Share of GDP spent on health services and health-care activities.
CHE per capita	Average health-care expenditure per person per year.
Domestic general government health expenditure (GGHE-D)	Government spending on health from domestic revenue, collected as % of current health expenditure and as % of general government expenditure (fiscal priority given to health).
Domestic private health expenditure	Spending by private individuals, private insurers and other private sources, as % of current health expenditure, with out-of-pocket expenditure reported separately as % of current health expenditure.
Financial resources for alcohol policy	
Government budget allocations for alcohol policy	Budget allocations for alcohol policy activities, programmes and enforcement.
Spending trends on alcohol-related health issues	Allocation of funds across prevention, treatment, research and enforcement.
Funding from other sectors relevant to alcohol policy	Transport, education, law enforcement, social services.
Investment and research on alcohol	Resources allocated to scientific research on alcohol and its harms.

Indicator	Definition
Earmarking and taxes	
Taxes on alcohol products and revenue generated	Excise taxes, sales taxes, import duties, other levies.
Earmarked alcohol taxes for the health system	Share of alcohol-tax revenue allocated to health programmes.
Use of earmarked funds for health promotion	Allocation and expenditure of earmarked funds on alcohol-related and broader health-promotion activities.
International assistance and donor funding	
Programmes for alcohol policy	Resources from international organizations, donors or foreign governments supporting alcohol policy.
Collaboration with international organizations	Partnerships providing technical or financial support.

1.1.5.7 Stakeholder mapping and political drivers

This section maps the actors involved in alcohol policy – government MDAs; CSOs, professional bodies, academia and research institutions; lawmakers and parliamentary committees; and local and international alcohol industry actors. The mapping is the input to two distinct subsequent activities: Joint Assessment planning (Module 2) on the one hand, and the COI assessment that runs through every Part of the handbook on the other (see Part 4). For industry actors, the mapping should describe lobbying activity, partnerships with government, corporate-social-responsibility initiatives and marketing strategies, with explicit attention to social-aspects/public-relations organizations and to non-State actors substantially funded by the alcohol industry. A fill-in template is provided in Table A13.2, and Annex 3 gives a worked example.

1.1.5.8 Potential collaborators in SAFER implementation

This section identifies actors who could be engaged as collaborators in SAFER assessments, technical work and implementation (Table 11). It is essentially a working list for the Joint Assessment and the Roadmap. Annex 4 provides a worked example.

Table 11. Potential collaborators in SAFER assessments and implementation

Reference list of potential collaborators in SAFER assessments and implementation; used when planning engagement.

Sector	Definition	Description of role
Government	Relevant ministries	Ministries directly relevant to alcohol policy (health, finance, trade, transport, interior, education, social protection) and their role in policy development and implementation.
Government	Parliamentary committees	Committees overseeing alcohol policy with the capacity to advocate for SAFER legislation.
International organizations	International organizations	UN entities, NGOs and intergovernmental bodies working in alcohol policy, public health, trade and human rights.
Civil society	Civil-society organizations	National and subnational CSOs active in alcohol policy and public health.

Sector	Definition	Description of role
Health care	Associations and providers	Professional bodies involved in prevention, treatment and harm reduction.
Research	Academia	Academic institutions focused on alcohol, public health, public policy, economics.
Research	Think tanks	Policy-research organizations with relevant portfolios.
Media	Media	News outlets, broadcasters and social-media actors with reach in the alcohol policy space.
Community	Religious organizations and disease associations	Faith-based organizations and associations focused on addiction, cancer, liver disease, violence and other alcohol-related issues.
Funding	Potential donors	Public, private and NGO funders with capacity and interest in SAFER and alcohol policy.

1.1.5.9 Past alcohol policy efforts and challenges

This section describes past alcohol policy efforts, successes and failures; challenges and constraints to adoption and implementation; critical success factors; and the level and nature of conflict of interest and industry interference observed historically. The honest treatment of past failure is unusually valuable in this section: it identifies the institutional patterns the current SAFER cycle will need to break.

1.1.5.10 Limitations on information availability

This section identifies and describes any gaps or limitations in data and information encountered during the review, and sets out possible mitigation measures for future cycles.

1.1.5.11 Recommendations

The analytical work generates specific recommendations on priority actions to strengthen alcohol policy; adoption of each of the five SAFER interventions in the country context; health-system strengthening; institutional strengthening; resource mobilization; opportunities and potential for change; and enabling factors and strategies to facilitate adoption and optimization of SAFER interventions.

1.1.6 Report adoption

The draft Landscape Report is approved by the SAFER Steering Committee (whose composition is described in Module 3) and submitted to the ministry of health for final adoption by government. Senior ministry leadership formally adopts the report to confer high-level ownership and legitimacy on its findings and on subsequent activities. Without formal adoption, the report risks being treated as a consultancy product rather than a government commitment.

1.1.7 Publishing and dissemination

Once adopted, the Landscape Report is published and disseminated to alcohol policy stakeholders and SAFER partners. Dissemination can take place through workshops, seminars, briefing meetings and the production of communication materials. Dissemination is itself an opportunity to consolidate the partnerships forged during the report-writing process; it should be planned and resourced as part of the cycle, not deferred.

1.1.8 Review and updating

The Landscape Report is reviewed and updated progressively during SAFER implementation to ensure that new and emerging information is integrated and that SAFER targeting is continuously refined. A typical refresh cycle is every three years; the Report is also refreshed when a new edition of the WHO *Global status report on alcohol and health and treatment of substance use disorders* is published *(1)*.

1.2 Module 2: Planning and conducting a Joint SAFER Assessment

1.2.1 Introduction

A Joint SAFER Assessment brings together key stakeholders to conduct a nationally led review of the country's alcohol policy situation – a shared understanding that informs the SAFER Roadmap. It brings government ministries, departments and agencies together with subnational authorities, civil society, academia and people affected by alcohol harms, joined, where the country chooses, by international expertise. Where the Landscape Report (Module 1) describes the country context analytically and from a distance, the Joint Assessment tests, deepens and contextualizes that analysis through direct engagement with the people and institutions that will need to implement SAFER. The two instruments are designed to be complementary: a Landscape Report without a Joint Assessment risks producing a document that is technically correct but politically and operationally untested; a Joint Assessment without a Landscape Report risks producing strong relationships but a thin evidence base. The combination produces what neither alone can deliver.

Joint assessments also serve a less obvious but equally important purpose. They are an instrument of inter-ministerial and inter-sectoral alignment. The act of convening a senior assessment team with members from the ministry of health, the ministry of finance, transport, justice, communication, licensing and law enforcement – together with civil society and, where the country invites them, WHO and international partners – is itself a political event. It signals to the country and to its partners that alcohol policy is a whole-of-government concern. That signal is often as valuable as the technical findings of the assessment.

1.2.1.1 Purpose and scope

The Joint SAFER Assessment provides an opportunity for the ministry of health, SAFER partners and local stakeholders to assess the country situation on alcohol policy and the adoption and implementation of alcohol policies, including the SAFER interventions; to understand stakeholder experiences, incentives and capacities; and to identify potential opportunities, entry points and priorities for SAFER implementation. Assessment findings inform the SAFER Roadmap (Module 3), enhance the Landscape Report (Module 1) and feed into the economic assessment that supports the Roadmap (Part 3).

Specifically, a Joint SAFER Assessment aims to:

- inform and enhance the Landscape Report, the SAFER monitoring system and the economic assessment with qualitative and detailed country information;
- facilitate participatory processes to determine priority SAFER interventions tailored to the country context – including epidemiological, political, cultural, social and economic factors;
- advocate and raise awareness within government and externally on alcohol harms, country impact and appropriate policy, legal and administrative measures;
- promote a whole-of-government and whole-of-society response to SAFER – and, where the assessment is conducted jointly with the United Nations system, a whole-of-UN response;

- empower civil society, government and citizens to contribute to the development and implementation of SAFER interventions; and

- where conducted jointly with the United Nations system, help establish a sustainable coordination mechanism for the UN country team to work jointly on alcohol policy. Box 4 sets out the joint-mission model.

Box 4. Joint missions with the United Nations system

Countries that wish to strengthen their links with the United Nations system can conduct the Joint SAFER Assessment as – or combine it with – a joint mission with the United Nations Inter-Agency Task Force on the Prevention and Control of NCDs, WHO and other United Nations agencies. A joint United Nations mission brings the political weight of whole-of-UN engagement; connects alcohol policy to the broader development frameworks through which the United Nations system supports the country, including the United Nations Sustainable Development Cooperation Framework; engages the United Nations country team and the resident coordinator system; and links the country into intercountry learning across the SAFER partnership.

The decision to invite a United Nations mission rests with the government. Earlier SAFER materials, including the WHO progress report on the SAFER initiative, refer to Joint SAFER Missions conducted on this modality; the guidance in this module applies to both formats.

1.2.1.2 Objectives

The specific objectives of a Joint SAFER Assessment are to: assess the political, legal and institutional environment for reducing the harmful use of alcohol, including any need to revise or update the Landscape Report; assess the capacity of primary health-care workers to screen, provide brief interventions and treat alcohol use disorders; assess the case for investing in alcohol policy, including the economic impacts of alcohol and the potential return on investment of policy implementation; evaluate the need for capacity-building efforts to support SAFER implementation and enforcement of alcohol laws and regulations; consult with ministries and agencies, advocacy groups, academia, professional associations, UN agencies and development partners to inform the SAFER Roadmap; and generate media and public awareness on alcohol harms and the potential benefits of SAFER implementation.

1.2.1.3 Principles for conducting a Joint SAFER Assessment

A Joint SAFER Assessment is conducted through partnership and collaboration, led by the ministry of health with other government MDAs and national partners, and joined, where the country chooses, by WHO, UN agencies and international non-State partners. Six principles guide the process:

- **Resourcefulness** – partners bring distinct organizational capabilities to address the diverse aspects of the assessment. The assessment is stronger than any single partner could be alone.

- **Focus** – the assessment is targeted on understanding the country situation on harmful use of alcohol and on the SAFER measures, not on the broader health agenda. A focused assessment delivers findings that can be acted on; a diffuse assessment produces a report that no one knows what to do with.

- **Ownership** – partners actively support national ownership by the ministry of health, other government agencies, UN entities and local partners. The assessment is nationally led by default; where international partners participate, they support and never substitute that leadership.

- **Productivity** – partners ensure the assessment delivers on its purpose, aim and objectives. The assessment is not a meeting; it is a piece of work with deliverables.

- **Flexibility** – the assessment accommodates the country's social, political and institutional dynamics and context. A fixed agenda inflexibly applied is likely to mean that the assessment misses what matters.

- **Consensus-building** – process and decisions are arrived at by consensus where possible. The assessment's recommendations carry weight only if the actors who must implement them have shaped them.

1.2.2 Planning for a Joint SAFER Assessment

A Joint Assessment cannot be improvised. The planning phase typically takes between three and six months and produces a detailed assessment plan, agreed assessment team, completed stakeholder mapping, prepared materials and a managed risk register. The planning steps below should be followed in approximate sequence; in practice they overlap.

1.2.2.1 Determining country readiness

Before a Joint Assessment is scheduled, three preliminary conditions should be met. First, the country has indicated willingness to participate in and lead SAFER implementation and the Joint Assessment – typically documented in correspondence between the ministry of health and WHO. Second, where international participation is sought, an inception meeting has taken place between the ministry of health, WHO, the lead UN agencies and the core international SAFER non-State partners. Third, the social and political environment is conducive – for example, the timing takes account of electoral periods and transitions that may limit participation, ownership or follow-up, and assessments are not scheduled during a national emergency that would limit the availability of government, UN and non-State actors. An assessment scheduled during an unfavourable political or operational window is unlikely to generate actionable findings or sustained government engagement.

1.2.2.2 Risk assessment and mitigation

During the planning phase, a comprehensive risk assessment is essential. Risks fall into two broad categories. Internal risks arise from team coordination, stakeholder engagement, scheduling, language and translation. External risks arise from the political environment, security, logistical disruption, public health emergencies and – in some contexts – coordinated industry counter-engagement. The four-step risk-management process used by SAFER assessments is:

1. **Identify** – enumerate political, institutional, coordination, security and external-shock risks. The assessment's local partners are the most reliable source for the political and security risks; the WHO country office for the operational risks.

2. **Prioritize** – rank risks by likelihood and potential impact, focusing mitigation effort on the most critical.

3. **Develop mitigation** – for each prioritized risk, agree specific actions to prevent, minimize or manage it. Contingency options – including conducting the assessment virtually if travel becomes impossible – should be in place before the assessment opens.

4. **Evaluate mitigation** – review the effectiveness of mitigation during and after the assessment to inform future assessments. Assessment risk registers are an under-used source of institutional learning.

Risk assessment should include potential commercial interference, legal challenges and implementation constraints. Mitigation measures may include transparency protocols, early legal review, documentation of all relevant stakeholder interactions and clear communication strategies for anticipated opposition or misinformation.

1.2.2.3 Communication

The ministry of health and WHO ensure timely and effective communication with all participating international and local partners. Communication covers the partner's role on the assessment, timelines, specific requests to the partner and assessment logistics. A short communication protocol agreed at the outset prevents the late-stage scrambles that have sometimes characterized under-prepared assessments.

1.2.2.4 The assessment team

The assessment is conducted by a team comprising the ministry of health as lead agency; other relevant government MDAs with a role in alcohol policy (typically ministry of finance, revenue authority, communication, licensing, the State Law Office or Attorney-General, and law enforcement); local non-State partners designated by the ministry of health; and, where the country invites international participation, WHO and relevant UN agencies and international SAFER non-State partners. Collectively, the assessment team should cover expertise across all five SAFER intervention areas, plus epidemiology, law and legislative drafting, public policy, research, economics, health-system financing, communication, stakeholder management, project management, law enforcement and advocacy.

The assessment team also benefits from the inclusion of at least one independent expert advisor with international experience of community-level SAFER implementation – a model that has worked well in the Building SAFER Communities project in Ireland and is described in Part 5, section 5.9.

1.2.2.5 Stakeholder mapping

A comprehensive stakeholder mapping exercise – building on the mapping produced for the Landscape Report – identifies the actors to be engaged during the assessment. The template in Table A13.3 may be used.

Note: Engagement of alcohol industry actors during a Joint Assessment is informational only and governed strictly by the conflict-of-interest principles in Part 4; representatives of the alcohol industry have no role in the development of SAFER policy or in the framing of assessment recommendations.

1.2.2.6 Assessment plan

A detailed assessment plan is prepared at the start of the planning phase, normally three to six months before the assessment start date. It describes key activities, tasks and responsibilities (Table A13.4). The plan is a working document; a typical Joint Assessment reissues several iterations of its assessment plan in the planning phase as logistics, stakeholder availability and political timing settle.

1.2.2.7 Materials development

All materials required for the assessment are prepared before commencement. They typically include slide decks, handouts, structured interview guides, briefing notes for team members and participants, the participants list and press releases. Team members are assigned topical areas consistent with their institutional mandates and expertise, and all materials are reviewed by SAFER partners prior to the assessment. Materials should be available in the working language of the assessment and, where appropriate, in the official language(s) of the country.

1.2.3 Conducting a Joint SAFER Assessment

1.2.3.1 Process management

The ministry of health coordinates all assessment activities with support from WHO. The senior leadership of the ministry of health is kept informed throughout preparation and during the assessment; a courtesy call with the Minister or equivalent senior official is held at the start of the assessment; all SAFER partners play an active role; and activities are aligned and harmonized to ensure a smooth and effective process. The assessment protocol – who speaks, in what order, on whose behalf – should be agreed in advance and adhered to. Improvised protocol, in cultural contexts where protocol matters, can derail months of preparation in a single morning.

1.2.3.2 Assessment activities and scheduling

A Joint SAFER Assessment in its concentrated format typically takes place over five working days, excluding, where applicable, the arrival day for international participants. Stakeholders are informed of the schedule at least two months in advance. Activities include workshops, key-informant interviews, field visits and briefing meetings (Table A13.5). Activities may be held concurrently to maximize use of the assessment team's time. Scheduling should allow adequate time per activity, including travel time between sessions; place meetings with subnational governments at the regional level where possible; and accommodate the preferences and constraints of participating stakeholders.

Where the assessment team is wholly national and no international travel is involved, the assessment may instead be distributed over several shorter blocks – for example, two or three blocks of one to two days within a fixed window of four to six weeks, adapting the programme in Table A13.5 across the blocks. A distributed format can ease access to senior officials and allow early findings to inform later sessions, but it dilutes momentum, extends the period during which findings can be contested as out of date, and gives interested actors more time to organize against the process (see Part 4). It should therefore retain the same team throughout, conclude with a single synthesis and debriefing, and be completed within the agreed window. Where a concentrated week is feasible, it remains the recommended format: compression secures senior attention, enables immediate triangulation across stakeholders and gives the assessment a disciplined endpoint.

1.2.4 Reporting on a Joint SAFER Assessment

The assessment report is the principal product of the assessment. It synthesizes findings into a single document that subsequently informs the SAFER Roadmap (Module 3) and the economic assessment (Part 3). The reporting process has five components.

1.2.4.1 Evaluation of the findings

After the assessment, team members collectively analyse findings against the predefined objectives, triangulating insights from workshops, interviews, field visits and documentary review. The triangulation exercise is essential: a single observation from a single source rarely supports a Roadmap recommendation; convergent observations from several sources usually do.

1.2.4.2 Identifying and prioritizing actions

Findings are translated into candidate actions, which are prioritized using agreed criteria (evidence of impact, feasibility, political readiness, cost, time to implementation, alignment with existing national plans). The prioritization conversation is where the most heated disagreements among team members typically occur, and where the consensus-building principle (section 1.2.1.3) is most needed.

1.2.4.3 Structure and content of the report

The assessment report typically covers an executive summary; assessment background, purpose, scope and methodology; an overview of alcohol harms and policy context in the country; findings against each SAFER intervention area; findings on cross-cutting issues (financing, health-system capacity, industry interference); identified priority actions and proposed Roadmap directions; risks and enabling conditions; and annexes (participants list, interview guides, raw observations).

1.2.4.4 Assessment evaluation

A short internal evaluation of the assessment process – timeliness, coordination, stakeholder engagement, risk mitigation – is completed within two weeks of assessment closure to feed into the improvement of future assessments. This evaluation should be honest and circulated, in summary form, to the WHO regional office and SAFER international partners so that lessons accumulate.

1.2.4.5 Assessment-report validation

The draft assessment report is shared with the ministry of health and SAFER partners for review. A validation meeting agrees on findings and priority actions. The final report is formally adopted by the ministry of health and is the principal evidence input into Module 3.

1.3 Module 3: Developing and implementing a SAFER Roadmap

1.3.1 Introduction

A SAFER Roadmap is the country's strategic implementation plan for SAFER. It synthesizes the analytical foundation laid by the Landscape Report (Module 1) and the political and operational findings of the Joint Assessment (Module 2) into a costed, time-bound, owned plan. Where the first two modules diagnose the situation, the Roadmap is the decision: what the country will do, by when, with whom and at what cost.

Two design choices distinguish a useful Roadmap from a document that ends up on a shelf. First, the Roadmap is selective. A typical Landscape Report identifies dozens of possible actions; roadmaps that include too many priorities may dilute resources and accountability. Effective roadmaps focus on two or three SAFER intervention areas chosen for their feasibility, political readiness and impact potential, with overarching actions on monitoring, economic evidence and financing, and protection from interference (see Parts 2, 3 and 4) running across all of them. Second, the Roadmap is operational. Activities are named at a level that someone can be held accountable for; deliverables are dated; costs are estimated; risks are identified; the monitoring plan is integrated from the start. A Roadmap that is too abstract to act on is not a Roadmap.

1.3.1.1 Purpose and scope

The Roadmap provides direction for SAFER implementation, sets out the goal and desired outcomes, and guides SAFER partners on programme priorities. Its scope is typically in-depth implementation of two to three SAFER intervention area priorities over a period of 12–24 months, supported by overarching workstreams on monitoring, financing and protection from industry interference.

1.3.1.2 Objectives

The objectives of a SAFER Roadmap are to align country alcohol policy needs with SAFER interventions; inform strategic prioritization, targeting and sequencing of SAFER implementation; optimize resource allocation; and guide SAFER partners on effective, efficient and comprehensive coordination of implementation.

1.3.2 Process of developing a SAFER Roadmap

On completion of the Landscape and Assessment reports, the SAFER team prepares the Roadmap through a structured six-step process. The steps are sequential but iterative: drafting Step 5, for example, often surfaces issues that send the team back to Steps 3 or 4.

Step 1: Summarize and analyse the Landscape Report recommendations

The Landscape Report provides recommendations based on a broad analysis of the policy, legal and institutional landscape. Each recommendation is mapped to the relevant SAFER intervention area and translated into feasible candidate actions. The mapping is the bridge between the analytical work of Module 1 and the operational planning of Module 3. Table 12 provides the working template.

Table 12. Summary and analysis of Landscape Report recommendations

Analysis matrix for summarizing Landscape Report recommendations and mapping each to a SAFER intervention area.

SAFER intervention area	Recommendations from the Landscape Report	Proposed feasible actions
S – Availability		
A – Drink–driving countermeasures		
F – Screening, brief intervention and treatment		
E – Advertising, sponsorship and promotion		
R – Tax and price measures		
Health-system strengthening		
Institutional strengthening		
Resource mobilization		
Opportunities and potential for change		
Enabling factors and strategies		

Step 2: Summarize and analyse the Joint SAFER Assessment recommendations

The Assessment report adds qualitative analysis of country context, stakeholders, readiness and political dynamics. Its recommendations are mapped onto the same SAFER framework, with explicit attention to areas where Module 1 and Module 2 recommendations agree (high confidence) and where they diverge (further work needed). Table 13 provides the working template.

Table 13. Summary and analysis of Joint Assessment recommendations

Analysis matrix for Joint Assessment recommendations; record where the two evidence sources agree and diverge.

SAFER intervention area	Recommendations from the Assessment report	Proposed feasible actions
S – Availability		
A – Drink–driving countermeasures		
F – Screening, brief intervention and treatment		
E – Advertising, sponsorship and promotion		
R – Tax and price measures		
Health-system strengthening		
Institutional strengthening		
Resource mobilization		
Strategies for engaging diverse stakeholder groups		
Strategies for protecting SAFER from conflict of interest and industry interference		
Strategies for overcoming barriers and leveraging opportunities		
Country readiness on SAFER and way forward on in-country support		

Step 3: Prioritize SAFER interventions and actions

From the combined set of candidate actions, the SAFER team selects two to three SAFER intervention areas to focus on over the 12–24 months of the Roadmap. Prioritization reflects the strength of evidence for the intervention; current political readiness and the proximity of relevant legislative or regulatory windows; institutional capacity to deliver; and projected impact on health, social and economic outcomes. The selected priorities are accompanied by overarching cross-cutting workstreams on monitoring (Part 2), economic evidence and financing (Part 3) and protection from industry interference (Part 4) – these workstreams are core components that run across all selected priorities.

Step 4: Identify activities to support priority interventions

Each SAFER intervention area or action can be delivered through one or more types of activity, each with its own expected outcomes. Table 14 provides the typology used by SAFER country partners, drawn from the United Nations Development Programme's results-based management framework *(7)*.

Table 14. Activity types and expected outcomes

Activity type	Definition	Expected outcomes
Advocacy	Promotion of change in alcohol-related policies or practices through public-awareness campaigns and engagement with decision-makers.	Improved transparency and accountability; adoption of appropriate policy, legislative or administrative decisions; reduced conflict of interest; increased awareness and knowledge; strengthened systems.
Research and data	Generation and analysis of information on alcohol consumption, harms and policy impact to inform evidence-based decisions.	Increased awareness and knowledge; informed and improved decision-making; strengthened systems.
Capacity-building and institutional strengthening	Strengthening the skills and resources of individuals and organizations to deliver alcohol policies.	Increased awareness and knowledge; improved capacity and competency; enhanced organizational performance; improved service delivery; strengthened systems.
Development of policy and legislative instruments	Drafting and adopting bills, regulations, rules, orders, plans, policies and standards for alcohol policy.	Improved legal and regulatory regime; enhanced compliance; strengthened systems; reduced incidence of harmful use of alcohol.
Monitoring and evaluation	Systematic assessment of progress and outcomes of alcohol policy actions to determine success and guide improvement.	Improved programme performance; strengthened systems.

Source: adapted from the results-based management framework of the United Nations Development Programme *(7)*.

Each activity is then matched to a primary delivery purpose. Table 15 provides the working typology.

Table 15. Purpose of SAFER intervention activities

Purpose	Definition
Policy reform	Adjusting or creating policy, regulation and legislation to address alcohol-related issues, production, supply and consumption.
Regulatory reform	Restructuring governance and rules overseeing alcohol industry practices.
Stronger implementation infrastructure	Strengthening systems and resources to enforce alcohol regulations, including coordination, monitoring and reporting mechanisms.
Increased human capacity for implementation	Strengthening the skills and supply of the workforce to deliver alcohol policies.

Step 5: Draft the Roadmap using the standard structure

Table 16 summarizes the structure and content of a SAFER Roadmap. Annex 6 provides an illustrative extract from a completed Roadmap matrix. The structure is a starting point and may be adapted to country context and to the findings of the Landscape and Assessment reports. What should not be removed is the explicit linkage to Parts 2, 3 and 4: a Roadmap without an integrated monitoring, evaluation and learning plan, an economic assessment supporting its budget and a conflict-of-interest framework is incomplete.

Table 16. Structure of a SAFER Roadmap

Section	Content
Introduction	Introduces the Roadmap and the link to the Landscape and Assessment reports.
Purpose and scope	States the Roadmap's purpose and scope; adapts the text from this module as necessary.
Goal	Defines the goal of SAFER in the country, aligned to the selected priority intervention areas.
Objectives	Specific objectives to be achieved within 12–24 months, linked directly to the selected priorities.
Summary of Landscape and Assessment recommendations	Consolidates the outputs of Step 1 and Step 2 (Tables 12 and 13).
Priority SAFER interventions, actions and activities	Describes the selected two to three SAFER intervention areas and their activities, with expected outcomes.
Stakeholder mapping and political drivers	Summarizes stakeholder analysis from the Landscape and Assessment reports (Table A13.6).
Analysis of barriers, facilitators and opportunities	Summarizes the factors that will help or hinder SAFER implementation (Table 17).
Matrix for implementation of SAFER interventions	A detailed matrix that operationalizes each objective; see Table 2 for the menu of SAFER interventions.
Monitoring, evaluation and learning	Links the Roadmap to the monitoring and evaluation framework in Part 2 of this handbook; defines the Roadmap-specific indicator set.
Economic evidence and budget	Summarizes the Roadmap budget and links to the economic assessment in Part 3, or the investment case where one is prepared.
Protection from conflict of interest	Identifies the COI risks attached to each priority and links to the principles, steps and tools in Part 4.
Community-level translation	Identifies how Roadmap activities will be translated to provincial, district, municipal and submunicipal level – the working list for Part 5.
Risks and assumptions	Key risks and mitigation; assumptions underpinning the plan.

Table 17. Barriers, facilitators and opportunities to strengthen SAFER implementation

Analysis matrix for barriers, facilitators and opportunities to strengthen SAFER implementation.

SAFER intervention area	Barriers	Facilitators	Opportunities
S – Availability			
A – Drink–driving countermeasures			
F – Screening, brief intervention and treatment			
E – Advertising, sponsorship and promotion			
R – Tax and price measures			

Step 6: Review, validation and adoption of the SAFER Roadmap

The SAFER partners review and validate the Roadmap, build consensus on its commitments and adopt it as the guiding framework. The ministry of health formally adopts the Roadmap as its action plan for SAFER implementation; senior leadership endorses the document on behalf of government. Adoption is the moment at which the Roadmap moves from a planning document to a live commitment, and at which the responsibility for delivery transfers from the Technical Team to the implementing line ministries and partners.

1.3.3 Implementing a SAFER Roadmap

Implementation is the test of every preceding step. A Roadmap that is technically excellent but operationally unsupported delivers little; a Roadmap that is technically modest but operationally well-supported can move outcomes substantially. Effective implementation rests on five components.

1.3.3.1 Implementation coordinating structure

SAFER implementation requires an inclusive, whole-of-government and multidisciplinary coordination mechanism. SAFER partners establish a SAFER Steering Committee as the SAFER Secretariat, comprising approximately 13–15 high-level representatives from the ministry of health; relevant government MDAs (transport, licensing, finance, revenue, advertising regulator, communication); global SAFER partners and UN agencies; civil-society and faith-based organizations; people with lived experience of alcohol harm; academia and research institutions; media; professional bodies; and professionals and experts in each SAFER intervention area, as well as in epidemiology, law, public policy, research, economics, communication, stakeholder management, project management, law enforcement and advocacy. The ministry of health leads the Steering Committee with support from WHO and other global SAFER partners. Sector Working Groups are established as needed.

1.3.3.2 Project management

A project manager and a project-management team – technical officers and consultants – support the Steering Committee. They manage day-to-day operations: project communication, delivery of milestones, convening meetings and liaison with international partners. The project manager should be a national, resident in the country, with deep knowledge and experience of alcohol policy, advocacy and institutional coordination. A recurring lesson across countries that have implemented alcohol policies is the importance of a named, senior, well-supported project manager *(8)*.

1.3.3.3 Resource mobilization and management

The Steering Committee mobilizes financial resources through partnerships with funders, national and international fundraising and allocations from partner budgets. International SAFER partners lead on international fundraising strategy. The economic assessment and, where one is prepared, the investment case (Part 3) support resource mobilization at national level; the parallel community-level resource-mobilization strategies are described in Part 5, section 5.8.

1.3.3.4 Budgeting

The Steering Committee prepares and oversees a Roadmap budget that consolidates partner contributions and projected external funding. The ministry of health and other represented government entities are encouraged to align ministerial annual plans and budgets with the Roadmap. A budget that exists only on paper, without ministerial annual-plan integration, is unlikely to deliver.

1.3.3.5 Execution

The Steering Committee ensures implementation as planned and adopts mitigation strategies where implementation is at risk. Implementation is monitored and evaluated in line with Part 2 of this handbook; the Roadmap is reviewed every six months, with a comprehensive mid-term review at the midpoint and a summative end-of-term evaluation at the end of the Roadmap period.

1.3.4 Reporting

The Steering Committee oversees regular reports on Roadmap implementation. The project manager and project-management team prepare reports on the cadence determined by the Steering Committee (monthly, quarterly, six-monthly or annual). Reports comprehensively cover progress, achievements, challenges and emerging issues; they feed into national reporting against the Global alcohol action plan, the WHO *Global status report on alcohol and health*, the WHO Global Monitoring Framework for NCDs and SDG target 3.5 *(1, 5, 9, 10)*.

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PART 2

Monitoring, evaluation and learning for SAFER implementation

PART 2

Monitoring, evaluation and learning for SAFER implementation

2.1 Introduction

As partners implement SAFER, it is essential to monitor implementation, evaluate the programme, and identify and integrate lessons. Continuous observation, reflective review and timely corrective action are core to successful SAFER implementation. They are also the activities most often under-resourced and under-prioritized in early SAFER cycles. The most successful country programmes are those that build monitoring, evaluation and learning into the Roadmap from the outset rather than treating them as an end-of-cycle add-on.

Part 2 provides the architecture for that work at national level. It is designed to be applied alongside Part 1 and to feed into Parts 3 and 4. The M&E framework should be drafted in parallel with the Roadmap, not afterwards. The economic assessment is built on the same evidence base as the M&E system, and the protection of policy from interference depends on transparent monitoring of decisions and stakeholder relationships. The community-level companion to Part 2 is Part 5, section 5.5, which anchors community-level monitoring on the WHO *City-level monitoring guidance for the prevention and control of noncommunicable diseases and injuries* (2023) and its 2024 assessment instrument *(1–3)*. Together, Parts 2 and 5 cover monitoring at every level from a single licensed premises to a country submission to GISAH.

2.2 Key definitions

Full definitions are in the Glossary at the front of this handbook. For working reference within Part 2:

- **Evaluation** – systematic and objective assessment of a planned, ongoing or completed intervention, its design, implementation and results, against the criteria of relevance, coherence, effectiveness, efficiency, impact and sustainability *(4, 5)*.
- **Monitoring** – continuous, systematic collection of data on specified indicators to track implementation progress, results and use of resources *(5)*.
- **Impact** – higher-level long-term effects of an intervention's outcomes, positive or negative, intended or unintended *(5)*.
- **Indicator** – quantitative or qualitative variable providing simple, verifiable means to track change and performance *(5)*.
- **Inputs** – financial, human, material and institutional resources used for an intervention.
- **Outcomes** – short- and medium-term effects of an intervention's outputs.
- **Outputs** – products, goods and services produced by an intervention's activities.

2.3 International monitoring frameworks

Country-level SAFER monitoring should be aligned with the international frameworks that already track alcohol-related harm and related health and development outcomes. Alignment reduces the country's reporting burden, supports international comparability and ensures that SAFER work is reflected in global reporting cycles. Seven frameworks are particularly relevant; Table 18 summarizes what each provides.

Table 18. International monitoring frameworks relevant to SAFER

Framework	What it tracks and how it connects to SAFER
WHO Global alcohol action plan 2022–2030 – indicators and milestones	Global and national targets and indicators directly aligned to the six action areas of the Global alcohol action plan, including at least a 20% relative reduction in harmful use of alcohol by 2030 (compared with 2010); reduction in alcohol per capita consumption; coverage of treatment for alcohol use disorders; and protection of populations through implementation of high-impact policies *(6)*.
WHO Global Monitoring Framework for NCDs	Voluntary global targets and indicators, now applied to the NCD action plan as extended to 2030 (2010 baseline), including a relative reduction in harmful use of alcohol (the original target of 10% by 2025 is, for alcohol, superseded by the Global alcohol action plan target of at least 20% by 2030), alcohol per capita consumption (APC) and heavy episodic drinking *(7)*.
UN Sustainable Development Goals – target 3.5	Two official indicators: (3.5.1) coverage of treatment interventions for substance use disorders; (3.5.2) harmful use of alcohol, defined as APC in litres of pure alcohol among persons aged 15 and older *(8)*.
WHO Global status report on alcohol and health and treatment of substance use disorders	Published periodically (latest edition: 2024), presenting country-level data on consumption, harms, policy responses and treatment capacity *(9)*.
WHO Global Information System on Alcohol and Health (GISAH)	WHO's online repository of country-level data on alcohol consumption, harms and policy responses. National monitoring outputs feed into and draw from GISAH *(1)*.
WHO NCD Progress Monitor	Country scorecard on implementation of the NCD best buys, including alcohol measures *(7, 10)*.
WHO City-level monitoring guidance for the prevention and control of noncommunicable diseases and injuries (2023) and assessment instrument (2024)	Standardized set of city-level indicators across eight policy areas including alcohol control. The operational scaffold for subnational monitoring; see Part 5, section 5.5 *(2, 3)*.

Source: developed for this handbook, drawing on the Global alcohol action plan, GISAH, the Global Monitoring Framework for NCDs, SDG indicator metadata and WHO city-level monitoring guidance.

2.4 Purpose of SAFER monitoring, evaluation and learning

The purpose of monitoring, evaluation and learning is to enable SAFER partners to observe and learn continuously from the implementation of the SAFER Roadmap and to make informed decisions to improve that implementation. Because the Roadmap is implemented over a period of 12–24 months, partners need to monitor progress to ensure the Roadmap is being delivered as planned, and to take mitigation measures where obstacles appear. Partners also need to evaluate results and capture lessons. Those lessons inform the next SAFER cycle and broader alcohol policy.

2.4.1 Objectives

The overall objective of SAFER monitoring, evaluation and learning is to ensure the SAFER implementation objectives are realized. Specific objectives are to assess the progress of Roadmap implementation; improve SAFER implementation through evidence-based decision-making; enhance resource allocation for effective SAFER implementation; assess performance and results; draw lessons applicable during SAFER or subsequent alcohol policy efforts; and enable international comparison and alignment with the Global alcohol action plan, NCD and SDG frameworks.

2.4.2 Key focus

Monitoring, evaluation and learning should answer the questions: Is SAFER implementation achieving the desired results? Is SAFER achieving results efficiently and effectively? Are SAFER partners doing things right? Could SAFER partners do things better? Are SAFER partners doing the right things? What lessons are SAFER partners learning from the process? *(5)*

2.5 SAFER implementation results framework

SAFER partners adopt a results-based approach in which planned change is framed along a results chain from inputs and activities through outputs, outcomes and ultimately impact *(5)*. The results framework describes the results to be monitored and evaluated, the key indicators, the baseline, the targets, the means of verification, and the risks and assumptions (Table 19). An illustrative completed framework is provided in Annex 7. Annex 8 of this handbook provides a community-level template that can be aggregated into the national results framework, and the WHO City-level monitoring guidance referenced in Part 5 provides the operational subnational scaffold *(2)*.

Table 19. SAFER key results framework

Fillable results framework for the monitoring plan: indicators, baselines, targets, means of verification, risks and assumptions.

Results level	Indicators	Baseline	Targets	Means of verification	Risks and assumptions
Impact					
Outcomes					
Outputs					
Activities					

Results-based management – the results chain

Inputs ➜ Activities ➜ Outputs ➜ Outcomes ➜ Impact

Results-based management enables SAFER partners to plan, monitor and evaluate against a clear chain of cause and effect. Inputs (resources) are used in activities (the SAFER intervention actions), which produce outputs (deliverables and immediate products), which lead to outcomes (short- and medium-term changes) and ultimately impact (reduced harmful use of alcohol and the consequent social, health and economic gains).

Source: adapted from the United Nations Development Programme handbook on planning, monitoring and evaluating for development results *(5)*.

2.5.1 Minimum indicator set for low-data settings

Countries with limited monitoring capacity should begin with a small set of indicators that can be collected reliably and used for decision-making. The minimum set should be aligned with the *Global alcohol action plan 2022–2030*, GISAH, SDG target 3.5 and the NCD monitoring framework, while remaining feasible for national institutions to maintain.

A minimum SAFER monitoring set may include: adult alcohol per capita consumption where available; heavy episodic drinking where available; alcohol-attributable mortality or proxy harm indicators; implementation status of the five SAFER intervention areas; enforcement activity for availability, drink–driving and marketing measures; coverage of SBI; existence and use of conflict-of-interest safeguards; and progress against Roadmap outputs and budget execution.

2.6 Planning and implementation of SAFER monitoring and evaluation

With the results framework in place, the SAFER team develops a monitoring and evaluation (M&E) plan. The M&E plan describes what is being monitored and evaluated; the M&E activities to be carried out; shared responsibilities across partners; the timeline; the methods to be used; and the resources needed *(5)*.

2.6.1 Organizational structure

The SAFER Leadership and Coordination Team (Module 1) coordinates the M&E process. It includes representatives of the ministry of health; the WHO country office; WHO headquarters; UN partners; SAFER international non-State partners; and SAFER local partners. The team may also include the national statistics agency and the national M&E authority. The ministry of health leads.

2.7 Monitoring process

The monitoring process entails: identifying and engaging the ministries, departments, agencies and stakeholders responsible for the data relevant to SAFER indicators; agreeing how data will be captured, collected, stored and shared; determining data sources and methods of collection; scheduling the frequency of data collection (at a minimum every three months, but indicator-specific); analysing data to assess progress against the results framework; and identifying trends, emerging issues and lessons to adapt implementation. The monitoring plan in Table A13.7 is the working template.

A monitoring report should cover scope (period covered); indicators monitored and targets for the reporting period; outputs achieved during the implementation period, with progress for each indicator; variance between targets and actual progress; challenges experienced; observed gaps in implementation; emerging issues; lessons learned; and recommendations.

2.7.1 Data governance and ethics

SAFER monitoring should be conducted in accordance with national data-protection requirements and ethical standards. Data-sharing agreements should define the purpose of data use, permitted users, storage arrangements, confidentiality protections, retention periods and reporting formats.

Small-area data should be aggregated where necessary to prevent identification or stigmatization of individuals, families or neighbourhoods. Surveys and qualitative research involving people with lived experience of alcohol harm should include appropriate consent, safeguarding, referral arrangements and review by relevant ethics bodies where required.

2.8 Evaluation process

The SAFER evaluation process has three components. An annual review of implementation, which – because the Roadmap is typically implemented over 12–24 months – can also function as the mid-term review. A summative end-of-term evaluation that assesses the OECD/DAC criteria *(11)*. And the engagement of key stakeholders who have been involved in implementation or who have relevant information about its impact.

The OECD/DAC evaluation criteria are:

- **Relevance** – is the SAFER intervention doing the right things?
- **Coherence** – how well does the SAFER intervention fit internally and with other government priorities and policies?
- **Effectiveness** – is the SAFER intervention achieving its objectives?
- **Efficiency** – how well are SAFER resources being used?
- **Impact** – what difference does the SAFER intervention make?
- **Sustainability** – will the SAFER intervention benefits last?

The evaluation report typically follows the structure: purpose and scope; evaluation questions, methodology and limitations; findings and evidence; conclusions; recommendations; and lessons learned *(12)*.

2.9 Learning from SAFER monitoring and evaluation

The overarching purpose of monitoring and evaluation is to ensure SAFER interventions achieve the intended results, to draw key lessons and to generate knowledge that advances alcohol policy. The results and lessons of SAFER implementation strengthen the country's position and capacity for alcohol policy; enable accountability on the use of financial and human resources; facilitate identification of advocacy opportunities; create opportunities for resource mobilization and planning; support framing and reframing of alcohol policy in country discourse; inform further and future programming; and enable scale-up and replication of SAFER interventions in the country and elsewhere *(5)*.

SAFER partners should create space and opportunities to integrate learning into the SAFER cycle; make decisions to improve SAFER implementation on the basis of the evidence generated; use lessons learned to scale up SAFER processes; share knowledge with broader national stakeholders; and promote and support internalization of knowledge by government MDAs.

2.10 Dissemination of SAFER monitoring and evaluation findings

SAFER partners disseminate findings and lessons to diverse stakeholders inside and outside the country, using publications (policy briefs, briefing reports, news bulletins); the ministry of health website and other online platforms; workshops, webinars and conferences; media briefings and engagement; high-level policy briefings to senior policy-makers, ministers, parliamentarians and other government officials; the WHO website and global knowledge products; academic-journal publications; conference papers; and training workshops *(5)*.

Findings should also be fed into international platforms – UNIATF, the WHO Global Coordination Mechanism on the Prevention and Control of NCDs and the WHO regional networks – so that country experience contributes to global learning.

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PART 3

Economic evidence and sustainable financing for alcohol policy

PART 3

Economic evidence and sustainable financing for alcohol policy

3.1 Introduction

Effective alcohol policy requires informed decision-making and sustained investment by government, partners and society. The high-impact SAFER interventions require financial and human resources and political commitment. In any public-management environment, governments routinely face competing demands – alcohol policy must compete for attention and resources with health, education, security, transport, agriculture, social welfare and economic-development priorities. Without robust economic evidence on the impact, affordability, value for money and feasibility of alcohol policy measures, the SAFER agenda may lose out to competing priorities even where the evidence supports action.

Economic evidence can help answer a range of policy and financing questions related to alcohol policy. These may include understanding the burden and costs of alcohol-related harm, estimating the expected health and social impacts of proposed interventions, assessing affordability and budget implications, evaluating value for money, analysing distributional consequences, examining fiscal effects and revenues, or identifying priorities within constrained budgets. Different policy questions require different analytical approaches, and no single method is appropriate in all circumstances.

Economic assessments can also help demonstrate how alcohol policy contributes to broader national objectives, including the prevention of NCDs and injuries, protection of children from harmful marketing, health-system resilience and the reduction of avoidable social and economic losses and, in the case of pricing measures, to increased revenues and fiscal space for health.

It is therefore important that the ministry of health, other government agencies and non-State actors develop and communicate robust economic and policy evidence to support decisions on alcohol policy and on its financing. The starting point for any analysis should be the decision that needs to be informed and the audience for whom the evidence is intended. The choice of analytical approach should follow from these considerations.

Part 3 starts from the policy or financing decision to be informed and describes the economic assessments that can inform it, including how a country prepares an investment case where that is the appropriate vehicle. It is designed to be used alongside the Landscape Report (Module 1, which provides much of the evidence base), the Joint Assessment report (Module 2, which provides the political and operational context) and the Roadmap (Module 3, which the analysis is intended to support). At community level, the parallel resource-mobilization architecture is described in Part 5, section 5.8; communities preparing community-level investment cases follow the same approach described here on a smaller scale.

3.2 Choosing the appropriate economic assessment approach

Economic evidence can support a wide range of alcohol policy and financing decisions. These decisions may include whether to adopt or strengthen specific SAFER interventions, how to allocate resources within a fixed budget, how to finance implementation and enforcement, whether a proposed tax measure is justified, or how to demonstrate the broader health, social and economic consequences of alcohol-related harm.

Common economic assessment approaches in alcohol policy include:

- cost-of-illness analysis, which describes the economic burden of alcohol-related harm;
- intervention costing, which estimates the resources required to implement, enforce and monitor policy actions;
- budget-impact analysis, which shows the implications for government budgets over the budget period;
- fiscal analysis, which estimates the revenue effects of tax and price measures;
- health-impact modelling and cost-effectiveness analysis, which estimate the health gains from interventions and compare interventions by cost per unit of health gain;
- benefit–cost or return-on-investment analysis, which compares the monetized benefits of a package of interventions with its costs (section 3.8); and
- priority-setting analysis, which supports choices between interventions within a fixed budget envelope.

An investment case is not a separate method. It is a decision-support product that synthesizes and presents the results of one or more of these analyses for a defined audience and decision, and it is the product most frequently requested from WHO by countries. The remainder of this Part describes the process and reporting standards that apply to any of these analyses, and the structure of an investment case where that is the chosen product.

An investment case presents an argument for investing in a particular health area or intervention, and offers an analysis of the value for money – and value for resources – of investment in a range of interventions to meet defined goals *(1)*. It is a compelling argument to potential investors (government, partners, donors, private sector) that sets out the impact, benefits and returns of targeted investment in a specific set of priorities in a given context.

An alcohol policy investment case:

- presents the value proposition for investing in high-impact alcohol policy interventions (typically the SAFER package);
- articulates the need, costs, benefits and returns from investment;
- informs policy-makers, government officials and stakeholders on the rationale for strategic investment; and

- supports policy dialogue and, where appropriate, advocacy by the ministry of health and partners within and beyond government, including with development partners, non-State actors and the private sector.

It is not, and should not be, a partnership instrument with the alcohol industry – the principles in Part 4 apply to its preparation as they do to all SAFER work.

An alcohol policy investment case should be more than a technical costing exercise. It should combine a quantified economic analysis of expected costs and impacts along with a policy and institutional analysis that helps decision-makers understand why alcohol policy matters, which interventions are most relevant, what resources are required, what benefits can reasonably be expected, and what conditions are needed for implementation. The analysis should be explicit about the policy question being answered, the time horizon, the comparator or "business-as-usual" scenario, and the perspective used for valuing costs and benefits. Benefits should be defined in advance and expressed in natural units (deaths, injuries and cases of disease averted) and, where the decision requires it, in monetary terms with the valuation method stated; the burden of alcohol-related harm may be described through a cost-of-illness analysis, which is distinct from the evaluation of interventions. The time horizon should be chosen with the audience and the decision in mind: effects on acute harms and on revenue appear early, whereas effects on chronic disease, and the lifelong consequences of harm to children and young people, accrue over decades (see section 3.8.4).

Investment cases are especially useful when they connect public health, public finance and sustainable development. The Sri Lanka investment case, for example, framed alcohol policy as a health and development investment and presented annual economic losses, the cost of implementing a package of five policy actions, lives saved, economic losses averted and return on investment over both five-year and 20-year horizons. The Estonia return-on-investment (ROI) analysis used a narrower one-year horizon and focused on direct health-care costs, productivity losses from premature mortality and, for taxation scenarios, government revenue effects. These examples illustrate that the design of an investment case should be fitted to the decision problem, data availability and political window.

Investment case methods from other public health areas reinforce several principles that are also relevant to alcohol policy:

- costs and benefits should be defined in advance;
- the time horizon should be explicit;
- health, economic and social returns should not be conflated;
- data gaps should be documented; and
- the institutional and political context should be analysed alongside the economic model.

These principles are more important than any single modelling platform.

3.3 Purpose and scope

The underlying purpose of the work described in this Part is to support policy and financing decisions on alcohol policy through appropriate economic assessment, and thereby the resource mobilization required to implement the SAFER Roadmap. Where an investment case is prepared, its purpose is to

articulate the value proposition for investing in alcohol policy. Value is expressed in natural units first (for example, reduced mortality, injuries averted and improved well-being), in monetary terms where the decision requires it, and in qualitative terms where quantification is not possible; monetization is not a required endpoint. The scope is the analysis of high-impact alcohol policy interventions, the associated costs and the value, benefits, outcomes and returns from investment. The scope typically covers the five SAFER intervention areas; some countries also include cross-cutting actions on monitoring (Part 2) and on protection of policy from industry interference (Part 4).

3.4 Defining the policy question and the objectives of the analysis

Before committing resources, the ministry of health and its partners should establish the specific rationale and purpose of the exercise. The team should set out, in writing:

- the policy and practice questions the analysis will answer, for example whether to adopt or strengthen particular SAFER interventions, how implementation and enforcement will be financed, or whether a specific tax proposal is justified;
- the decision-making processes the investment case is intended to inform, such as a cabinet submission, a budget negotiation, a national strategy process or a legislative timetable, and when those decisions will be taken; and
- the follow-up actions, responsibilities and mandates through which its recommendations would be implemented.

These questions determine the appropriate methods, and an investment case is not the right instrument for every one of them. Where the aim is to secure additional resources for a package of interventions, a benefit–cost or return-on-investment analysis presented as an investment case is often the most useful product. Where the budget envelope is fixed, priority-setting analysis, budget-impact analysis, comparative cost-effectiveness analysis or efficiency analysis may be more relevant to decision-makers in the ministries of health and finance. The target audience and the decision to be informed should be defined at the outset. An investment case is not a necessary step for every country. Where the relevant decisions have already been taken, where the data are insufficient to support a credible analysis, or where no realistic decision-making window exists, resources may be better directed to implementation, monitoring or data strengthening. The instrument should be used where it is genuinely needed and where its specific benefit to the country can be articulated in advance.

Where an investment case is the chosen product, its objectives are typically to:

- identify the social, economic and health need for investment in evidence-based, high-impact alcohol policy interventions;
- analyse the costs of investing in those interventions;
- demonstrate the value, benefits and returns to be realized, social, economic and health;
- guide the ministry of health, other government agencies and partners in championing strategic investment in alcohol policy; and
- promote the prioritization of alcohol policy as a national development agenda.

The analysis should also state the perspective it adopts and apply it consistently. A societal perspective counts costs and benefits wherever they fall, including those borne by the person drinking and by their household. A government or fiscal perspective counts only flows through the public accounts; a health-system perspective counts only those falling on health services. The perspective determines which costs may legitimately be counted, and it is often the first point challenged when an assessment is scrutinized. Where the audience is a ministry of finance, it is usually worth presenting the fiscal perspective alongside the societal one, so that the decision-maker can see both the full burden of alcohol-related harm and the narrower question of what the public purse pays and receives.

3.5 Link to the global policy framework

The economic assessments described in this Part, and an alcohol policy investment case where one is prepared, directly support implementation of the WHO Global strategy to reduce the harmful use of alcohol *(2)*, the Global alcohol action plan 2022–2030 *(3)*, SDG target 3.5 *(4)* and the WHO Global action plan for the prevention and control of noncommunicable diseases 2013–2020 (extended to 2030) *(5)*. The analysis should explicitly reference these frameworks and show how national investment contributes to their global targets. That linkage is also a practical advocacy device: it allows Ministers to present alcohol investment as a contribution to commitments their government has already made internationally rather than as a new and contested commitment.

3.6 Process of undertaking economic assessment to support alcohol policy decisions

3.6.1 Overview

Development follows six phases: securing government buy-in; establishing leadership and coordination; preparatory work; writing; review and validation; and adoption. Each is summarized below. The six phases apply to any economic assessment product, whether a full investment case, a budget-impact or fiscal analysis prepared for a budget submission, or a short policy brief; the depth of each phase should be proportionate to the product and to the decision it is intended to inform.

The process should begin by agreeing the decision problem. The Technical Team should specify whether the purpose is to support a national alcohol strategy, compare a limited number of policy options, inform a budget or cabinet submission, support a tax proposal, mobilize partner resources, or strengthen implementation of the SAFER Roadmap. This determines not only the level of detail needed but also the choice of economic assessment method: costing, budget-impact analysis, fiscal analysis, cost-effectiveness analysis or benefit–cost analysis, alone or in combination, rather than a standard set of analytical components.

Depending on the decision problem, an investment case may include:

- a burden analysis;
- a description of the current policy and institutional context;
- a clearly defined package of policy or service interventions;
- costing of implementation, enforcement, monitoring and evaluation;
- estimation of health and economic benefits;

- a cost-effectiveness, return-on-investment or benefit–cost analysis;
- a budget-impact or fiscal analysis;
- sensitivity analyses; and
- recommendations that distinguish technical evidence from political and implementation choices.

Not all of these components are required in every case: some countries primarily need an affordability or fiscal analysis, others a comparative cost-effectiveness or implementation assessment.

The institutional and context analysis is an essential component. It should identify governance arrangements, responsible institutions, financing mechanisms, likely supporters and opponents, implementation bottlenecks, data constraints and opportunities to expand political space. This is particularly important for alcohol policy because effective measures may face commercial opposition or require action by finance, transport, education, justice, trade, local government and enforcement authorities.

Analytical tools may be used where they are validated, available and appropriate to the policy question. However, the analysis should not rely on the name of a tool as a substitute for transparent methods. Where the status, scope or suitability of a tool is uncertain, the report should describe the equations, assumptions, input data, intervention effects and sensitivity analyses used, and should make clear which results are measured, modelled or assumed. In practice, WHO/UNDP investment cases for noncommunicable diseases, including the Sri Lanka alcohol policy investment case, have generally applied the OneHealth Tool for intervention costing and health-impact modelling *(6, 7)*. Countries considering the same route should confirm with WHO the current status, maintenance and suitability of the tool for the specific alcohol policy analysis before relying on it.

3.6.2 Government buy-in and commitment

High-level buy-in – usually from the senior leadership of the ministry of health and the national treasury – is essential. Both are target audiences of the analysis, together with the other budget holders whose action the interventions require. The intended audience and the decision to be informed should be defined at the outset: an investment case is usually most useful when additional resources are sought, and less so when the budget envelope is fixed (section 3.4). Without it, the analysis ends as a technical exercise rather than a budget commitment. Partners prepare a concept note and secure early engagement before committing resources to the analytical work.

3.6.3 Leadership and coordination

The SAFER Leadership and Coordination Team (Module 1) leads the process. A dedicated Technical Team – including health economists, public-finance specialists, alcohol policy experts, analysts and programme staff – undertakes the analytical work. International partners (WHO, UNDP and where present the UN Inter-Agency Task Force on NCDs) often provide technical and methodological support, including the WHO/UNDP investment case methodology adapted to alcohol *(6, 8)*.

3.6.4 Preparatory work

Preparatory work includes a review of existing evidence and analyses on alcohol harms, costs and policy effectiveness in the country, including any national NCD investment case, cost-of-illness study, tax analysis by the ministry of finance or WHO-CHOICE cost-effectiveness estimates that already exist; a mapping of available administrative data and of critical data gaps; scoping of cost-effectiveness or other modelling work (national or international) to adapt; and stakeholder scoping to identify key informants

and validation partners. The analysis should build on and extend existing evidence wherever possible rather than commissioning a new economic evaluation by default.

3.6.5 Writing

The Technical Team writes the analytical product in consultation with the Leadership and Coordination Team, using the structure in section 3.7 where an investment case is prepared.

3.6.6 Review and validation

A draft is reviewed by the Leadership and Coordination Team and key partners. The final draft is validated by the ministry of health, the national treasury, other key government MDAs and partners (UN agencies, non-State actors, academia).

3.6.7 Adoption

The Leadership and Coordination Team presents the final product (investment case or other analysis) to the senior leadership of the ministry of health for formal adoption. Once adopted, it becomes a basis for policy dialogue and decision support within government and with external partners, and for advocacy where appropriate.

3.7 Communicating economic evidence and the structure of an investment case

Economic evidence informs a decision only if it reaches the decision-maker in a form that fits the decision: a cabinet memorandum, a budget submission to the ministry of finance, a policy brief for parliament or for the national coordination mechanism, a technical report, or a full investment case. The format should be agreed with the intended audience at the outset (section 3.4). Whatever the format, the reporting standards below apply, and results should be presented in the units and over the time horizon that the audience uses to decide (section 3.8.4). Table 20 gives the structure of the fullest of these formats, the investment case.

Where an investment case is prepared, it should specify the minimum methodological elements: the intervention package assessed; the comparator or business-as-usual scenario; time horizon; discount rate; perspective; data sources; population covered; costing method; valuation of health gains; valuation of productivity gains; whether government revenue is included; sensitivity analyses; and the extent to which results are based on national data, modelled estimates or international assumptions. Where a benefit–cost analysis is undertaken, the reference case guidelines for benefit-cost analysis in global health and development provide standard methods for several of these elements, including the valuation of mortality and morbidity risk reductions, discounting, the assessment of distributional impacts, the treatment of uncertainty and the presentation of results *(9)*.

The report should identify which type of analysis each result comes from (section 3.2) and should not conflate them: a cost-of-illness estimate is not a measure of the benefit of an intervention, and a return on investment that includes tax revenue is not comparable with one that does not. Methods and scope, including the time horizon and the effects captured, are selected according to the audience, the decision question and the context; there is no single investment case approach that applies across all settings.

The structure in Table 20 is indicative and applies where a country chooses to prepare an investment case; the Leadership and Coordination Team adapts it to local context, to the decision problem and to available data, and omits elements that the decision does not require. The structure is aligned with common WHO/UNDP investment case practice *(6, 8)*.

Table 20. Structure of an alcohol policy investment case

Section	Content
1. Introduction	Analytical background on the need for and justification of an alcohol policy investment case in the country. Draws partly from the Landscape Report (Module 1).
2. Country context	Core social and economic indicators: governance structure; demographics; economic factors (GDP, GDP per capita, income level, poverty, inequality, employment, core economic sectors); social factors (education, universal health coverage).
3. Alcohol consumption status and patterns	Current status of consumption: alcohol per capita consumption (APC), prevalence of heavy episodic drinking, age-specific patterns, demographic, social and geographic characteristics, household expenditure, patterns by beverage type, unrecorded/informal/illicit consumption.
4. Alcohol marketing status, patterns and trends	Current status and trends in alcohol marketing across all strategies and channels, with particular attention to digital marketing and to marketing targeting children and reaching children's settings.
5. Health, social and economic impact of alcohol consumption and marketing	The core evidence base for the case: health impact (road traffic injuries; alcohol use disorder (AUD) prevalence and incidence; alcohol-related liver disease and cirrhosis; alcohol-related cancers; alcohol dependence; years of life lost; alcohol-attributable disability-adjusted life years; alcohol-attributable fraction of all-cause deaths); social impact (social-service use; criminal-justice costs; environmental costs; alcohol-related gender-based and intimate-partner violence; child maltreatment); economic impact (health-care costs; productivity losses; criminal-justice costs; drink–driving enforcement costs; insurance and property costs).
6. Methods and data	Transparent statement of the minimum methodological elements set out in section 3.7: • intervention package assessed; • comparator or business-as-usual scenario; • time horizon; • discount rate; • perspective; • data sources; • population covered; • costing method; • valuation of health and productivity gains; • treatment of government revenue; and • sensitivity analyses. Identifies which results are based on national data, modelled estimates or international assumptions, and which type of analysis each result comes from (cost-of-illness, intervention costing, health-impact modelling, cost-effectiveness analysis, return on investment, budget-impact or fiscal analysis).
7. Economic assessment of alcohol policy interventions	Analytical section (see 3.8 below). Presents the costs and benefits of the SAFER interventions and any additional relevant measures, using the analyses selected for the decision problem.
8. Key strategic recommendations	The recommendations the government will adopt regarding investing in alcohol policy – usually a phased implementation and financing plan.

3.8 Costs and benefits of alcohol policy interventions

The economic assessment presents the costs of implementing alcohol policy interventions and the benefits expected from them, using the analytical approach selected for the decision problem (section 3.6.1): a benefit–cost or return-on-investment analysis where the purpose is to make the case for additional resources, and costing, budget-impact, fiscal or cost-effectiveness analysis where the question concerns affordability, budget implications, revenue effects or comparative value for money. The cost and benefit categories below apply to all of these analyses. Depth of analysis depends on the availability of country data; partial analysis using internationally modelled benefit estimates is preferable to no analysis at all. Where a benefit–cost analysis is undertaken, it should follow the reference case guidelines for benefit-cost analysis in global health and development, so that methods and default values are consistent and results are comparable across countries and over time *(9)*.

3.8.1 Costs of implementing alcohol policy interventions

The main cost categories to consider are summarized in Table 21.

Table 21. Illustrative cost categories by SAFER intervention area

Pick-list of illustrative cost categories by SAFER intervention area, for costing the interventions.

SAFER intervention area	Typical cost categories to include
S – Strengthen restrictions on alcohol availability	Design of the licensing system; legal drafting and legislative process; law enforcement; inspections; research to set the number, density and location of outlets.
A – Advance and enforce drink–driving countermeasures	Legal drafting and legislative process; law enforcement; blood alcohol concentration (BAC) testing equipment; training for enforcement officers, health providers and motorists; research and evaluation.
F – Facilitate access to screening, brief interventions and treatment	Health-care service costs; legal drafting; health-personnel costs; training; facilities development and maintenance; research and evaluation.
E – Enforce bans or comprehensive restrictions on alcohol advertising, sponsorship and promotion	Legal drafting and legislative process; systems development (surveillance and enforcement); law enforcement; capacity-building; research and evaluation.
R – Raise alcohol prices through excise taxes and pricing policies	Legal drafting and legislative process; tax-administration and stamp systems; monitoring and surveillance; research and economic modelling.

3.8.2 Benefits of implementing alcohol policy interventions

The main benefit categories to consider are summarized in Table 22.

Table 22. Illustrative benefit categories for the economic assessment of alcohol policy interventions

Pick-list of illustrative benefit categories for the economic assessment of alcohol policy interventions.

Benefit type	Illustrative benefit categories (including costs saved or revenues realized)
Health	Lives saved/deaths prevented; injuries and disability prevented; alcohol dependence and alcohol use disorders prevented; cases of disease prevented; increased life expectancy and healthy life years.
Social	Crimes and delinquency prevented; violence cases prevented (including intimate-partner and gender-based violence); criminal-justice cases avoided; improved child development and family welfare.
Economic	Health-care costs saved; labour productivity gains; criminal-justice costs avoided; insurance claims avoided; assets preserved; deaths, injuries and disabilities prevented; tax revenue from well-designed alcohol-tax increases.

3.8.3 Displaced expenditure and transitional effects

Measures that reduce alcohol consumption also reduce spending on alcohol, and this is routinely raised in policy debate as an economic cost of the measure, expressed as lost output, lost employment or lost revenue in the alcohol and hospitality sectors. The assessment should address the argument directly rather than leave it unanswered, and should do so on the correct analytical basis. Expenditure not made on alcohol is not withdrawn from the economy: households redirect it to other goods and services, which generate output, employment and tax revenue in turn. The question is therefore not whether activity in the alcohol sector falls, but what the displaced expenditure supports instead and whether the net effect across the economy differs materially from zero. An assessment that enters the gross turnover or total employment of the alcohol sector as a benefit foregone does not compare like with like, because it counts the loss to one sector without counting the corresponding gain to others. Where national household expenditure data are available, the analysis should use them to model where displaced expenditure goes; where they are not, the assumption used should be stated.

What displacement does not answer is the distribution and the timing of adjustment. Reallocation is neither instantaneous nor evenly spread. It falls on particular firms, occupations and localities, and it can fall heavily where alcohol production, raw-material supply or on-premise hospitality is concentrated in one region or one part of the workforce. These effects are real, and they are transitional and distributional rather than a permanent loss of national output. The assessment should identify who bears them, quantify them where the data allow, report them separately from the aggregate effect, and set out the transition measures that would address them, such as retraining, diversification support or phasing of the measure. Presenting an adjustment cost as a permanent economic loss overstates it; passing over it in silence understates what implementation will require and weakens the case with the ministries and communities most affected.

Revenue effects call for the same care. Whether a change in excise raises or lowers total revenue depends on the price responsiveness of demand, on the degree to which the change is passed through to retail prices, and on any shift towards unrecorded or cross-border purchase; it should be modelled rather than assumed in either direction. Where redirected expenditure attracts other taxes, that should be accounted

for as well. Where a revenue effect is projected, the elasticity assumed, the pass-through assumed and the treatment of substitute expenditure should all be stated (section 3.8.4).

Where the analytical framework places a value on consumer welfare, it should state explicitly how it treats the welfare of people who drink without experiencing harm, and the grounds for any adjustment: incomplete information about risk, the effect of dependence on choice, consumption by minors, and harms falling on people other than the person drinking. An assumption that is left unstated invites the objection that it was made silently.

3.8.4 Uncertainty and sensitivity analysis

Economic assessments should describe key assumptions, data limitations and sources of uncertainty. Where modelling is used, sensitivity analyses should be presented for major assumptions, including intervention coverage, enforcement intensity, tax pass-through, price elasticity, health-care costs, productivity losses, discount rates and whether fiscal revenue is included in the return.

Tax revenue should be reported transparently. Where alcohol-tax scenarios are modelled, increased government revenue may be highly relevant to ministries of finance and cabinet discussions. At the same time, revenue effects should be reported separately from health-care savings and productivity gains, so that decision-makers can understand whether the estimated return is driven mainly by public health gains, productivity gains or fiscal revenue. More generally, returns should be reported by type: fiscal revenue; health and social gains; avoided public expenditure; and wider economic gains such as productivity. For each, the period over which it accrues and the budget holder or group to which it accrues should be stated, since savings that accrue to other budgets, to households or to employers, or only in the longer term, are valued differently by a ministry of finance than revenue or savings within the budget period. A short budget-cycle presentation and a longer strategic presentation may both be needed, as the Estonia and Sri Lanka examples in section 3.9 illustrate.

Where country data are incomplete, the analysis should distinguish national data, modelled estimates and regional or international assumptions. Partial analysis using clearly labelled assumptions is preferable to presenting uncertain estimates as if they were directly measured.

3.9 Key strategic recommendations

Any economic assessment should conclude with clear, actionable recommendations linked directly to the decision question being addressed. Recommendations should be costed and sequenced – specifying a phased implementation and financing plan, the ministries and agencies responsible, and the indicators that will demonstrate results and, where relevant, return on investment. Where the country also has an active community-level SAFER programme, the recommendations should explicitly address how national investment supports community-level resource mobilization (Part 5, section 5.8).

Recommendations should specify the financing route and safeguards for implementation. Where external financing is proposed, the recommendations should identify the responsible public authority, the due-diligence process, the accountability mechanism and the measures used to protect the work from conflicts of interest and commercial influence.

Recommendations should also include an implementation narrative: which institution should lead, which sectors are required, what legal or administrative actions are needed, which data systems must be strengthened, how enforcement will be financed, how progress will be monitored and what political or commercial risks need to be managed. Boxes 5 and 6 present two national experiences.

Box 5. Estonia: short-horizon return-on-investment analysis to support alcohol policy decisions

A 2026 article in the European Journal of Public Health presented a return-on-investment analysis of potential alcohol policy changes in Estonia. The analysis used mostly national data sources and focused on direct health-care costs and productivity losses due to premature mortality. The interventions compared were availability restrictions and alcohol excise-tax increases; for taxation, the analysis also included associated government revenue increases.

The Estonia analysis estimated that, in 2023, alcohol-attributable harms totalled €510.00 million, equivalent to 1.3% of Estonia's GDP, including €263.91 million in direct costs and €246.08 million in indirect costs. Proposed availability restrictions were projected to yield a net benefit of €6.33 million, while a 15% alcohol excise-tax increase was projected to lower health-care and productivity-loss costs by €1.77 million and increase tax revenue by €32.27 million. The reported ROI was approximately €15 per €1 invested for availability interventions, €477 per €1 invested for a 15% tax increase when revenue was included, and €25 per €1 invested without revenue.

The Estonia example illustrates three practical lessons for SAFER investment cases. First, a short time horizon can be useful when decision-makers need results that are politically and administratively verifiable. Second, tax revenue can dominate ROI estimates and should therefore be reported separately from health and productivity gains. Third, sensitivity analyses are essential: the Estonia analysis reported positive ROI across sensitivity analyses, but also made clear that results depended on assumptions about enforcement costs, policy impact, tax pass-through and valuation of productivity losses.

Source: adapted from reference (10).

Box 6. Sri Lanka: national investment case for a SAFER-aligned package of alcohol policy actions

The Sri Lanka investment case quantified the health and economic costs of alcohol consumption and estimated the benefits of implementing five WHO-recommended alcohol policy actions: strengthening restrictions on availability; advancing and enforcing drink–driving countermeasures; facilitating access to screening, brief interventions and treatment; enforcing bans or comprehensive restrictions on advertising, sponsorship and promotion; and raising excise taxes on alcoholic beverages.

The report estimated that alcohol consumption causes around 335 billion Sri Lanka rupees in economic losses every year in Sri Lanka, equivalent to 2.18% of GDP, and kills more than 20 000 people each year. It estimated that strengthening the five policy actions over 20 years (2024–2044) would save around 104 000 lives, avert 643 billion Sri Lanka rupees in economic losses and provide an ROI of 12.2:1. The report also presented a five-year ROI of 2.5:1.

The Sri Lanka example illustrates the value of a broader national investment case that combines burden analysis, multisectoral recommendations and development framing. It also shows the importance of being explicit about methods and limitations. The report described the use of national data where available, public and international data where needed, discounting, human-capital valuation and modelling limitations, including limitations of available tools for estimating health impacts. Sri Lanka raised alcohol excise duties twice during 2023, the year in which the draft investment case was presented to the government and to the public, and the investment case has since been cited in WHO and partner country documentation. No published evaluation has yet attributed specific policy decisions to it.

Source: adapted from reference (7).

3.10 Sustainable financing and resource mobilization for SAFER at national level

3.10.1 The financing problem

Sections 3.1 to 3.9 set out how a country builds the analytical case for investing in SAFER, embedded within the overall case for investing in health, using the economic assessment that fits the decision to be informed. That analysis, whether or not it is presented as an investment case, does not itself answer the question of where the resources should come from on a sustained basis. SAFER work has been under-financed in most countries, and the Global alcohol action plan 2022–2030 identifies the lack of financial and human resources as a primary barrier to introducing or accelerating action *(3)*. Investments in SAFER need to be considered within the overall financing constraints of the country. Those constraints can present obstacles to implementation of the full range of policies, and efforts to prioritize and to create efficiencies should therefore be considered wherever possible.

This section addresses the financing question directly. It first considers policy and coverage gaps and the financing constraints that contribute to them, and then sets out three arrangements for sustained financing of SAFER work at national level, with the conditions, advantages and limitations of each stated plainly. It also separates the public health rationale for alcohol taxation from the distinct question of how the resulting revenue is allocated. Box 7 sets out the mandate on which this section rests.

Box 7. The mandate for resource mobilization

The Global strategy to reduce the harmful use of alcohol (2010) identified mobilization of resources as one of the key components of global action, and urged Member States to mobilize political will and allocate financial resources for its implementation *(2)*. Progress since then has been uneven, attributed in part to insufficient governance, funding and infrastructure for implementation.

The Global alcohol action plan 2022–2030, endorsed by the Seventy-fifth World Health Assembly in decision WHA75(11), carries this forward as action area 6, resource mobilization, one of its six action areas *(3)*. It identifies the lack of the required financial and human resources as a primary barrier to introducing or accelerating action. Its global target 6.1 reads: "At least 50% of countries have dedicated resources for reducing the harmful use of alcohol by implementing alcohol policies and increasing the coverage and quality of prevention and treatment interventions for disorders due to substance use and associated health conditions." Progress is reported at milestones in 2022, 2025, 2027 and 2029/2030.

Two of the proposed actions for Member States under action area 6 bear directly on financing. Proposed action 1 is to "increase the allocation of resources, including international and domestic financial resources generated by new or innovative ways and means to secure essential funding, for reducing the harmful use of alcohol and increasing the coverage and quality of prevention and treatment interventions, according to the scope and nature of public health problems caused by alcohol consumption". Proposed action 2 is to "consider, when appropriate in national contexts, the development and implementation of earmarked funding or contributions from alcohol tax revenues or other revenues that are linked to alcohol beverage production and trade, or establishing a dedicated fund for reducing the harmful use of alcohol and increasing the coverage and quality of prevention and treatment interventions for disorders due to alcohol use and associated health conditions" *(3)*.

Progress towards target 6.1 is tracked through three indicators, quoted here as adopted: the number of countries "that have secured dedicated resources for the implementation of alcohol policies at the national level" (6.1.1); the number "that have secured dedicated resources for increasing the coverage and quality of prevention and treatment interventions within health systems for disorders due to substance use" (6.1.2); and the number "that introduced, when appropriate, dedicated funding for reducing the harmful use of alcohol from alcohol tax revenues or other revenues linked to alcohol production and trade" (6.1.3) *(3)*.

Two features of the mandate shape this section. First, the primary objective is sufficient and sustainable resourcing of implementation and service coverage, from whatever source, including regular public budget allocations. Proposed action 1 and indicators 6.1.1 and 6.1.2 concern the resources themselves, not the mechanism that delivers them. This handbook reads "dedicated resources" in that sense, as resources committed to the purpose, and uses "earmarked" only where earmarking is meant. Second, earmarked funding and dedicated funds are set out as measures that Member States may consider when appropriate in national contexts, and are reported separately under indicator 6.1.3. This section follows that structure: a secured public budget commitment is presented as the foundation for financing SAFER work in all countries, and earmarking and dedicated levies as supplementary arrangements that some countries may consider, with their conditions and limitations stated.

The WHO 3 by 35 Initiative, launched in July 2025, calls on countries to raise the real prices of tobacco, alcohol and sugary drinks by at least 50% by 2035 through health taxes, in a context of rising noncommunicable disease burden, constrained fiscal space and declining development assistance. It reflects the framing used throughout this section: health taxes are pursued primarily to make harmful products less affordable, while the revenue generated contributes to overall domestic resource mobilization, including for health systems and universal health coverage. Decisions on the allocation of that revenue remain subject to national budget processes and decision-making.

Countries should therefore pursue sustainable financing for alcohol policy as part of wider health-system and government financing, consistent with WHO health financing guidance on adequate and sustainable public funding and on using resources efficiently and equitably to achieve financial protection and service coverage.

3.10.2 Policy and coverage gaps, financing constraints and arrangements for sustained financing

Countries should start from the policy and coverage gaps, not from the funding gap. The first step is to establish which SAFER interventions are in place and enforced and which are not; the coverage and quality of screening, brief intervention and treatment services; unmet need; and the extent of household out-of-pocket payment for alcohol-related care (Part 1, section 1.1.5.6; Part 2). The second step is to work backwards from those gaps to identify where financing is the constraint: no allocation, allocations that are not executed, services outside the benefit package, enforcement functions without a budget line, or fragmented and short-term funding. Starting from the policy objective keeps attention on what is to be achieved rather than on obtaining more money.

Most SAFER interventions are implemented and financed outside the health sector. Availability, drink–driving, marketing and pricing measures depend on the budgets of finance, customs, police, transport, justice, trade and local government. Health-sector financing is central mainly to screening, brief intervention and treatment and to the coordination, monitoring and evaluation functions. The purpose of this step is not to construct a separate account of alcohol-related expenditure, which is rarely possible or necessary; it is to understand where financing constrains coverage and implementation. Where allocation and execution data exist for specific functions, they should be used (the Landscape Report collects them, Part 1, Table 10), and gaps in the data should be reported rather than filled with estimates.

Against that baseline, this section sets out three arrangements for sustained financing. Option A, a secured multi-year public budget commitment, is the foundation in all countries. Options B and C, an earmarked share of alcohol excise revenue and a dedicated public health levy on alcohol products, are supplementary arrangements that Member States may consider when appropriate in national contexts; they add to Option A and do not replace it. Both carry known public finance objections, which are set out with each option below; neither is required for SAFER implementation.

Option A. Secured multi-year public budget commitment (the foundation in all countries)
A multi-year public budget commitment to SAFER work is negotiated through the medium-term expenditure framework, or through an agreement between the ministries of finance and health and the other responsible sectors, and translated into the annual plans and budgets of the implementing agencies. It works through established budget, procurement, accounting and audit systems, integrates SAFER into routine health-system and government financing, and preserves the government's ability to prioritize across competing needs. This arrangement is necessary in all countries, whatever else is put in place: earmarked revenue and dedicated levies (Options B and C) are supplementary to it and are not a substitute for it.

The commitment is best organized around a single costed plan, with coordinated contributions from the budgets of health, finance, transport, justice, interior, education and social protection, since most of the work is implemented outside the health sector. The country's Roadmap should identify recurrent costs, responsible budget holders and medium-term commitments, and should avoid creating parallel financial-management arrangements. Durability depends on sustained political commitment, clear budget lines or programme allocations in each responsible sector, and monitoring of budget execution and of the coverage and results achieved, not of allocation alone.

Option B. Earmarked share of alcohol excise revenue (supplementary; where appropriate)
A defined amount from existing alcohol excise revenue is earmarked for alcohol policy work and related public health functions. The share is a national design choice; the experience of earmarking for health and of health promotion funds offers reference points *(11–13)*. The link between alcohol harm and the revenue that funds the response is direct, visible and politically intuitive. However, because alcohol taxation is intended to reduce affordability, consumption and harm, the base is meant to shrink, so essential functions should not be pegged to a floating share of it; a fixed or periodically adjusted commitment is more predictable.

Ministries of finance commonly raise concerns about earmarking, because it reduces budgetary flexibility and can complicate fiscal discipline. Country experience with earmarked tobacco and alcohol taxes shows that these concerns can be resolved, notably where the earmark accompanies a tax increase and therefore draws on new revenue rather than on existing allocations, and where the ministry of finance retains the ability to adjust other allocations *(11, 12)*. The productive approach is to identify where the interests of the ministries of finance and health converge, rather than to position one against the other. Where earmarks have created a visible and durable funding channel, it has typically been through a semi-autonomous fund with a defined purpose and its own governance. The Thai Health Promotion Foundation (ThaiHealth) is the most widely cited example: an autonomous public body financed by a 2% surcharge on tobacco and alcohol excise, governed by its own board, that finances population-based health promotion, including alcohol policy advocacy and community action, rather than individual clinical services *(11, 13)*. The equity case in section 3.10.3 gives the earmark its strongest form: revenue directed to coverage of prevention, treatment and financial protection for lower-income groups, channelled through the pooled health financing system rather than through a parallel fund. The Philippines applied this design in its 2012 tobacco and alcohol excise reform, directing more than 85% of the incremental revenue to health, principally to universal health coverage for the poor and to public health facilities *(11)*.

Earmarking also has clear limitations. Offsetting of discretionary allocations is common once an earmark is introduced, so earmarked revenue does not necessarily represent additional public funding for health or for alcohol policy; additionality is difficult to specify and harder still to monitor. An earmark can act as a ceiling rather than a floor on the resources allocated to its purpose, and by bypassing the regular budget allocation process it can weaken rather than strengthen the political prioritization of that purpose. Earmarked revenue can be volatile and poorly matched to expenditure needs, and where it is not pooled

with other resources it can work against the integration of service delivery. Good practice therefore includes a sunset or periodic review provision: an earmark exists to protect a purpose from political fluctuation, and it should be reconsidered after a defined period if political support for the purpose has not been sustained *(12)*. Any proposal should be developed with the ministry of finance and assessed for net additional resources, predictability, administrative cost, review provisions and consistency with the overall budget framework.

Earmarking is one option that Member States may consider, when appropriate in national contexts, as recognized in the Global alcohol action plan 2022–2030 *(3)*. It is not presented here as a preferred option, and SAFER implementation does not depend on it.

Option C. Dedicated public health levy on alcohol products (supplementary; where appropriate)
A separate, named levy is added to alcohol products at the point of production, import or first sale, with the proceeds directed by statute to a defined receiving body. International practice, and the design considerations in section 3.10.5, favour a receiving body with a population-based public health mandate, of which alcohol policy is one funded priority, over a vertical alcohol-only fund. The action plan also recognizes the establishment of a dedicated fund for reducing the harmful use of alcohol as a measure that countries may consider where their context justifies it *(3)*. The advantage of Option C over Option B is visibility: the levy has a distinct identity and the public can see what it pays for. The disadvantages are three. It requires new primary legislation, which is typically harder than amending budgetary allocations. It duplicates excise-administration infrastructure. And it carries a structural risk: a small visible "health levy" can be adopted by governments as a substitute for proper alcohol excise reform, with the political credit for the visible levy obscuring the absence of the more powerful pricing intervention.

Option C may be considered in specific contexts, particularly where political support exists for a visibly named public health levy and where it operates as a complement to, not a substitute for, excise reform. The limitations of earmarking set out under Option B, including offsetting, additionality, volatility, review provisions and administrative duplication, apply in full. Where Option C is adopted, the legislation should prevent it from being used as a basis for arguing against excise reform and should provide for periodic review and public reporting.

Models incompatible with this framework
Models in which the SAFER funding flow is paid directly by alcohol industry actors are not presented as an option in this handbook. An example would be a producer or importer levy paid into a public health body that subsequently funds SAFER work. Such models create a structural conflict of interest: the alcohol industry funds the public health response and can plausibly claim resulting standing in how the response is designed, monitored and spent. Even where the receiving body resists that claim, the appearance of dependence on industry revenue is corrosive to public trust and inconsistent with the conflict-of-interest principles in Part 4. The financing flow must operate state-to-state, never direct payment by industry to a public health body: from general revenue (Option A), from an earmarked share of state-collected excise (Option B), or from a state-collected dedicated levy (Option C).

3.10.3 Alcohol taxation, equity and the separate question of revenue use

Alcohol excise, the revenue it raises and the use of that revenue are related but separate questions. The public health case for alcohol excise rests on three propositions about taxation, affordability and the distribution of alcohol harm. They support effective taxation and an equitable policy response; they do not by themselves determine whether revenue should be earmarked. Read together, they show that what is regressive is the payment of the tax, while the health gains it produces, and its net effect where the revenue reaches those bearing most of the harm, are not.

First proposition. Alcohol excise is a public health instrument. Its purpose is to reduce the affordability of alcohol and the harm it causes, and it is the core of the pricing area of the SAFER package. It is designed, legislated and evaluated within national alcohol policy. Revenue is a consequence of the tax, handled through the general budget process, not its objective, and should be reported separately from health and productivity gains *(14)*.

Second proposition. Considered on its own, and before the use of the revenue is taken into account, the payment of alcohol excise is regressive in incidence. This is the standard equity objection to alcohol taxation. Lower-income households spend a larger share of household income on alcohol than higher-income households do, even where lower-income households drink less on average. The alcohol tax therefore falls disproportionately on lower-income drinkers as a share of disposable income. That objection is empirically supported in many settings *(14, 15)*.

Third proposition. The incidence of alcohol harm is even more regressive than the incidence of the tax. Per litre of alcohol consumed, lower socioeconomic-status (SES) populations experience substantially higher mortality, morbidity, social and economic harm than higher-SES populations. This is the alcohol harm paradox, documented in a substantial body of epidemiological evidence *(16, 17)*. The mechanisms include co-occurring risk factors, weaker health-system access, more dangerous drinking patterns in some lower-SES subpopulations, less protected employment, and concentrated harm in lower-SES neighbourhoods through outlet-density and night-time economy effects.

Taken together, the three propositions support combining effective taxation with measures ensuring that lower-income groups obtain the health gains of reduced affordability and can reach prevention, treatment and financial protection. They strengthen the case for adequate public financing of the alcohol response, particularly for the populations bearing the highest harm. They do not by themselves require that a defined share of excise revenue be earmarked, which remains a public finance decision. They do supply the strongest argument for earmarking where it is used. Directing part of the revenue from a regressive tax to prevention, treatment and financial protection for the lower-income populations that pay it and bear most of the harm makes the net effect of the tax progressive. Where the regular budget process has not delivered adequate financing for those services, an earmark of this kind has been the route by which it was secured *(11, 12)*.

The equity argument carries weight with both the public health and the ministry of finance audience. The health gains from taxation accrue disproportionately to lower-income groups, who bear a higher burden of alcohol-attributable harm and are typically more responsive to price. Where treatment for alcohol use disorders sits outside the benefit package, those same households also pay out of pocket for care or go without it. Financial protection and equitable access to treatment therefore belong alongside taxation in the policy response.

3.10.4 What financing for SAFER should fund

Whatever the source of funds, resources should be directed to evidence-based priorities and to the implementation capacity needed to deliver them. The funded work should include prevention and treatment and the structural interventions that reduce the SES gradient in alcohol harm. The handbook suggests that countries structure the allocation around five priorities:

- screening and brief intervention (SBI) coverage in primary care serving lower-SES populations, including primary-care providers, emergency-department teams and community-health workers in higher-deprivation areas (SBI is a short, structured conversation delivered by a health worker to people whose drinking puts them at risk; see the glossary);

- licensing review, enforcement and outlet-density measures in neighbourhoods with concentrated harm;
- night-time economy enforcement, including drink–driving and responsible-server activity, in zones with concentrated harm;
- specialized treatment-service capacity for alcohol use disorders, with attention to access for under-served subpopulations;
- child-protection, perinatal and family-support services in communities with high alcohol-related family-violence rates and high prevalence of fetal alcohol spectrum disorders.

Each priority is already part of the SAFER architecture. Allocation should follow the country's priorities and the policy, coverage and financing constraints identified in section 3.10.2, together with burden and unmet need, and should name the responsible institution, the expected outputs and the indicators. The financing route differs by type of activity. Individual clinical services (screening, brief intervention and treatment) should be financed through the regular health financing arrangements: inclusion of evidence-based interventions in the benefit package, adequate provider capacity and referral pathways, and reduced reliance on out-of-pocket payment; screening should not be scaled ahead of the capacity to treat. Where those arrangements do not reach lower-income groups, earmarked revenue can be channelled into them for that purpose (Option B); that is different from creating a separate fund for clinical services. Population-based functions (policy implementation and enforcement support, monitoring, community action, public communication in support of policy implementation and enforcement, and support to independent civil society) are financed through the responsible sectoral budgets and are the functions for which a dedicated fund, where one exists, is best suited (section 3.10.5). Stand-alone education and awareness campaigns are not among them: the evidence that they reduce consumption or harm is weak *(18)*, and communication is effective where it accompanies enforcement, as with drink–driving publicity, or builds public support for the SAFER measures.

Institutional infrastructure is equally important for the durability of the policy work itself: the national alcohol policy authority or equivalent body, monitoring and evaluation capacity (Part 2), the conflict-of-interest architecture (Part 4), the community-level coordination structures (Part 5 section 5.7), and support to independent CSOs operating in the alcohol policy field (section 5.7.5). Much of this sits outside the health sector and may need to be financed through the relevant sectoral budgets. Efficiency should be judged on budget execution, service coverage, enforcement activity and results, not on the amount allocated alone.

3.10.5 Design of the receiving body

Where a country establishes a dedicated fund, budget window or receiving body, its purpose and its governance matter as much as the source of funds. International experience indicates that such funds work best when they finance population-based public health functions rather than individual clinical services: policy implementation and enforcement support, monitoring, community action, public communication in support of implementation and enforcement, and support to independent civil society. Clinical services should remain within the regular health financing arrangements, so that service delivery stays integrated. The ThaiHealth model is built on that distinction *(12, 13)*. Where earmarked revenue is intended for treatment coverage of lower-income groups, it should therefore be channelled into the regular health financing arrangements rather than through the fund (Option B). Experience also points to five design principles for the receiving body *(11, 13)*. A new body should not be created where an existing public institution can perform the function with the same safeguards.

- The receiving body should have a public health mandate written into its founding instrument, with the prevention and reduction of alcohol-related harm among its statutory objectives.

- The receiving body should be structurally protected from alcohol industry funding, partnerships and influence, with the prohibition written into its founding instrument and applied to its staff, board, advisory bodies and procurement processes (consistent with Part 4).

- The receiving body should have governance arrangements that protect it from the political direction of any single ministry, typically through an independent board with multi-stakeholder composition including civil society, professional bodies, academia and lived experience, but excluding the alcohol industry and its trade associations.

- The receiving body should report publicly on the use of funds against the priorities set out in section 3.10.4, with annual reports tabled in parliament and audited by the relevant supreme audit institution.

- The receiving body should have a defined mechanism for funding independent CSOs operating in the alcohol policy field, with funding decisions made on transparent criteria and structured to preserve the independence of the funded organizations from both industry and government direction (consistent with section 5.7.5 of Part 5).

Where these principles are not met, the design of the levy makes little difference: revenue paid into a poorly governed body cannot be relied on to deliver the intended public health benefit. The receiving-body design should be specified at the same time as the revenue source, not subsequently.

3.10.6 Order of magnitude: illustrative country profiles

To help ministries of health and finance judge affordability, three illustrative country profiles, generic rather than naming actual countries, indicate the order of magnitude of the recurrent cost of a national SAFER programme and set it beside the scale of alcohol excise receipts. Excise receipts are used only as a comparator of scale, because that is the comparison a ministry of finance most often asks for; where data allow, comparison with government health expenditure and with the estimated costs of alcohol-related harm (section 3.8) is more informative. The profiles say nothing about the source of funds, which is decided under Options A to C, and they do not recommend any share of excise revenue.

These hypothetical calculations are included solely to illustrate arithmetic scale. The assumed consumption levels, excise receipts, programme costs and programme content are not WHO estimates, benchmarks or recommendations and should not be used for country planning.

Profile 1. Low-income country, 8 million adults.
Adult per capita alcohol consumption 3 litres of pure alcohol; alcohol excise duty modest relative to international benchmarks; net excise receipts of approximately the equivalent of US$ 40 million per year. A national SAFER programme costing about US$ 1.2 million per year could comprise a small national alcohol policy coordination unit, basic SBI training for primary-care staff across the priority districts, a community-level implementation programme in a small number of pilot communities, and basic monitoring infrastructure aligned with the WHO city-level monitoring guidance.

Profile 2. Middle-income country, 25 million adults.
Adult per capita consumption 6 litres of pure alcohol; alcohol excise duty in line with WHO recommendations; net excise receipts of approximately the equivalent of US$ 400 million per year. A programme costing about US$ 12 million per year could comprise a national alcohol policy authority with a fully staffed

secretariat, community SAFER implementation across multiple provinces, an economic analysis team and monitoring-and-evaluation infrastructure at national level, scaled SBI training across primary care, and a defined budget line for independent civil-society work.

Profile 3. High-income country, 40 million adults.
Adult per capita consumption 9 litres of pure alcohol; alcohol excise duty at or above WHO benchmarks; net excise receipts of approximately the equivalent of US$ 4 billion per year. A programme costing about US$ 120 million per year could comprise a comprehensive package: a national alcohol policy authority with an independent programme budget, multi-year community programmes across all subnational tiers, full SBI scale-up across primary and emergency care, a substantial monitoring and evaluation programme with peer-reviewed publication output, comprehensive support to independent civil-society work, and a research and innovation budget.

The three profiles are illustrative and the figures should be replaced with country-specific figures in any operational application. What they show is that the recurrent cost of a national programme is a small fraction of alcohol excise receipts and, as section 3.8 shows, of the costs of alcohol-related harm; that is the point of relevance to a ministry of finance. Whichever arrangement in section 3.10.2 is chosen, the proposal should show what the resources would purchase and how expenditure, coverage and results would be monitored. Where earmarked revenue is proposed, expected gross revenue should be reported separately from the net additional resources remaining after any offsetting of other allocations.

3.10.7 Connection to sustainability and durability

The design choices in this section connect back to the institutionalization argument articulated in the executive summary and in the “How to use this handbook” page. Alcohol policy gains secured during a window of political opportunity, but without sustained financing, have often been reversed when the political window closes. Durability depends on political commitment backed by medium-term funding, integration into routine health-system and government financing arrangements, clear responsibility across sectors, governance that protects funds from capture, and transparent monitoring of expenditure and results.

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PART 4

Protecting SAFER interventions from conflict of interest

PART 4

Protecting SAFER interventions from conflict of interest

4.1 Introduction

Several SAFER interventions require legislative or regulatory action – either building on an existing framework or creating new law. Many of those measures restrict commercial activity. As a result, governments have faced – and will continue to face – opposition when putting these recommendations into practice. The focused nature of SAFER demands that public health objectives are safeguarded and that speculative, unproven or distracting alternatives are rejected. Involvement of the alcohol industry in the formulation of public health-oriented policies carries substantial risks that must be minimized. Reliance on scientific evidence, and on transparent processes that are visibly independent of commercial influence, is the ultimate protection for the integrity of the SAFER package *(1, 2)*.

Part 4 provides the principles, the steps and the working tools by which countries put that protection in place at national level. The companion treatment at community level is in Part 5 section 5.6 and Part 6, with a workplace alcohol guidance template in Annex 12 (supporting Part 6). The principles anchoring this part are summarized in Box 8.

Box 8. Key principles for safeguarding alcohol policy from conflict of interest

- Define conflict of interest (COI) clearly – financial, professional and personal.
- Identify potential sources – industry representatives, lobbyists, policy-makers and researchers with ties to the alcohol industry, including bodies substantially funded by the industry.
- Require disclosure – establish clear rules on what must be disclosed, by whom and when.
- Restrict industry involvement in policy development – without exception.
- Ensure transparency of funding – research, policy and advocacy funding sources must be disclosed.
- Engage non-industry expertise – public health, epidemiology, addiction science and affected communities.
- Align with international guidance – the WHO Global strategy and the *Global alcohol action plan 2022–2030*; workplace alcohol guidance discussed among United Nations agencies *(2, 3)*.

4.2 Conflict of interest in alcohol policy

The SAFER initiative recommends that public health objectives be protected from undue influence through active management of COI. This has historically been poorly understood at country level, and as a result alcohol industry interests have impeded the implementation of effective interventions. This Part should be read alongside the WHO *SAFER technical package (1)*.

When commercial actors influence alcohol policy there is a conflict of interest. It needs to be identified and managed effectively. This responsibility applies across all government departments. SAFER interventions require close working between ministries; the target audience for this guidance therefore includes officials in every government department involved with SAFER, with particular relevance to health officials given the lead roles played by health. Key terms are defined in Box 9.

Box 9. Key terms relating to alcohol policy and conflict of interest

Alcohol policy – all aspects of public policies relating to alcohol, health and social well-being.

Alcohol policy development – the process through which issues and problems are identified, policy aims and goals are set, and policies are prioritized, formulated and consulted upon prior to decision and formal adoption.

Alcohol policy implementation – the processes of introducing and routinizing the actions identified in policy decisions.

Alcohol policy interference – alcohol industry efforts, intentional or unintentional, to stop, delay or weaken progress towards public health goals in alcohol policy development and implementation.

Conflict of interest (COI) – a situation in which the independence or objectivity of professional judgement or action regarding a primary interest (for example, public health) is unduly influenced, or may reasonably be perceived to be so, by a secondary interest (for example, commercial interest).

COI in alcohol policy – the inherent contradiction between the commercial interests of alcohol producers and retailers in increasing consumption, and public health interests in reducing consumption. Because increased consumption creates more profit and more harm, the alcohol industry seeks to influence policy against the public health interest.

4.3 Guidance on COI from international policy documents

4.3.1 Global strategy to reduce the harmful use of alcohol (2010)

The Global strategy to reduce the harmful use of alcohol, endorsed by consensus of all Member States at the Sixty-third World Health Assembly in 2010 (resolution WHA63.13), laid the foundations on which the current protections stand *(3)*. Three of its guiding principles are central to this Part. First, public policies and interventions to prevent and reduce alcohol-related harm are to be guided and formulated by public health interests, based on clear public health goals and the best available evidence. Second, all involved parties have the responsibility to act in ways that do not undermine the implementation of those policies and interventions. Third, public health is to be given proper deference in relation to competing interests, and approaches that support that direction are to be promoted.

On the industry itself, the strategy speaks in the register of encouragement within limits: it acknowledges economic operators as important players in their roles as developers, producers, distributors, marketers and sellers of alcoholic beverages, encourages them to consider effective ways to reduce harmful use within those core roles, including self-regulatory actions, and notes that they could contribute data on sales and consumption *(3)*. The strategy contains no refrain clauses; the sharpening came first through the Secretariat's stated position and then through codification. Responding in The BMJ in 2013 to reporting on relations between doctors and the alcohol industry, Director-General Margaret Chan set out the Organization's reading in two sentences: "The development of alcohol policies is the sole prerogative of national authorities. In WHO's view, the alcohol industry has no role in formulating policies, which must be protected from distortion by commercial or vested interests" *(4)*. The strategy established this as principle: public health-led formulation, the universal duty not to undermine and proper deference to public health. The Director-General thus stated it as exclusion, and the Global alcohol action plan 2022–2030 subsequently codified it as an operational principle, together with an endorsed description of the conduct operators must abstain from (section 4.3.2).

4.3.2 Guiding principles from the Global alcohol action plan 2022–2030

The Global alcohol action plan 2022–2030, endorsed by the Seventy-fifth World Health Assembly in 2022, provides the contemporary guiding principles *(2)*. Three are central to this Part. First, protection is an operational principle of the plan itself: public policies to reduce the harmful use of alcohol are to be protected, in accordance with national laws, from commercial and other vested interests that can interfere with and undermine public health objectives. Second, the plan recognizes the inherent contradiction between the commercial interests of alcohol producers and the interests of public health, and therefore the need to shield public health-oriented policy-making from interference rather than to balance it against industry preferences. Third, the plan confines any industry contribution to measures stringently within operators' core commercial roles, and asks operators to desist from activities that harm health or obstruct policy.

The plan goes further than principle. In each of its six action areas it addresses economic operators directly, and the operative content of those statements is a set of refrain clauses that all Member States have endorsed: an agreed description of the industry conduct that is incompatible with protected policy-making. Section 4.11 sets out those clauses and translates each into the corresponding government-side practice, so the principles stated here are carried through to operational instruments rather than left as preamble *(2)*.

4.3.3 Workplace alcohol guidance and protection from industry interference

Workplace alcohol guidance – including provisions on protection from industry interference – has been discussed among United Nations agencies. The principles that have emerged from those discussions are directly transferable to other large public-sector employers and to community authorities. Part 6 of this handbook and Annex 12 set out an adaptable template for that broader use.

4.4 Using this guidance

The purpose of Part 4 is to help protect policy-making from undue influence and thereby accelerate implementation of the SAFER interventions. The principles, toolkits and associated materials offer practical resources to strengthen governance, to be used flexibly as circumstances develop. There can be no engagement with commercial interests in alcohol policy that is not well managed by rigorous COI provisions, if alcohol policy is to progress. Experience shows that unmanaged engagement enables policy interference that damages health. The more rigorous the management of COI, the more there is to gain for health.

4.5 What is alcohol policy interference?

There are important similarities in the strategies and tactics used by all commercial sectors to influence public policies. Like any commercial sector, the alcohol industry seeks to create an environment conducive to its commercial interests. For alcohol – and some other sectors – this causes damage to public health and social welfare. The direct effects on health of the use of unhealthy commodities, and undue influence on policy, create a conflict of interest. Where commercial interests influence alcohol policy, successful outcomes are undermined *(1, 5)*. Box 10 defines the industry-related terms used in this part.

Box 10. Key terms relating to the alcohol industry

Alcohol industry – manufacturers of alcoholic beverages, wholesale distributors, major retailers and importers, as well as business associations or other non-State actors representing them.

Transnational alcohol corporations – the section of the alcohol industry dominated by a small group of companies that control a large share of the global market.

Trade associations – voluntary groupings of companies that explicitly represent collective interests in policy-making and thereby influence policy. They perform important policy-interference functions within the industry and are usually organized sectorally and nationally.

Corporate social responsibility (CSR) – the idea that a business has a responsibility to the society around it. In practice, alcohol industry CSR has been widely characterized by activities that are not stringently within core roles and that support policy interference.

Social-aspects/public-relations organizations (SAPROs) – industry-funded bodies whose stated purposes include public information, self-regulation or CSR. They are widely understood to perform public-relations and policy-interference functions.

4.5.1 Operational definition for handbook purposes

For operational purposes, references to the alcohol industry in this handbook include the following actors, whether or not alcohol is each actor's primary line of business:

- producers, manufacturers, importers, distributors and wholesalers of alcoholic beverages;
- retailers – including supermarket and other retail chains – where alcohol represents a material share of revenue or product range;
- online alcohol sellers, platform retailers, marketplace operators and rapid-delivery platforms when carrying alcohol;
- alcohol industry trade associations and trade-promotion bodies, including beer-, wine-, spirits- and hospitality-sector associations;
- social-aspects/public-relations organizations funded by the alcohol industry;
- corporate foundations and philanthropic vehicles controlled or substantially funded by alcohol industry actors;

- marketing, communications, advertising and public-relations agencies where alcohol industry accounts represent a material share of billings;
- event organizers, venue operators and festival promoters whose business model includes alcohol industry sponsorship or alcohol sales as a material revenue source;
- third parties acting on behalf of any of the above; and
- organizations substantially funded by any of the above, including for research, communication, advocacy or training purposes.

"Material share" is a country-specific judgement, typically operationalized through procurement-disclosure thresholds set by the relevant ministry or independent body. The threshold is not the whole test: classification is also situational. Whatever an actor's overall business mix, it should be treated as an economic operator for a given policy process whenever the matter under discussion engages its commercial role in the production, distribution, marketing or sale of alcohol. This topic-contingent limb closes a gap that a "primary business" or share-only test would leave. Under such a test, a multi-product actor would fall outside the conflict-of-interest framework on the very decisions in which its commercial interest is engaged, such as pricing, licensing or promotion rules. A large supermarket chain that is a major alcohol retailer is one example. The situational classification applies for that process and topic; it does not attach a permanent label.

4.5.2 Distinguishing policy partnership from regulatory contact

The alcohol industry should not participate in the formulation of public health-oriented alcohol policy. Where governments are legally required to communicate with regulated entities for compliance, enforcement, data collection or statutory consultation, such interactions should be limited, transparent, documented and governed by conflict-of-interest safeguards.

Routine regulatory contact is different from policy partnership. Licensing inspections, enforcement actions, tax administration, product-safety checks and legally required consultations may be necessary, but they should not provide alcohol industry actors with a role in setting public health objectives, drafting policy options or framing SAFER recommendations.

Alcohol industry interests, and how they are organized, vary greatly between countries. It is important to consider within the country context the specific nature of the policy-interference threat. In some countries alcohol is a major export; in others, large retailers are particularly prominent in alcohol policy. All commercial entities and related organizations relevant to alcohol policy need to be identified, with particular attention given to the largest and most powerful actors and entities with which they have relationships.

Alcohol industry actors use very similar arguments in different countries to frustrate effective policy implementation *(6)*. They usually portray alcohol harms narrowly, as caused by a minority of drinkers who need targeted responses; in reality, the overall level of drinking in a population is a key determinant of its alcohol harms *(5)*. Commercial interests promote partnership working to gain access to, and influence over, alcohol policy *(6)*. Within such partnerships, attention is diverted from the health and social harms of the product; responsibility is relocated to the individual drinker; harms are deflected onto unrecorded alcohol; and the legal product itself is implied to be harmless if used in "moderation". Policy interference thus results in action on alcohol being opposed, obstructed and delayed; where action is inevitable, attempts are made to avoid or dilute the measures that evidence shows will make the greatest difference, with the spectre of unrecorded alcohol regularly invoked against pricing and availability measures in

particular *(5, 6)*. Unrecorded alcohol is a genuine public health concern that warrants its own response; the interference lies in invoking it as a reason to leave recorded alcohol unregulated.

4.6 Practical steps to protect against alcohol policy interference

This section introduces practical tools for getting started on COI management and for specific use cases. The tools should be discussed in groups – within the ministry of health and in inter-departmental contexts. Adapt the tools to the national situation, altering questions or adding new ones as needed; this can be done when translating the tools into local language.

4.7 SAFER principles for management of COI in alcohol policy

The principles below (Table 23) are aligned with the global mandates for 2030 and with workplace alcohol guidance discussed among United Nations agencies *(2)*. Communities adapting them at subnational level should also consult Part 5 section 5.6.

Table 23. SAFER principles for management of COI in alcohol policy

No.	Principle	Summary
1	Shared responsibility across government	All government departments have a role in ensuring alcohol policy is free from undue influence.
2	Protecting alcohol policy governance	Governments establish robust safeguards against interference, particularly from the alcohol industry.
3	Public health leadership	Public health leadership is crucial for prioritizing alcohol as a public health issue, coordinating policy efforts and allocating resources.
4	Conflict-of-interest management	Addressing COI is essential to advancing public health. Integration of COI governance into national strategies supports the targets of the Global alcohol action plan and the 2030 Agenda.
5	No role for the alcohol industry in policy development	Consistent with the SAFER technical package and the Global alcohol action plan 2022–2030, the alcohol industry has no role in policy development. Any such engagement creates a conflict of interest.
6	Avoiding industry funding	Governments – particularly ministries of health – do not accept funding from the alcohol industry for programmatic support, policy formulation or research.
7	Governance structures for COI	National arrangements, such as high-level committees or advisory groups, are tailored to local context and oversee COI management and transparency.
8	Learning from tobacco control	Alcohol policy requires measures to prevent undue influence similar to those used for tobacco control. Although there is no equivalent to Article 5.3 of the WHO Framework Convention on Tobacco Control, the underlying public health imperative is the same (7).
9	Balancing partnerships	Collaboration with stakeholders is common in policy-making but must not compromise public health objectives.
10	International cooperation and knowledge-sharing	Global collaboration, including with WHO and public health-oriented organizations, strengthens governance.
11	Beyond the alcohol industry	COI considerations extend to related sectors – advertising, retail, tourism and hospitality.

No.	Principle	Summary
12	Cross-governmental engagement	Ministries outside health may interact with the alcohol industry for legitimate non-health purposes; governance arrangements ensure such interactions do not influence public health policy.
13	Transparency and public trust	Transparency in interactions with the alcohol industry is essential for public trust.

Governance structures for the management of conflict of interest should recognize the watchdog role of independent CSOs, particularly those protected from alcohol industry funding. Civil-society monitoring of industry interference complements but does not replace official monitoring, and is structurally important during periods when ministries face political constraints on speaking publicly about industry behaviour.

4.8 Steps for managing conflict of interest and policy interference

Effective governance of alcohol policy requires a structured and strategic approach. The following five steps guide the identification of risks, the planning of action and the embedding of safeguards. They are sequential but iterative.

Step 1: Laying the groundwork for action

The first step focuses on understanding the value of acting on COI and identifying initial priorities. Consider the current need or opportunity for progress in alcohol policy; how SAFER interventions are being prioritized; how COI governance is currently approached; the most obvious risks of policy interference; the key allies for COI work; the consequences of delaying action; and the actions that are manageable and deliverable within defined timelines.

Step 2: Early action planner

Step 2 helps identify COI risks, plan specific actions and integrate safeguards. Consider what COI and policy-interference issues have been encountered so far; what future risks can be anticipated; whether risks are captured in SAFER landscape reports or other relevant documents; whether existing plans include a robust COI strategy; whether partnerships with the alcohol industry need review; what specific actions are required; how COI governance can be integrated into legislative drafting; and the timeline for each action.

Step 3: Checklists for meetings involving the alcohol industry

Two checklists support due diligence for two common contexts.

Checklist 1 – Attendance at meetings organized by others

- What role do the alcohol industry or related organizations play in the meeting?
- What is the government's role at the meeting?
- Who is organizing and funding the meeting?
- Who is attending?
- Where is the meeting held?
- How can unstructured engagement with the alcohol industry be avoided?
- How might the interaction be perceived by other stakeholders?
- How will transparency be ensured throughout the meeting?

Checklist 2 – Meetings organized by government

- Who are the alcohol industry participants and what are their affiliations?
- Has all requested information been provided by the participants?
- Has a concept note been prepared covering purpose, scope and COI governance?
- Has the concept note been shared with COI advisors?
- Is the meeting appropriate to proceed?
- What are the staffing requirements?
- Who is chairing and setting the agenda?
- Have restrictions on misrepresentation or promotion of industry CSR been communicated in advance?
- Who will document the meeting, and how will the records be published?

Step 4: Inter-departmental policy-interference protocol

Step 4 helps ministries of health work with other government departments to manage COI risks. A typical inter-departmental protocol defines the public health objectives that must be protected; establishes a central point of contact in each department for COI issues; requires pre-notification of meetings with the alcohol industry, with standardized reporting; requires pre-approval of any request for industry data, analysis or funding; sets out clear rules on staff movement between the alcohol industry and government; and provides a dispute-resolution mechanism between departments.

Step 5: Implementation engagement checklist

Step 5 provides a summary checklist for implementation engagement across the SAFER package. Has the Landscape Report identified alcohol industry actors and their likely positions? Have the SAFER Steering Committee and technical working groups reviewed their membership for COI? Are disclosure requirements in place for all SAFER consultants, experts and advisors? Are all partnerships, grants and sponsorships reviewed against the COI framework? Are media communications reviewed to ensure they do not inadvertently amplify industry narratives? Is there a route for whistle-blowing and for reporting attempted interference?

Interaction decision guide

If the interaction is about policy development, the appropriate response is not to engage: the alcohol industry has no role in policy formulation. If the interaction is a legally required consultation, it should be conducted transparently, with submissions documented and published where lawful. If the interaction is about regulatory compliance or enforcement, it should be limited to the regulatory purpose and recorded. If the interaction is an offer of funding, sponsorship, research support, communication support or in-kind contribution for public health alcohol work, it should be declined. If the interaction concerns data or technical information, the request and response should be documented, the methodology should be transparent, and the information should not shape policy outside the public health process.

All SAFER steering committees, technical working groups and assessment teams should maintain a simple interaction log for contacts with alcohol industry actors or organizations linked to them. The log should record the date, parties, purpose, summary of discussion, documents shared and any follow-up action.

4.9 Capacity-building

COI governance requires sustained capacity-building – induction and refresher training for new officials; peer-learning with counterparts in tobacco, food and other commercial-determinants-of-health areas; and technical collaboration with WHO and public health-oriented partners.

Training should reach everyone who may be approached by the alcohol industry: officials in every department involved in SAFER, the COI contact points named under the inter-departmental protocol (Step 4), and the members of SAFER steering committees, technical working groups and assessment teams. It is most useful when built around the instruments of this Part – the principles in Table 23, the meeting checklists, the inter-departmental protocol, the interaction decision guide and the interaction log – and when it teaches recognition before response, using section 4.5 and the case studies in Boxes 11–13 to show how interference presents in practice. It should be refreshed when membership or mandates change, and ahead of predictable pressure points such as legislative drafting (Box 12).

Capacity-building for alcohol policy should itself be protected. Training for officials, enforcement staff and the health workforce should come from public health channels, and training, secondments and technical support offered by economic operators should be declined (Table 24, action area 4).

4.10 Case studies

The following case studies, drawn from the WHO progress report on the SAFER initiative, illustrate three recurring situations in protecting alcohol policy from conflict of interest (8). Boxes 11–13 present the three cases.

Box 11. Getting going on conflict-of-interest safeguards: the i-Mark in Ireland

Communities participating in Ireland's Building SAFER Communities project adopt independence from alcohol industry influence as a foundational principle from the outset. The i-Mark – Supporting Communities Free from Alcohol Industry Influence – was launched in 2022 by Alcohol Forum Ireland and the Irish Community Action on Alcohol Network. Organizations adopting the i-Mark receive a toolkit, resources, campaigning support and training, and join a network that works collectively to resist alcohol industry interference; a second iteration followed in mid-2025. The lesson is that a written, visible independence standard – adopted before engagement pressures arise – is a practical first step in operationalizing the principles in this part (see also Part 5, section 5.9) *(8)*.

Box 12. Managing engagement during legislative drafting: Uganda's Alcohol Control Bill (2022)

In 2022 a private member's Alcohol Control Bill was introduced in Uganda's parliament to strengthen regulation of alcohol marketing, production and availability. The Bill was eventually rejected after sustained opposition from major brewers and their trade association. Analyses document how these actors used corporate social responsibility and strategic communication to frame themselves as indispensable development partners and to mobilize political allies against the Bill. They foregrounded support to farmers, road safety and other development initiatives, and close partnerships with government. The experience illustrates why clear conflict-of-interest rules, transparency requirements and firewalled consultations need to apply across all sectors and parliamentary processes during legislative drafting, not only within the health ministry *(8)*.

Box 13. Learning from a lapse: alcohol sponsorship in Nepal

Nepal has some of South Asia's strongest statutory bans on alcohol advertising, yet enforcement has been uneven: sponsorship of sports events and football teams by multinational alcohol brands continued across media and venues. In response, CSOs – notably the Forum for Protection of Consumer Rights Nepal – petitioned the Supreme Court, which in 2023 issued an interim order upholding the legal ban and instructing government bodies to monitor and enforce compliance. The case shows that adopting a ban is not the end of the work: partnerships between industry and popular institutions can quietly undermine a SAFER intervention unless monitoring, enforcement and independent watchdog scrutiny are built in *(8)*.

4.11 From the action plan's refrain clauses to government practice

The Global alcohol action plan 2022–2030 includes, in each of its six action areas, a statement addressed to economic operators in alcohol production and trade *(2)*. Each contains refrain clauses, endorsed by all Member States, that spell out what economic operators should abstain from. Taken together, the refrain clauses are the closest thing international alcohol policy has to an agreed, operational description of industry conduct that is incompatible with protected policy-making. The six statements are summarized below.

Action area 1 – Implementation of high-impact strategies and interventions

Economic operators are called on to focus on measures that reduce the harmful use of alcohol within their core roles – as developers, producers, distributors, marketers and sellers of alcoholic beverages – and to refrain from interfering with alcohol policy development and from activities that prevent, delay or stop the development, enactment, implementation and enforcement of high-impact strategies and interventions to reduce the harmful use of alcohol.

Action area 2 – Advocacy, awareness and commitment

Operators are invited to strengthen their contribution to reducing the harmful use of alcohol within their core roles and to take concrete steps towards eliminating the marketing and advertising of alcoholic products to minors. Where relevant, they should develop and enforce self-regulatory measures on marketing and advertising in conjunction with statutory frameworks; enable easily understood consumer information on labels (including composition, age limits, health warnings and contraindications); refrain from promoting drinking; eliminate and prevent positive health claims related to alcohol; and ensure responsible practice within regulatory or co-regulatory frameworks.

Action area 3 – Partnership, dialogue and coordination

Operators are invited to substitute, where possible, higher-alcohol products with no- or low-alcohol alternatives in their portfolios – while avoiding the circumvention of regulation and the targeting of new consumer groups. Operators in alcohol production and trade and in related sectors (retail, advertising, hospitality, tourism, social media and communication) are encouraged to contribute to the elimination of marketing and sales to minors and other high-risk groups.

Action area 4 – Technical support and capacity-building

Operators are invited to implement capacity-building within their core roles and sectors, and to refrain from engagement in capacity-building outside their core roles that may undermine public health community activity.

Action area 5 – Knowledge production and information systems

Operators are called upon to disclose, with due regard for the limitations of commercial confidentiality, data of public health relevance – including methodology – to improve WHO estimates of alcohol consumption.

Action area 6 – Resource mobilization

Operators are invited to allocate resources for measures that reduce harmful use of alcohol within their core roles; to refrain from funding public health and policy-related activities and research to avoid bias in agenda-setting from the underlying conflict of interest; and to cease sponsorship of scientific research on the public health dimensions of alcohol consumption.

The action plan addresses these clauses to the operators; it does not instruct governments, and this handbook does not claim otherwise. Endorsement nonetheless changes what a government can do with the text. Because every Member State has endorsed a statement of what operators should refrain from, a government that declines to accept precisely those behaviours is not improvising a national standard: it is applying a boundary that its own government, along with every other, has already drawn. Table 24 translates each refrain clause into the corresponding government-side practice, in that spirit and without claiming more than the endorsed text supports. The disclosure clause in action area 5 is a positive ask rather than a refrain and is served by the monitoring provisions of Part 2.

Table 24. The action plan's refrain clauses translated into government practice

Derived from the statements addressed to economic operators in the Global alcohol action plan 2022–2030 (2); refrain clauses paraphrased.

Action area (clause)	What operators are asked to refrain from	The corresponding government practice
1 – High-impact strategies (1.4.1)	Interfering with alcohol policy development; activities that might prevent, delay or stop the development, enactment, implementation and enforcement of high-impact strategies and interventions.	Develop policy in spaces from which operators are excluded; treat attempts to prevent, delay or weaken measures as interference to be documented, disclosed and set aside rather than negotiated; protect the policy timetable (sections 4.5 to 4.8).
2 – Advocacy and awareness (2.4.1)	Promoting drinking; making positive health claims for alcohol.	Accept no operator material that promotes drinking in any setting under government influence, including school and community education; prohibit health claims in labelling and marketing by statute rather than self-regulation.
3 – Partnership and coordination (3.4.1)	Circumventing regulations for alcoholic beverages; reaching minors, other high-risk groups or new consumer groups with marketing, advertising and promotion.	Monitor and enforce against circumvention, including brand-sharing and promotion through no- and lower-alcohol product lines; close the regulatory gaps that permit marketing to reach minors, high-risk or new consumer groups.

Action area (clause)	What operators are asked to refrain from	The corresponding government practice
4 – Technical support and capacity-building (4.4.1)	Capacity-building outside operators' core roles that may undermine or compete with the activities of the public health community.	Source training and capacity-building for officials, enforcement staff and the health workforce from public health channels; decline operator-provided training, secondments and technical support (section 4.9).
6 – Resource mobilization (6.4.1)	Funding public health and policy-related activities and research; sponsoring scientific research on the public health dimensions of alcohol and using it for marketing or lobbying purposes.	Accept no operator funding for public health or policy work; screen the provenance of evidence used in policy processes and, where consistent with applicable law and policy, give operator-sponsored science on the public health dimensions of alcohol no standing in them (section 4.7).

Each practice in Table 24 runs through the instruments of this Part – exclusion from policy development, the meeting checklists, the interaction log and the funding safeguards – and applies at community scale through the industry-interference protocol of the Community SAFER Plan (Part 5 and Annex 10).

4.12 Conclusion

The SAFER interventions offer the most cost-effective, evidence-based tools to reduce alcohol-related harm at population level. They will succeed only if public health policy-making is protected from undue influence by commercial interests. The principles, steps and checklists in this part provide a practical starting point. Countries are encouraged to adapt them to context and to share experience with WHO and partners so that future iterations of this guidance reflect accumulated practical learning. The community-level companion is in Part 5 section 5.6 and Part 6; an adaptable workplace alcohol guidance template is provided in Annex 12.

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[6] All web links in this list were accessed on 7 July 2026.

Further reading

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PART 5

Community-level implementation of SAFER

PART 5
Community-level implementation of SAFER

Parts 1 to 4 of this handbook describe how the SAFER package is established, monitored, financed and protected at country level. Part 5 addresses how the same package is translated into action at the community level – across the range of subnational governance arrangements found in WHO Member States.

National policy is necessary but not sufficient. The implementation, monitoring and enforcement of alcohol policy measures takes place largely in communities – in towns, cities, districts and provinces; in licensing decisions, primary-care encounters, road traffic enforcement and school environments. Equally, much of the social context that shapes drinking is experienced at community level: outlet density, marketing exposure, and the visibility of alcohol in public space and in the lives of children. National laws define what communities can do; community delivery determines whether those laws have any effect; and the evidence and political momentum generated by community work are among the principal forces shaping national policy reform. Sustained progress requires the national and community levels to operate as a coherent system. That system is wider than government: the communities in which policy takes effect, civil society and people affected by alcohol harms are the constituency whose trust and ownership sustain alcohol policy across electoral cycles, at every level at which government formulates and implements it.

Part 5 is structured around the same architecture as the rest of this handbook. Sections 5.4, 5.5 and 5.6 apply the three SAFER strategies – Implement, Monitor and Protect – to the community level. Section 5.7 sets out the practical pathway by which a community moves from political mandate to delivered, monitored and protected action. Section 5.8 addresses resource mobilization. Section 5.9 presents the Building SAFER Communities project in Ireland (2024–2026) as a contemporary case study. Section 5.10 condenses the critical success factors and common pitfalls. Section 5.11 concludes. Workplaces and institutions – including community authorities themselves, the United Nations system and other large public-sector employers – are addressed separately in Part 6 of this handbook.

Cross-references. Part 5 builds on Parts 1 to 4 throughout. Module 1 (landscape reports) provides the analytical foundation that community plans should also use; Module 3 (the Roadmap) defines the national plan within which community work must be visibly nested; Part 2 provides the global monitoring architecture against which community-level monitoring is calibrated; Part 3 provides the approach that community-level economic assessments and investment cases adapt; Part 4 provides the conflict-of-interest principles that community-level COI work operationalizes. The annexes supporting Parts 5 and 6 (Annexes 8 to 12) provide community-level templates and, in Annex 12, a model workplace alcohol guidance supporting Part 6.

5.1 Introduction

Alcohol is a leading preventable cause of death, disease and social harm worldwide, responsible for an estimated 2.6 million deaths globally in 2019; the full burden statistics are set out in the Introduction. Progress has been uneven across WHO regions and unequal between high- and low-income settings *(1, 2)*.

The *Global alcohol action plan 2022–2030*, endorsed by the Seventy-fifth World Health Assembly, builds on the target areas for policy measures and interventions set out in the Global strategy to reduce the harmful use of alcohol, in which community action sits alongside nine other action areas *(3)*. The Plan emphasizes empowerment of people and communities and recognizes that local government – provincial, municipal and submunicipal – has direct competence over many of the levers that determine the level and pattern of alcohol consumption and harm. Box 14 summarizes the case.

Box 14. Why the community level matters

Communities are regulators. Many of the levers that shape alcohol availability – licensing of premises, opening hours, outlet density, public-space byelaws, advertising on public property – sit at subnational level in most jurisdictions.

Communities are large employers. Provincial and municipal authorities, school systems, health services and public utilities are typically among the largest employers in their territories; their workplace alcohol policies reach many thousands of staff and signal expectations to other employers.

Communities are service providers. Primary care, social services, road and traffic operations, schools and recreation are typically delivered or co-commissioned at subnational level.

Communities are convenors. Subnational authorities can bring together police, justice, health, civil society, business and citizens in ways that national ministries cannot.

Communities can act when nations cannot, or do so faster. Community action is often more visible and accountable to citizens than national action, and can build the evidence and the political momentum that subsequently shape national policy.

Part 5 is intended for: provincial, district and municipal officials with policy or operational responsibility for alcohol-related matters; health departments at subnational level; police, justice and licensing authorities; community-based organizations and civil-society partners; WHO country offices and partners supporting Member States with subnational implementation; and people with lived experience of alcohol harm whose participation in the design and oversight of community SAFER work is increasingly recognized as essential.

It builds on three decades of community-level alcohol action and on contemporary practice. That experience includes the WHO Healthy Cities movement, which has supported municipalities worldwide to put health on the political agenda, and the Multi-City Action Plan (MCAP) on Alcohol of the WHO Regional Office for Europe in the 1990s. The Building SAFER Communities project in Ireland (2024–2026), which translates the SAFER package into 15 community-level actions delivered in 10 communities *(4, 5)*, is presented in detail in section 5.9 as one of the first national, multi-site translations of SAFER into a coherent community-level demonstration *(6)*.

5.2 Defining the community level

In this handbook, "community" refers primarily to a level of governance and implementation, not only to a population group, neighbourhood identity or civil-society constituency. The relevant level may be provincial, district, municipal, submunicipal or another statutory coordination structure, depending on where alcohol-relevant powers and responsibilities sit.

"Community" in this part is therefore used as the umbrella term for any subnational level at which SAFER actions can be planned, implemented and monitored. In practice it covers a wide spectrum of governance arrangements (Table 25):

Table 25. Tiers of subnational governance and typical alcohol-relevant competencies

Exact powers vary substantially across countries and should be mapped during the preparation of the Community SAFER Plan.

Tier	Examples (terminology varies by country)	Typical alcohol-relevant competencies
Provincial/state/regional government	States in federal systems; provinces; regions; oblasts; departments.	Often share or hold legislative powers over alcohol availability, licensing frameworks, school health curricula, regional public health surveillance and enforcement coordination.
District/county government	Districts; counties; metropolitan areas; second-tier authorities.	Licensing administration; coordination of police, health and social services; oversight of municipal authorities; data aggregation.
Municipal/city government	Cities; towns; municipalities; town councils.	Licensing decisions on individual premises; outlet density and zoning; opening hours; advertising on municipal property; municipal workplace policies; recreation and event management; primary-care commissioning in some jurisdictions.
Submunicipal/local council	Wards; parishes; barangays; communes; neighbourhoods; village councils.	Community engagement in licensing objections; community representation on coordinating structures; local enforcement of byelaws; neighbourhood-level prevention activities.
Statutory community-coordination structures	Drug and Alcohol Task Forces (Ireland); community drug-and-alcohol boards; municipal health councils; district AIDS/NCD committees in some African and Asian settings.	Multisectoral coordination and community-level planning; bridge between statutory and voluntary sectors; in some jurisdictions, a delegated budget and accountability for drug- and alcohol-related action.

Three points follow from this variation:

- **Match the action to the competence.** An action that requires a licensing decision must be planned at the tier that holds licensing powers. An action that requires a national legislative change cannot be delivered by a community alone – but the community can advocate for it, generate the local evidence to support it, and prepare to enforce it once enacted.

- **Build coalitions across tiers.** Most community SAFER work involves more than one tier: a city plan needs provincial enforcement support; a provincial plan needs municipal partnerships to deliver. Effective community SAFER implementation is multi-tier by design.

- **Link explicitly to the national plan.** Community SAFER work should be visibly nested within the SAFER Roadmap (Module 3 of Part 1) so that community actions contribute to – and are recognized in – national reporting against the *Global alcohol action plan 2022–2030* and SDG target 3.5.

5.3 The community SAFER framework: five intervention areas, three strategies

Community-level SAFER implementation rests on the same architecture as the national framework: the five SAFER intervention areas, delivered through the three SAFER strategies of Implement, Monitor and Protect *(7, 8)*. The three strategies are not phases of work but parallel and continuous functions; they are addressed in turn in sections 5.4, 5.5 and 5.6 (Table 26).

Table 26. The three SAFER strategies, applied at community level

Strategy	Meaning at community level	Where addressed in this part
Implement	Translate each of the five SAFER intervention areas into community-level actions, matched to the competencies that exist at provincial, municipal and local-council level. Mobilize the resources, capacities and partnerships needed for delivery.	Section 5.4
Monitor	Track the inputs, outputs, outcomes and impact of community SAFER work using indicators that are aligned with – and feed into – the global monitoring architecture (Global alcohol action plan, GISAH, SDG 3.5, Global Monitoring Framework for NCDs). At community level, this is operationalized through the WHO *City-level monitoring guidance for the prevention and control of noncommunicable diseases and injuries* (2023) and its 2024 assessment instrument *(9, 10)*.	Section 5.5
Protect	Safeguard community-level alcohol policy from interference by the alcohol industry and related commercial interests, including in sponsorship of community events, partnerships with municipal authorities and engagement with subnational steering groups *(3)*.	Section 5.6

Sections 5.4 to 5.6 describe each strategy in turn. Workplaces and institutions are addressed separately in Part 6.

5.4 Implement: translating the five SAFER areas into community action

This section describes how each of the five SAFER intervention areas can be translated into community-level action. The actions are drawn from accumulated international experience, and have been generalized so that they can be adapted across community types and tiers. That experience includes the WHO Regional Office for Europe MCAP cities of the 1990s, the WHO Healthy Cities movement globally, the Building SAFER Communities project in Ireland (section 5.9), and contemporary multi-country initiatives in low- and middle-income countries *(7, 11–13)*.

Not all actions are appropriate or feasible everywhere. Communities should select and sequence actions according to local need, mandate, capacity and the tier of governance that holds the relevant competence. Each table below identifies the partners and competencies typically needed to deliver each action.

5.4.1 S – Strengthen restrictions on alcohol availability

Many of the actions in this section involve operational engagement with the hospitality sector, alcohol retailers and licensed-premises operators – for example, on test-purchasing, server training, compliance inspections and licensing-hearing representations. Such operational contact is appropriate and often necessary for enforcement of statutory rules. It is distinct from policy partnership. The alcohol or hospitality sectors should not participate in setting public health objectives, designing the Community SAFER Plan, framing priorities or shaping the monitoring architecture. The distinction is set out in detail in Part 4 section 4.5.2 and should be applied throughout this section.

Availability theory, supported by a substantial international evidence base, holds that elevated levels of physical and social availability of alcohol increase both rates of consumption and consequent harm *(14)*. Communities have direct or shared powers over many of the levers that govern availability: licensing of premises, opening hours, outlet density and zoning, public-space byelaws, enforcement of minimum-purchase-age provisions and standards of service in licensed premises. Restrictions on availability are particularly important where outlet density is high, where unrecorded or informally produced alcohol contributes substantially to overall consumption, or where vulnerable groups (children, young people, populations affected by violence) are disproportionately exposed.

Unrecorded, informal, illicit and surrogate alcohol require context-specific public health, enforcement and safety responses. However, concern about unrecorded alcohol should not be used to delay evidence-based measures addressing the recorded commercial market, including taxation, availability restrictions, marketing controls and drink–driving countermeasures.

Suggested community-level actions for this intervention area are set out in Table 27.

Table 27. Suggested community-level actions for SAFER intervention area S – Strengthen restrictions on alcohol availability

Menu of suggested community-level actions for intervention area S (availability), with the partners and competencies each requires.

Action	Description	Tier and partners typically needed
S1. Education on the availability–harm relationship	Brief community members, elected representatives and inter-agency stakeholders on the evidence linking outlet density, opening hours and licensing practice to consumption and harm; integrate alcohol awareness into induction for new councillors and officials.	Any tier; community SAFER coordinator with public health team.
S2. Mapping outlet density over time	Map the number, type and geographic distribution of alcohol licences; relate, where data permit, to indicators such as ambulance call-outs, alcohol-related assaults, hospital emergency-department attendances and public-disorder incidents.	Municipal/district licensing authority; police; public health authority; academic partner.

Action	Description	Tier and partners typically needed
S3. Capacity-building of community and law-enforcement on licensing law	Provide training and accessible resources on the licensing regime to community organizations, civil society and police, so that they can engage effectively in licensing decisions and detect and report breaches. The Building SAFER Communities project in Ireland supports a specialized certificate in alcohol licensing law, developed as a collaboration between Well Ireland (formerly Alcohol Forum Ireland), the Irish Community Action on Alcohol Network and the Atlantic Technological University (ATU). This stand-alone module complements the Level 7 Certificate in Licensing Law and Practice offered by the Department of Law, Public Service and Education, and is delivered to officers of An Garda Síochána and to community workers *(4, 5)*.	Provincial or municipal legal team; police training; academic partner.
S4. Active community engagement in licensing decisions	Use existing rights of objection, representation and consultation in licensing hearings; develop a plain-language Community Guide to Alcohol Licensing for residents.	Municipal legal team; community organizations; civil society.
S5. Enforcement of minimum legal purchase age	Test purchasing and age-verification activities in cooperation with police, regulators and licensees; publish measurable targets and outcomes for enforcement actions.	Police; licensing authority; trading-standards body.
S6. Responsible-server training and accreditation	Deliver structured training to bar, off-trade and event staff covering refusal of service to under-age and intoxicated patrons, conflict management and basic alcohol education. National responsible-server training programmes are operating or in development in many countries; a national manual is being developed under the Building SAFER Communities project in Ireland *(4, 5)*.	Hospitality sector partners; training providers; licensing authority.
S7. Restrictions on alcohol use in public space	Where appropriate and lawful, designated alcohol-free zones (parks near schools, transport interchanges, family-event venues, religious or cultural sites); review and update local byelaws.	Municipal legal team; police; community organizations.
S8. Action on unrecorded and informally produced alcohol	Where unrecorded or informally produced alcohol is a substantial part of the local market, work with national authorities, primary care and community organizations on safety, surveillance and harm reduction; do not allow this issue to be used to delay action on the recorded market *(11)*.	Provincial/national authorities; revenue and customs; community organizations; primary care.

Among the structural options for limiting alcohol availability, several Member States operate a public retail monopoly on the off-trade sale of alcohol – wholly (as in Sweden, Norway, Finland, Iceland and several Canadian provinces) or for specific beverage categories (as in several states of the United States of America and historically in other jurisdictions). The international evidence on retail monopoly indicates substantial effects on consumption and harm, attributable principally to the removal of commercial pressure to maximize sales volume, to the slower introduction of new products and price-promotional mechanics, and to the consistent enforcement of minimum-age and licensing rules *(14, 15)*. Retail monopolies are not appropriate or feasible in all national contexts, and this handbook does not recommend any single structural model. Where countries are actively considering stronger structural controls on availability, the retail monopoly option should be on the menu of structural choices, alongside outlet density controls, hours of sale, minimum legal purchase age, licensing and the online-sales measures set out below. This applies particularly to countries facing rising harm associated with rapid commercialization of the alcohol retail environment.

Online sales, home delivery and platform retailing

Online alcohol sales and rapid-delivery channels are an increasingly central availability question in many markets. They cut across all five SAFER intervention areas: availability; drink–driving, through delivery to drivers; advertising, through platform-based and influencer marketing; pricing, through promotional pricing on platforms; and screening. They belong primarily under availability, because they expand the channels through which alcohol is physically obtained. Five issues deserve specific attention at community level.

Minimum-age verification at the point of delivery

Where alcohol is delivered to a residential address, the operative age-verification point shifts from the retail premises to the doorstep. Effective regimes require: identity check by the delivery agent at the point of handover; refusal protocols; mandatory training of delivery agents; and platform-level liability for delivery to minors. Communities can document compliance rates through test-purchase activity covering the delivery channel, not only the on-premises and off-premises retail channels.

Application of opening-hours restrictions to online and rapid-delivery channels

Statutory restrictions on hours of alcohol sale lose force if online ordering with delivery in the same time window is permitted outside those hours. Communities should advocate, where the legal power sits at subnational level, for the application of hours-of-sale restrictions to the entire transaction chain – order, payment, and delivery – not only to physical retail.

Application of marketing and pricing rules to online channels

Statutory marketing restrictions, minimum unit pricing and promotional-pricing prohibitions need to apply explicitly to online and platform channels, including to dynamic pricing, recommendation algorithms, loyalty programmes, basket discounts and “buy more, save more” promotions that operate within the platform interface. Cross-reference section 5.4.4 on advertising and section 5.4.5 on pricing.

Cross-border distance-selling

Cross-border online alcohol sales – particularly between jurisdictions with differing tax and availability regimes – can substantially weaken national alcohol policy by routing consumption through the regulatory weakest link. Community-level data on cross-border purchasing can support national policy work on bilateral or regional agreements, on platform liability, on customs enforcement of personal-import limits, and on application of the destination-country tax regime to distance sales.

Application of monopoly or licensing rules to online channels

Where the country operates a retail monopoly or a restricted-licence regime, the application of these structures to online sales is a critical policy question. Approaches in monopoly jurisdictions have included: extension of the monopoly to online sales operated by the monopoly itself; prohibition of third-party online retail; and platform-level enforcement of licensing rules. The specific structure is a national policy matter; community authorities should be aware of which regime applies and ensure that licensing-compliance work covers it.

These five issues are particularly visible in the Nordic, Baltic and several Western European markets in 2024–2026 but are emerging across most regions. Community-level monitoring data on each is a valuable input to national policy reform.

5.4.2 A – Advance and enforce drink–driving countermeasures

Drink–driving countermeasures are among the most cost-effective alcohol policy interventions globally *(7, 14)*. Most are anchored in national legislation, but their operational delivery is largely subnational: enforcement intensity, location, timing and communication are decided and resourced at the level of provincial and municipal road-safety and law-enforcement authorities.

Suggested community-level actions for this intervention area are set out in Table 28.

Table 28. Suggested community-level actions for SAFER intervention area A – Advance and enforce drink–driving countermeasures

Menu of suggested community-level actions for intervention area A (drink–driving), with the partners and competencies each requires.

Action	Description	Tier and partners typically needed
A1. Community–police partnership on drink–driving enforcement	Joint plan for visible enforcement at known high-risk times and locations (weekends, late-night periods, post-event hours); quarterly review of enforcement data; integration with the community road-safety strategy *(4, 5)*.	Police; road-safety authority; municipal traffic department.
A2. Local data and communication on alcohol-related road harm	Document and publicize alcohol-related fatal and non-fatal road traffic injuries in the community; targeted campaigns timed to major events and seasonal peaks.	Public health team; road-safety authority; communications office.
A3. Safe alternatives to drink–driving	Subsidy, scheduling or licensing terms that improve night-time public transport; designated-driver and ride-share partnerships; review of taxi availability around licensed venues.	Municipal transport authority; licensing authority; hospitality sector.
A4. Pre-journey screening in at-risk professional groups	Where authorized, support compulsory pre-journey alcohol screening for professional drivers (taxi, public transport, freight); the Kyiv Sociotherapy Corporation, based in Kyiv, Ukraine, pioneered this in cooperation with traffic police in 1995 *(11)*.	Provincial transport authority; employers; primary care; police.

5.4.3 F – Facilitate access to screening, brief interventions and treatment

Where responsible-server training is delivered to hospitality-sector workers (see section 5.4.1, action S6), the training is a regulatory and workforce-capacity intervention, not a partnership with the hospitality sector on public health objectives. The training content, accreditation framework, evaluation and oversight should sit with the public health authority and the licensing authority, not with the hospitality-sector trade body. Operational coordination on training logistics is appropriate; policy partnership is not. See Part 4 section 4.5.2.

Brief interventions delivered by primary-care providers – including general practitioners, practice nurses, occupational-health staff, mental-health workers and emergency-department clinicians – are evidence-based, cost-effective and deliverable at scale *(7, 14)*. Communities are the level at which primary care is most often organized, commissioned and delivered. Engaging primary care in alcohol work has, however, been a recurring implementation challenge across 30 years of community alcohol projects worldwide; effective community action treats this as a system-design problem rather than a training problem *(4, 5, 11)*.

That system-design problem has a specific and recurring form. Where screening is introduced without a visible referral pathway, providers disengage from screening, because asking the question leads nowhere for the patients who need help most. Community-level SBI should therefore be planned together with the referral and treatment pathways set out in action F3, and not ahead of them.

Suggested community-level actions for this intervention area are set out in Table 29.

Table 29. Suggested community-level actions for SAFER intervention area F – Facilitate access to screening, brief interventions and treatment

Menu of suggested community-level actions for intervention area F (screening, brief interventions and treatment), with the partners and competencies each requires.

Action	Description	Tier and partners typically needed
F1. Brief-intervention training across primary care	Roll-out of a structured SBI model across primary-care, occupational-health and emergency-department staff. Country-adapted models include the SAOR (Support, Ask & Assess, Offer Assistance, Referral) model used by the Health Service Executive in Ireland *(4, 5)*.	Public health authority; primary-care directorates; training partner.
F2. Integration into routine contacts	Embed alcohol screening and brief advice into routine health and social-care contacts using a Making Every Contact Count or equivalent framework, so that screening is part of a wider health-conversation approach rather than a stand-alone activity *(4, 5)*.	Public health authority; primary-care providers.
F3. Visible referral and treatment pathways	Maintain up-to-date, accessible, multilingual information on local alcohol-treatment services for patients, families, employers, courts and emergency services; publish pathways for vulnerable groups (pregnant women, people experiencing homelessness, people in contact with the criminal-justice system).	Public health authority; treatment services; community organizations.
F4. Integration with mental-health and chronic-disease care	Co-locate or formally link alcohol services with mental-health, HIV, tuberculosis and noncommunicable-disease services where the evidence base supports doing so; reduce stigma and fragmentation.	Public health authority; clinical services; primary care.
F5. Workforce planning and ongoing professional development	Plan adequate workforce capacity and continuing professional development; target identified gaps (often emergency department staff, social workers, occupational-health workers).	Public health authority; professional bodies; training institutions.

5.4.4 E – Enforce bans or comprehensive restrictions on advertising, sponsorship and promotion

Comprehensive restrictions on alcohol advertising, sponsorship and promotion are among the most cost-effective alcohol policy measures and are increasingly required to address the digital marketing environment in which children and adolescents are exposed *(3, 7)*. Most statutory advertising restrictions are national. Communities nevertheless have meaningful operational levers – over their own assets, over events they license or sponsor, and over compliance monitoring – and can play an important role in surfacing breaches and protecting child-rich environments.

Co-regulation in this handbook refers to a regime in which the binding marketing rules are set in statute and enforced by an independent public regulator, with industry compliance arrangements operating within that statutory framework. Industry-led self-regulation operating outside or in place of statutory rules is not co-regulation as the term is used here and is not adequate for SAFER purposes.

Where evidence indicates that a co-regulatory regime is not delivering the marketing-restriction objective in practice, countries are encouraged to move to comprehensive statutory restrictions rather than to invest further in strengthening the co-regulatory regime. Indications include persistent advertising breaches, low complaint upheld rates, weak or non-deterrent sanctions, narrow scope of the regime, and industry capture of the regulatory body.

Suggested community-level actions for this intervention area are set out in Table 30.

Table 30. Suggested community-level actions for SAFER intervention area E – Enforce bans or comprehensive restrictions on advertising, sponsorship and promotion

Menu of suggested community-level actions for intervention area E (advertising, sponsorship and promotion), with the partners and competencies each requires.

Action	Description	Tier and partners typically needed
E1. Community engagement in identifying and reporting advertising breaches	Where comprehensive restrictions exist, build community capacity to identify and report breaches via the relevant national channel. The Building SAFER Communities project in Ireland builds community capacity to identify breaches of the Public Health (Alcohol) Act 2018 – for example near schools, on public transport and in children's media *(4, 5, 16)*.	Public health authority; community organizations; civil society.
E2. Awareness of digital marketing harms	Raise awareness among parents, schools, youth services and young people of the volume and design of online alcohol marketing; integrate into school health curricula and community campaigns *(4, 5)*.	Education authority; youth services; communications office.
E3. Removal of alcohol advertising from public assets	Prohibit alcohol advertising on community-owned bus shelters, transport vehicles, advertising sites, sports facilities and event venues; align with bans on tobacco advertising *(11)*.	Municipal estates/ advertising contracts; transport authority.
E4. Decline alcohol industry sponsorship of community activities	Adopt a policy refusing alcohol industry sponsorship, in-kind contributions or funding for any event, activity or publication controlled, owned or supported by the community authority. The prohibition extends to sponsorship offered through no-alcohol or low-alcohol product lines where these use the same or confusingly similar branding, naming, logo, colour scheme or imagery as the alcoholic parent product (see explanatory paragraph below).	Community executive; communications office.
E5. Protection of school and child-rich environments	Audit alcohol-related marketing exposure in routes to school, around schools, sports facilities and youth-centred events; advocate for and enforce buffer zones where these exist in national law *(5)*.	Education authority; municipal planning; police.

Brand-extension sponsorship deserves specific attention. In recent years, the alcohol industry has substantially expanded its no-alcohol and low-alcohol product lines, marketed under the same brand names, logos, packaging and visual identity as the alcoholic parent products. The public health argument for these products as standalone goods is reasonable; the marketing function of these products as

brand extensions is well-documented *(17, 18)*. When a no-alcohol beer or a 0.0% spirit branded identically to its alcoholic counterpart appears as event sponsor or visual branding in public space, the marketing reach extends to the alcoholic parent product whether or not the alcoholic product is physically present. Community authorities, public-sector employers and the institutions covered in Part 6 of this handbook should therefore treat brand-extension sponsorship as covered by the alcohol industry sponsorship prohibition. This is consistent with the treatment in Annex 12.

5.4.5 R – Raise alcohol prices through excise taxes and pricing policies

Pricing is the most evidence-based of the SAFER intervention areas: increases in alcohol price reduce both consumption and harm, and minimum unit pricing is a particularly effective instrument for reducing the consumption of cheap, high-strength alcohol disproportionately consumed by heavier drinkers and by young people *(7, 14)*. Excise taxation and minimum unit pricing are national competences in nearly all jurisdictions; the community role is therefore one of advocacy, evidence-generation and consistent practice within the levers communities do hold. Those levers are wider than is often assumed: in many jurisdictions communities can monitor and support enforcement of restrictions on price promotions and discounting, use licensing conditions to address high-risk pricing practices in licensed venues, act on pricing in the night-time economy, and generate the local price and affordability evidence on which national taxation advocacy depends (Table 31) *(7, 14)*.

Community-level pricing action matters even where taxation is entirely national. National duty increases and price floors are undermined locally when deep discounting, multi-buy promotions and below-cost selling go unmonitored; the acute harms of the night-time economy concentrate in a small number of venues and hours in which price is one of the strongest short-term levers; and evidence on the affordability of the cheapest alcohol is generated, or missed, at community level.

Table 31. Suggested community-level actions for SAFER intervention area R – Raise alcohol prices

Menu of suggested community-level actions for intervention area R (pricing), with the partners and competencies each requires.

Action	Description	Tier and partners typically needed
R1. Local advocacy for national price reform	Council and provincial-government resolutions and joint advocacy with national public health bodies in support of inflation-indexed excise duty, removal of subsidies and (where appropriate) minimum unit pricing. Underpin this advocacy with locally generated evidence: periodic surveys of the cheapest alcohol available in the community, tracking of affordability relative to local incomes, and documentation of the depth and frequency of price promotions, reported through the monitoring framework in section 5.5.	Provincial/municipal executive; public health authority.
R2. Community understanding of pricing measures	Public-information work to explain pricing measures to communities, to counter industry framing, and to build civic support for evidence-based pricing. The Building SAFER Communities project in Ireland delivers community education on minimum unit pricing as one of its 15 core actions *(4, 5)*. *In Nepal, the Forum for Protection of Consumer Rights has assessed the implementation of pricing and other SAFER measures in five municipalities from a consumer-protection standpoint, showing how civil-society monitoring can complement official public-information work (19)*.	Public health authority; community organizations.

Action	Description	Tier and partners typically needed
R3. Responsible pricing in venues under community control	Where the community authority has control over pricing (recreation centres, community halls, civic events, leisure facilities), ensure that low-alcohol and non-alcoholic drinks are competitively priced and that volume promotions are not used *(11)*.	Municipal recreation; events; estates.
R4. Local evidence on the impact of price measures	Where national price measures have been introduced, contribute to the local evidence base on impact, using routine surveillance data (consumption surveys, hospital admissions, ambulance call-outs); cooperate with academic partners on natural-experiment evaluations.	Public health authority; academic partner.
R5. Monitoring and enforcement of price-promotion restrictions	Where national or subnational rules restrict price promotions, discounting or below-cost selling, monitor compliance in retail outlets and licensed venues, provide accessible routes for reporting breaches, and support the responsible enforcement authority. Where no such restrictions exist, document promotional practices systematically as evidence for advocacy under R1 *(7, 14)*.	Licensing authority; trading standards/ consumer protection; police; public health authority.
R6. Pricing conditions in licensing and the night-time economy	Where the licensing framework permits conditions on pricing practice, work with the licensing authority to address high-risk practices such as all-you-can-drink offers, deep late-night discounting and quantity-based deals; in night-time economy zones, combine licensing conditions with venue codes of practice agreed with hospitality operators, police and transport partners (see section 5.4.6) *(14)*.	Licensing authority; municipal executive; police; hospitality-sector forum.

5.4.6 Cross-cutting community priorities

Several of the most consequential alcohol-related issues at community level sit across the SAFER areas rather than within any one. Three deserve specific attention in community planning.

- **Children, adolescents and the developing brain.** Alcohol use in adolescence is shaped by physical availability, marketing exposure and adult role-modelling – addressable through SAFER areas S, E and (indirectly) F. The Letterkenny SAFER steering group in Ireland framed this in operational terms: "can young people walk to school or to local shops without being bombarded by alcohol branding and product display?" The group also drew attention to widespread parental under-recognition of the neurotoxic effects of alcohol on the developing brain *(5)*. Many countries also report high rates of fetal alcohol spectrum disorders. These issues call for explicit cross-area action plans for children, adolescents and pregnant women.

- **Equity and the social gradient of harm.** Alcohol harm is unequally distributed: lower-income communities and neighbourhoods often have higher outlet densities and experience higher harm at similar levels of consumption. SAFER actions should be designed to reduce, not widen, those gradients – for example by prioritizing enforcement and licensing review in neighbourhoods with concentrated harm, while ensuring brief-intervention and treatment services are accessible to under-served groups.

- **The night-time economy.** Many alcohol-related harms cluster in night-time economy zones. Effective community action requires partnerships with hospitality operators, transport providers, police and emergency services that go beyond traditional licensing – covering pre-loading, discount practices, last-orders timing, transport availability and the design of public space around licensed clusters. Pricing belongs in these partnerships: licensing conditions on late-night discounting and venue codes

of practice on promotions are night-time economy instruments as much as policing and transport planning are (Table 31, actions R5 and R6).

Partnerships in the night-time economy involve operational coordination between police, transport, emergency services, public health and hospitality operators. The operational coordination is appropriate; it is distinct from policy partnership. Hospitality-sector operators do not have a role in setting the night-time economy strategy or in framing community alcohol policy. See Part 4 section 4.5.2.

5.4.7 Digital marketing and youth exposure

Community authorities should consider how children and adolescents are exposed to alcohol marketing through digital and social-media platforms, influencer activity, event sponsorship, sports-related promotion and outdoor advertising linked to digital campaigns. Where national restrictions exist, community actors can support monitoring, reporting of breaches and protection of school routes, youth services and child-rich settings.

Where national restrictions are incomplete, community evidence on youth exposure can support advocacy for stronger national regulation. Community-level monitoring should record the type of exposure observed, the setting, the apparent audience and the reporting pathway used, while avoiding collection of unnecessary personal data from children or young people.

5.5 Monitor: community-level monitoring, evaluation and learning

Communities that measure systematically learn faster, attract sustained political and financial support, and contribute to the wider evidence base on community SAFER action. Communities that do not measure tend to overstate process activity (training delivered, materials disseminated) and understate the structural changes that actually reduce consumption and harm. A standardized local indicator set, anchored in the WHO city-level monitoring tools, is an effective safeguard against that pattern.

Cross-reference. Section 5.5 is the community-level companion to Part 2. The architecture and definitions in Part 2 apply equally at community level; this section adds the operational scaffold for subnational application.

5.5.1 The WHO City-level monitoring guidance and assessment instrument

In December 2023, WHO published the *City-level monitoring guidance for the prevention and control of noncommunicable diseases and injuries (9)*. It is the first WHO normative instrument to bring the monitoring of policy interventions for NCDs and injuries – including alcohol control – to the city and community level using a common, standardized set of indicators. The guidance was developed by the NCD Surveillance, Monitoring and Reporting Unit at WHO headquarters, in collaboration with other WHO technical departments, and was piloted across 20 cities in all six WHO regions between 2021 and 2023 *(9, 20)*. In October 2024, WHO released an Excel-based assessment instrument that operationalizes the guidance and enables city-level authorities to generate the status of their indicators across all or selected modules *(10)*.

The guidance covers eight policy areas in which city-level action is considered feasible: air pollution reduction, alcohol control, overdose prevention, promoting healthy diets, road safety, safe walking and cycling, tobacco control, and NCD and injury surveillance. For community-level SAFER implementation, the alcohol-control module – together with the road-safety, healthy-diets and surveillance modules where they overlap – provides the primary monitoring scaffold *(9, 10)*. Box 15 sets out the rationale.

Box 15. Why use the WHO City-level monitoring guidance for community SAFER?

Standardized. It uses common indicator definitions that are interoperable with national and global monitoring frameworks (GISAH, Global alcohol action plan, Global Monitoring Framework for NCDs, SDG 3.5).

Comparable. Cross-city and cross-country comparison becomes possible, enabling peer learning and the surfacing of good practice.

Pragmatic. Each indicator has graded status levels (typically nascent ➜ established ➜ advanced) so that communities at different starting points can locate themselves and chart progress.

Operational. The 2024 Excel assessment instrument provides a usable workflow rather than only a conceptual framework.

Pilot-tested. The 20-city pilot demonstrated that the indicators are feasible across diverse contexts and that meaningful subnational progress can be captured *(20)*.

5.5.2 The alcohol-control module

The alcohol-control module of the WHO city-level monitoring guidance is structured around the SAFER intervention areas. Its core indicators address the existence, scope and enforcement of: pricing and taxation measures (R); restrictions on the physical availability of retailed alcohol, including reduced hours, days and outlet density (S); bans or comprehensive restrictions on alcohol advertising, sponsorship and promotion (E); drink–driving laws and blood alcohol concentration limits with enforcement via sobriety checkpoints and random breath testing (A); and the provision of brief psychosocial interventions for hazardous and harmful alcohol use, and prevention, treatment and care for alcohol use disorders and comorbid conditions in health services (F) *(9)*. Optional indicators in the module extend monitoring to community-level outcome measures, including local consumption, alcohol-related morbidity and mortality, and harm experienced by subpopulations.

Communities should adopt the alcohol-control module (Table 32) as the backbone of their community SAFER monitoring. Where the city-level monitoring guidance overlaps with the road-safety module (for drink–driving) and with the NCD and injury surveillance module, those overlaps should be resolved by the steering group at the start of the planning cycle, so that data are collected once and used multiple times.

Table 32. Structure of the alcohol-control module of the WHO City-level monitoring guidance, mapped to the five SAFER intervention areas

SAFER area	City-level alcohol-control indicators	Typical status levels and verification
S – Strengthen restrictions on availability	Existence and enforcement of restrictions on the physical availability of retailed alcohol – for example, reduced hours and days of sale, outlet density limits and restrictions on sale to minors.	Nascent (no restrictions in place)/ established (restrictions in law but limited enforcement)/advanced (restrictions in law and routinely enforced, with monitoring data). Verification: licensing records; police enforcement data.

SAFER area	City-level alcohol-control indicators	Typical status levels and verification
A – Advance drink–driving countermeasures	Existence and enforcement of drink–driving laws and BAC limits, including via sobriety checkpoints and random breath testing.	Nascent/established/advanced. Verification: police enforcement data; road-safety authority records; conviction data.
F – Facilitate access to screening, brief interventions and treatment	Provision of brief psychosocial interventions for hazardous and harmful alcohol use; prevention, treatment and care for alcohol use disorders and comorbid conditions in health services.	Nascent/established/advanced. Verification: health-system service data; primary-care training records; treatment-coverage data.
E – Enforce restrictions on advertising, sponsorship and promotion	Existence of bans or comprehensive restrictions on exposure to alcohol advertising across multiple media; restrictions or bans on promotions in connection with sponsorships and activities targeting young people.	Nascent/established/advanced. Verification: legal instruments; complaints register; media-monitoring evidence.
R – Raise alcohol prices	Existence of pricing measures (excise taxation, minimum unit pricing or equivalent) applied within the community's jurisdictional reach; advocacy and local enforcement of national measures.	Nascent/established/advanced. Verification: national/subnational legal instruments; sales price monitoring.

Source: adapted from the WHO city-level monitoring guidance *(9)*.

5.5.3 Aligning with the global monitoring architecture

The city-level monitoring guidance does not replace existing national, regional and global monitoring frameworks; it nests within them. The principal frameworks with which community-level monitoring should align are summarized in Table 33, and are also discussed in Part 2 (Table 18 of this handbook).

Table 33. Monitoring frameworks relevant to community-level alcohol work, with the WHO City-level monitoring guidance as the operational scaffold

Framework	What it provides	Relevance to community-level monitoring
WHO City-level monitoring guidance for the prevention and control of noncommunicable diseases and injuries (2023) and assessment instrument (2024)	Standardized set of city-level indicators across eight policy areas, including alcohol control; structured around development, implementation and monitoring of key policy interventions; graded status levels *(9, 10)*.	Primary operational tool at community level for community SAFER monitoring.
WHO Global Information System on Alcohol and Health (GISAH)	Global repository of country-level data on consumption, harm and policy responses; published in the WHO *Global status report on alcohol and health and treatment of substance use disorders* (most recent: 2024) *(1, 21)*.	Community data feed into national returns; GISAH definitions ensure community indicators are interoperable with national and global figures.
WHO International guide for monitoring alcohol consumption and related harm	Methodological guidance on indicators of consumption and harm at population level, including for subnational surveillance *(22)*.	Foundation for survey design and the optional consumption indicators in the city-level guidance.
WHO Global alcohol action plan 2022–2030 – indicator set	Indicators and milestones aligned to the six action areas of the Global alcohol action plan, including the 20% relative reduction target by 2030 *(3)*.	Community contribution to national reporting and to the action plan target.

Framework	What it provides	Relevance to community-level monitoring
UN Sustainable Development Goal target 3.5	Indicator 3.5.2 – alcohol per capita consumption (15+); indicator 3.5.1 – coverage of treatment for substance use disorders *(23)*.	Direct link between community action and the SDG architecture; useful for political communication.
WHO Global Monitoring Framework for NCDs	Voluntary global targets, including the relative reduction in harmful use of alcohol *(24)*.	Connects community alcohol work to the wider NCDs agenda and best-buys framework.

5.5.4 Extending the indicator set with community-level outcome measures

The core indicators of the WHO city-level monitoring guidance focus on the existence, scope and enforcement of policy interventions. Communities will typically wish to extend the set with outcome and impact indicators that reflect local harm, drawn from existing administrative data. Suggested extensions, all of which are commonly available at subnational level and are aligned with the optional indicators of the city-level guidance, include:

- alcohol-attributable mortality rate (community or smallest available area), per 100 000 population, annual;
- alcohol-related hospital admissions per 100 000 population, annual;
- alcohol-related violent and disorder incidents per 1000 population, quarterly;
- alcohol-related road traffic injuries (fatal and serious), quarterly;
- alcohol-related ambulance call-outs and emergency-department attendances, with weekend/weekday split, quarterly;
- outlet density (on-licence and off-licence premises per 1000 adults, by smallest available area), annual; and
- heavy episodic drinking prevalence among adults and among young people aged 15–24 years, every two to three years, drawn from population, school or youth surveys.

Where disaggregated subnational data are not currently available for some indicators, the assembly of those data is itself a valuable output of the Community SAFER Plan. Two practical approaches reported by Building SAFER Communities sites in Ireland are: include a deliberate workstream to assemble outlet-density data at small-area level (typically a partnership of licensing authority, police and academic partner); and commission a baseline community survey at the start of the planning cycle and a repeat at the end, with the cost shared across partners *(4, 5)*. Annex 8 provides a working template.

5.5.5 Process and outcome evaluation

Evaluation should be built into the Community SAFER Plan from the outset, not added at the end. Two evaluation modes are normally appropriate.

- **Process evaluation –** assesses how the plan is being implemented: readiness for change, enablers and barriers, fidelity to the SAFER package, quality of inter-agency working. The Building SAFER

Communities project in Ireland uses an implementation-science framework (Burke et al. *(25)*; informed also by the Consolidated Framework for Implementation Research *(26)*), combining key-informant interviews, documentary analysis of meeting records and non-participant observation at steering-group meetings *(4, 5)*.

- **Outcome evaluation –** assesses whether the plan is producing measurable change in the indicators of interest. Outcome measures at the community-population level include the indicators in section 5.5.4 – local alcohol-related crime figures, emergency-department attendances, outlet density, advertising breaches – measured before and after implementation. Outcome evaluation is most credible where there is a comparison group (other communities or unaffected wards within the same community) and where measurement is conducted by an independent academic partner *(4, 5)*.

In addition, formative evaluation – short, structured reflection at key milestones – helps the steering group adjust the plan during implementation rather than only at the end. The realistic ambition for a 24-month community plan is structural change in the regulated environment (licensing, advertising, enforcement, screening coverage) plus measurable changes in proximal harm indicators (police, ambulance, emergency-department data). Long-term population consumption changes will follow but typically over five to ten years *(14)*.

5.5.6 Sharing learning

Community alcohol work is most valuable when its lessons travel. Established channels include: WHO regional networks, including the WHO European Healthy Cities Network and equivalent networks in other regions; the United Nations Inter-Agency Task Force on the Prevention and Control of NCDs; multi-country alcohol policy initiatives such as RESET Alcohol, which supports SAFER policy implementation primarily in Latin America, Africa and Asia; global civil-society platforms including the World Assembly on Community Action on Alcohol; and peer-reviewed publication. The 20-city pilot of the WHO city-level monitoring guidance has itself created a nascent global community of practice that other cities can join *(9, 20)*.

5.6 Protect: managing conflict of interest at community level

The *Global alcohol action plan 2022–2030* is explicit that public health-oriented policy-making must be protected from interference by commercial and other vested interests, and that the alcohol industry has no role in policy development *(3)*. Part 4 of this handbook sets out the principles, steps and tools for managing conflict of interest at national level. Those principles apply equally – and arguably with greater operational urgency – at community level, where the alcohol industry's points of contact are typically more numerous, more local and more diffuse: licensing applications, hospitality industry forums, sports and cultural sponsorship, charitable donations, partnerships with police and youth services, and so-called social-aspects or corporate-social-responsibility bodies.

Community-level protection has three components.

A curated list of resources for community-level SAFER implementation is provided in Annex 11.

5.6.1 Recognizing community-level interference

Common channels of alcohol industry influence at community level include:

- sponsorship of community events, sports, cultural festivals, schools' programmes, charitable causes and emergency-services equipment;

- partnerships with municipal authorities or statutory agencies framed as “responsible drinking” or “prevention” initiatives;
- provision of training materials, speaker fees or conference sponsorship to professional groups (police, hospitality, youth workers);
- hospitality-sector trade associations representing collective industry interests at licensing hearings and in night-time economy partnerships;
- philanthropic foundations or corporate-social-responsibility arms of major producers and retailers;
- commissioning of consultants, opinion polling or research that frames alcohol harm narrowly (for example, as a problem of a small minority of drinkers) and presents commercial measures favourably.

These channels are often individually small, but cumulatively they can constrain the agenda of community SAFER work – typically in the direction of prioritizing education and treatment over the structural measures (availability, advertising, pricing) that the evidence shows are most effective *(14, 27, 28)*.

5.6.2 A community-level COI protocol

Communities should adopt a written protocol on the management of conflict of interest in alcohol policy, modelled on the principles in Part 4 of this handbook and adapted to the community’s legal framework. Discussions among United Nations agencies on workplace alcohol guidance have produced a transferable architecture covering knowledge and awareness, interactions and partnerships, conflicts of interest, procurement and participation, investments, events and hospitality, staff health and safety, and governance and reporting. Annex 10 of this handbook provides illustrative provisions for community use, drawing on the same architecture; Annex 12 sets out an adaptable workplace alcohol guidance template that incorporates these protections (see also Part 6). The Building SAFER Communities project’s i-Mark initiative, developed by Alcohol Forum Ireland and the Irish Community Action on Alcohol Network, supports communities and organizations to operate independently of alcohol industry influence and offers a practical accreditation route that other communities and networks may find adaptable *(4, 5)*.

5.6.3 Active vigilance

Adopting a protocol is necessary but not sufficient. Communities should review industry-related risks at every steering-group meeting and as a standing item in mid-term and end-of-term evaluations; communicate publicly about the protocol so that citizens, partners and industry actors understand the rules; and maintain an active connection to the wider movement for protection of alcohol policy from commercial interference. Vigilance is particularly important during periods of fiscal pressure, when offers of industry funding or partnership may appear attractive.

5.7 Establishing the community SAFER process

Communities that have made measurable progress on alcohol have followed a similar five-step pathway, with local variation (Table 34). They include the WHO Regional Office for Europe MCAP cities of the 1990s, contemporary Healthy Cities and the Building SAFER Communities sites in Ireland *(4, 5, 7, 11)*. The steps are sequential but iterative: communities revisit earlier steps as evidence accumulates and political windows open.

Table 34. The five-step pathway for community-level SAFER implementation

Step	Purpose	Typical duration	Key outputs
1. Establish governance	Secure political mandate; set up the community SAFER steering group; adopt a conflict-of-interest protocol.	1–3 months	Steering group; terms of reference; COI protocol.
2. Assess the baseline	Build a community alcohol profile (licensing, harm and survey data, plus qualitative community evidence); map stakeholders and industry actors.	3–6 months	Community alcohol profile; stakeholder and industry maps; community readiness assessment.
3. Plan the action	Translate evidence into a 24-month Community SAFER Plan organized around the five SAFER areas, the three strategies and the competencies of the relevant tier(s).	3–6 months (overlapping with Step 2)	Adopted Community SAFER Plan with measurable objectives, named leads, indicators and budget.
4. Implement and protect	Deliver the agreed actions through line departments and partners; manage industry-interference risks; communicate progress.	12–24 months	Delivered actions; quarterly progress reports; protected policy decisions.
5. Monitor, evaluate and share	Process and outcome evaluation; feed lessons into next-cycle planning and into national and international learning networks.	Continuous; formal reviews at 12 and 24 months	Monitoring dashboard; mid-term and end-of-term evaluation; published learning.

5.7.1 Building on existing structures

Most communities will not need to create a new structure to deliver SAFER. Instead, the steering group is most effective when it builds on, or sits within, an existing coordination mechanism. Examples include: WHO Healthy City designation and city health coordinators, which provide an established multisectoral mandate; Drug and Alcohol Task Forces (Ireland), which are statutory community-level structures with a delegated coordination role and budget *(4, 5)*; provincial or municipal NCD coordination committees; district health-management teams in many low- and middle-income countries; community safety partnerships, where these have a public health mandate; and primary health-care committees and intersectoral committees on social determinants of health.

Whichever structure is used, it should have a multisectoral composition that includes police, justice, licensing and child-protection partners, not only health; be anchored politically in the community's elected leadership; protect itself from industry interference through a written COI protocol; and link explicitly to the national SAFER Roadmap and to the international monitoring frameworks described in section 5.5.

5.7.2 Composition of a community SAFER steering group

Table 35 sets out a suggested composition. A terms-of-reference template for a community SAFER steering group is provided in Annex 9.

Table 35. Suggested composition of a community SAFER steering group

Suggested composition of a community SAFER steering group; adapt to existing coordination structures.

Member type	Role on the steering group
Community authority (chair)	Political and administrative leadership; alignment with community development plan; convening authority. May be the mayor, district commissioner, provincial governor or equivalent.
Public health authority (co-chair)	Health-system perspective; surveillance data; pathways to screening and treatment; brief-intervention training.
Police/law enforcement	Drink–driving enforcement; public-order data; licensing compliance; enforcement of underage sale.
Justice and probation	Court referrals; work with offenders with alcohol-related offences; restorative pathways.
Child and family services/education	Child-protection perspective; alcohol-related harm to children; perinatal services; school environments.
Statutory community-coordination structure (where it exists)	Pre-existing multisectoral coordination capacity; community-action expertise. In Ireland, the relevant Drug and Alcohol Task Force *(4, 5)*.
Community and voluntary-sector organizations	Community mobilization; reach into specific populations; advocacy capacity.
Youth organizations	Engagement of children and young people; school links.
People with lived experience of alcohol harm	Recovery community; family members; ensures the steering group is grounded in real harm experienced in the community.
Academic/research partner	Independent monitoring and evaluation; publication; peer review of design.
Independent expert advisor (optional)	External challenge; international perspective; quality assurance.

Source: generalized from Building SAFER Communities, Ireland *(4, 5)*.

5.7.3 The community SAFER coordinator

A named coordinator – variously called an alcohol action officer, healthy-city coordinator, community action on alcohol coordinator or equivalent – is a key condition for follow-through in community SAFER work. Without a coordinator with adequate seniority, a protected budget and an institutional home in a public health or community-development team, even well-drafted plans tend to stall *(4, 5, 11)*. Where resources allow, a small project team supports the coordinator and provides continuity if the coordinator changes.

5.7.4 Minimum pathway for low-capacity communities

Communities with limited staff or resources can begin with a simplified SAFER process: appoint a focal point; map alcohol outlets and alcohol-related harms using available data; adopt a conflict-of-interest protocol; select one or two priority actions; define a small indicator set; report progress to the relevant elected or administrative authority; and link the work to the national SAFER Roadmap.

This simplified pathway should not be treated as a lower standard of ambition. It is a staged approach that allows communities to begin implementation while building the governance, data and financing capacity needed for a fuller Community SAFER Plan.

5.7.5 The independent role of civil society in community-level SAFER

CSOs have historically been one of the most durable forces for evidence-based alcohol policy in many countries. The substance of community SAFER work is delivered by statutory authorities – health, justice, licensing, education – but the political and accountability conditions under which that work is sustained over electoral cycles are shaped substantially by civil society.

Four functions of civil society in community-level SAFER deserve specific recognition.

- **Independent monitoring.** CSOs can produce monitoring evidence – on industry interference, on licensing decisions, on advertising compliance, on community harm – that is independent of government reporting cycles and not subject to the political constraints of the government of the day. This evidence is not a substitute for official monitoring (Part 2 and section 5.5) but complements it.

- **Continuity across electoral cycles.** Ministries of health, by virtue of their position in the executive branch, operate within the political direction of the government of the day. Even where individual officials understand the public health evidence, the scope for action is bounded by the wider policy line. CSOs are not bounded in the same way and can therefore sustain advocacy, evidence production and community-level work across changes of administration.

- **Surfacing of industry interference.** CSOs – particularly those that are themselves protected from alcohol industry funding (see Part 4) – are often the first to identify patterns of industry interference, third-party advocacy fronts, captured research, and shifts in industry strategy. Their watchdog function is structurally important to the protection architecture of SAFER.

- **Support to people with lived experience.** CSOs are the principal channel through which people with lived experience of alcohol harm – those in recovery, family members, bereaved relatives – participate in policy and programme design. The community SAFER steering group composition recommended at section 5.7.2 reflects this.

The independence of civil society from alcohol industry funding and from government direction is what makes these functions valuable. Community SAFER plans should therefore support, but not seek to direct, the work of independent CSOs operating in the community. Funding arrangements between community authorities and CSOs should be designed to preserve, not erode, the organizations' independence. The i-Mark accreditation framework developed in Ireland is one operational model (see section 5.9).

5.8 Resource mobilization for community SAFER

Community SAFER work is regularly undermined less by lack of evidence than by lack of sustained resources. Plans are drafted, steering groups are convened, coordinators are recruited under short-term project funding – and then the funding ends, the coordinator leaves, and the work stalls. Resource mobilization is therefore not an afterthought but a strategic component of community SAFER implementation. It is also the area in which national and community levels most need to act in concert: most pricing and tax revenue is national, but most operational delivery is subnational, and the financing relationship between the two determines whether community action is sustainable.

Cross-reference. Section 5.8 is the community-level companion to Part 3. The approach in Part 3, starting from the decision to be informed and choosing the economic assessment that fits it, applies on a smaller scale to community-level analyses and investment cases, and the financing arrangements in section 3.10 apply to community budgets as they do to national ones. Communities should also draw on the

financing analysis in Module 1, section 1.1.5.6, which provides the analytical foundation for both national and community-level resource mobilization.

5.8.1 What needs to be funded

A typical Community SAFER Plan needs predictable funding for six categories of cost over its 24-month cycle:

- **Coordination.** A named community SAFER coordinator with a protected budget; secretariat support to the steering group.
- **Service delivery.** Brief-intervention training, treatment service capacity, server training, enforcement operations and licensing-authority capacity.
- **Communication and community mobilization.** Communication in support of the plan's measures on availability, marketing exposure, drink–driving enforcement and pricing, rather than stand-alone awareness campaigns; protection of school and child-rich environments.
- **Monitoring, evaluation and learning.** Baseline and end-line surveys, the WHO city-level monitoring assessment, an academic partner for process and outcome evaluation, publication and dissemination.
- **Inter-agency convening costs.** Travel, meeting venues, translation, support to lived-experience and community-organization participation.
- **Strategic reserve.** A small contingency for unanticipated opportunities (window of political opportunity, partnership offers from non-conflicted partners) and risks (legal challenge to a licensing decision, industry-led campaign requiring counter-communication).

5.8.2 Sources of funding

Most community SAFER plans are financed from a mix of sources. The mix should be deliberate: dependence on a single source – particularly short-term project funding – is a recurrent cause of programme failure (see section 5.10.2). Seven source categories are typically available, in approximate descending order of sustainability (Table 36):

Table 36. Sources of funding for community-level SAFER implementation

Menu of funding sources for community-level SAFER implementation; combine sources deliberately.

Source	What it is	Sustainability and constraints
Domestic public budgets – community level	Allocations from municipal, district or provincial public health, community-development, social services, justice and police budgets, channelled to a single Community SAFER Plan.	Most sustainable when alcohol action is built into routine line-department budgets rather than a single central pot. Constrained by overall fiscal space and competing political priorities.
Domestic public budgets – national level	Transfers from national health, drugs/alcohol policy, road safety or NCD programme budgets to community-level implementation, often via existing coordination structures (NCD focal points, drug and alcohol task forces, district health teams).	Sustainable when underpinned by national policy. Visible link between community plan and national SAFER Roadmap is essential. The Drugs Policy Unit of Ireland's Department of Health funds Building SAFER Communities at €100 000 per year for three years *(4, 5)*.

Source	What it is	Sustainability and constraints
Earmarked alcohol-tax revenue	Earmarked allocation of a defined share of national or subnational alcohol-tax revenue to public health and community alcohol action; sometimes administered through a national health-promotion foundation. See Part 3, section 3.10.2, Option B.	Politically powerful, and the equity case for directing revenue to the populations bearing most of the harm is set out in Part 3, section 3.10.3. Supplementary to the community's own budget base, not a substitute for it. Requires a national legislative or budgetary instrument; may be opposed by industry. Subject to the limitations of earmarking (offsetting, volatility, review provisions). Where in place, can finance multi-year community work.
Pooled inter-agency financing	Combined contributions from multiple statutory agencies (health, justice, police, transport, education) to a shared community SAFER budget, with shared governance.	Reduces fragmentation; aligns financing with the multisectoral nature of the work. Requires strong steering-group governance and explicit agreement on respective contributions.
International technical and financial cooperation	WHO country and regional offices; United Nations Inter-Agency Task Force on NCDs and joint programming; bilateral and multilateral donors with health, NCDs or development portfolios; international technical-assistance initiatives such as RESET Alcohol.	Important catalyst – particularly in low- and middle-income countries – but typically time-limited; should support transition to domestic financing rather than substitute for it.
In-kind contributions and shared infrastructure	Staff time of partner agencies; use of municipal venues for meetings and training; access to police, hospital and licensing data; voluntary community-organization contributions; pro bono academic partnerships.	Often substantially under-counted; valuing them in the plan budget makes the true scale of inter-agency commitment visible and supports the case for further investment.
Philanthropic foundations and charitable funders	Grants from philanthropic foundations and charitable trusts, directly or through civil-society intermediaries, typically for time-limited programmes, evidence generation or innovation.	Catalytic and often fast; rarely a permanent funding base, so plan a transition to domestic sources, and apply the Part 4 safeguards to any funder with alcohol industry links.

5.8.3 The role of an investment case

Part 3 of this handbook describes the alcohol policy investment case as one form of economic assessment that can support decisions on resourcing SAFER, alongside other fit-for-purpose analyses. The same logic applies at community level: the starting point is the decision to be informed and its audience, whether a council budget allocation, a licensing or bylaw decision, a request to a national programme for transfers, or an approach to a philanthropic funder. For most community decisions, a costed Community SAFER Plan with a one-page statement of budget impact and expected results is the right product. A community-level investment case is warranted where the community seeks a multi-year budget commitment, pooled inter-agency financing or external financing, and where the audience expects the case in that form. It sets out, for the community concerned: the burden of alcohol harm and its associated economic cost; the costs and feasibility of the proposed SAFER actions; the projected benefits in health, social and economic terms; and a phased implementation and financing plan. It is most useful when prepared in cooperation with the community's finance authority and academic partners.

Communities should not over-engineer the investment case. A short, well-evidenced 10–15 page case, with clear headline numbers and a credible financing plan, is typically more effective in budget negotiations than a comprehensive but unread report. Where a national economic assessment or investment case has already been developed (Part 3), the community version can adapt the underlying methodology and present subnational figures. Whatever the product, the financing plan should follow the same order as Part 3, section 3.10: the community's own budget base first, supplemented where available by national transfers, pooled inter-agency financing and, where a national instrument exists, earmarked revenue (Table 36).

5.8.4 Sustainability – the explicit goal

Sustainability – the continuation of structural action on alcohol beyond any single project cycle – should be an explicit goal stated in the plan and tracked as part of the monitoring framework (see section 5.5). Practical strategies that have repeatedly been observed to support sustainability include:

- integrating the community SAFER coordinator post into the establishment of the public health authority or community-development team, rather than maintaining it as a project post;
- building alcohol-action commitments into routine line-department workplans (police, health, licensing, education) so that delivery does not depend on a separate project budget;
- aligning the planning cycle with the community's strategic-planning cycle (city health plan, NCD plan, community-development plan);
- publishing results and committing to peer-reviewed publication, which generates external recognition that supports continued political backing;
- active linkage to the national SAFER Roadmap, so that the community plan is visibly part of national reporting; and
- declining all alcohol industry funding offers (see Annex 10 and Part 6 on workplaces and institutions), even when the offers are presented as solutions to short-term funding gaps. Box 16 describes a community-level assessment from Nepal.

Box 16. Nepal: assessing alcohol regulation and SAFER implementation in five municipalities

In 2023, the Forum for Protection of Consumer Rights Nepal published a situation assessment on the status of alcohol regulation and the implementation of SAFER initiatives in five municipalities of Nepal. The assessment combined documentary review, key-informant interviews and community-level observation across selected urban and peri-urban settings, and reported considerable variation in licensing practice, enforcement, marketing exposure and visibility of brief-intervention services.

The Nepal assessment is illustrative of how a community SAFER analysis can be initiated in a low- or middle-income country setting using available administrative data, primary qualitative methods and a small but explicit indicator set, before scaling up to a fuller Community SAFER Plan. It also illustrates the value of civil-society-led evidence in surfacing implementation gaps that might not be visible from national reporting alone.

Source: adapted from reference *(19)*.

5.9 Country experience: Building SAFER Communities, Ireland (2024–2026)

This section presents the Building SAFER Communities project in Ireland in some detail. As of 2026, the project has been described as a pioneering effort – national in scale and implemented across multiple sites – to translate the SAFER package into a coherent, evaluated community-level demonstration. Its design choices, action set and evaluation architecture offer a transferable reference point for other Member States considering community-level SAFER implementation, while recognizing that any adaptation must reflect each country's legal framework, governance structure and cultural context *(4, 5)*.

5.9.1 Origins and rationale

Building SAFER Communities is led by Well Ireland (formerly Alcohol Forum Ireland) in partnership with the Irish Community Action on Alcohol Network (I-CAAN). It is funded by the Drugs Policy Unit of Ireland's Department of Health, at €300 000 over three years (2024–2026), as one of five projects under a national drug prevention and education funding scheme. The scheme supports Priority 1 of the mid-term review of the National Drugs Strategy, Reducing Harm, Supporting Recovery, which commits to strengthening the prevention of drug and alcohol use and associated harms among children and young people *(4, 5)*.

The rationale for the project rests on three observations. First, alcohol harm in Ireland is substantial: an average of four deaths from alcohol use every day *(29)*; approximately one in two people who drink alcohol doing so at hazardous levels *(30)*; an estimated 578 000 people with an alcohol use disorder in 2019–20 *(31)*; and the third-highest estimated prevalence of fetal alcohol spectrum disorders among 187 countries *(32)*. Second, the heavy normalization of alcohol in Irish life is supported by intensive marketing, widespread availability and culturally embedded consumption practices that no individual-focused intervention can offset. Third, the WHO SAFER package – though developed for national policy-makers – could be operationalized at community level if its five intervention areas were translated into a defined set of practical community actions and tested across a representative range of community settings.

5.9.2 The 10 Building SAFER Communities sites

Through a consultative process with all members of I-CAAN, seven Local and Regional Drug and Alcohol Task Forces volunteered to host the project. Ten communities were identified across the seven Task Force areas, covering a combined population of approximately 190 000 people and including a mix of urban, suburban, peri-urban and small-town settings *(4, 5)*.

5.9.3 Translation: five SAFER areas into 15 community actions

The project's defining design choice was to translate the SAFER package into 15 specific, deliverable community-level actions across the five SAFER areas. The 15 actions are listed in Table 37. They have informed the generic action sets in section 5.4 and are presented here in their original form so that other countries can see exactly how SAFER was operationalized in one national context *(4, 5)*.

Table 37. The 15 Building SAFER Communities actions, organized by SAFER intervention area

SAFER area	Building SAFER Communities actions
S – Strengthen restrictions on alcohol availability	1. Community and agency education on the relationship between availability, outlet density and harm. 2. Alcohol-outlet density studies measuring changes to the number and type of licences in the community over time. 3. Increasing capacity of community and Garda Síochána to engage in licensing (provision of training and resources). 4. Promoting Garda uptake of the specialized certificate in alcohol licensing law. 5. Strengthening restrictions on youth access to alcohol through increased monitoring and enforcement of laws relating to legal purchase age (age checks; test purchasing). 6. Delivery of Responsible Server training to strengthen restrictions on youth access to alcohol and on serving to intoxicated persons.
A – Advance and enforce drink–driving countermeasures	7. Working with Garda Síochána to ensure increased surveillance and enforcement of drink–driving laws. 8. Documenting and promoting community awareness of drink–driving offences and of alcohol-related fatal and non-fatal accidents.
F – Facilitate access to screening, brief interventions and treatment	9. Delivery of SAOR training to local providers. 10. Promotion of Making Every Contact Count among general practitioners and health services. 11. Promotion of existing referral and treatment services.
E – Enforce restrictions on advertising, sponsorship and promotion	12. Supporting increased community engagement in identifying and reporting breaches of the Public Health (Alcohol) Act. 13. Increasing awareness among parents, young people and other relevant stakeholders of the impact of alcohol marketing on young people.
R – Raise alcohol prices through excise taxes and pricing policies	14. Supporting communities to understand and support minimum unit pricing. 15. Increasing awareness of the evidence on the positive impact of minimum unit pricing.

Source: adapted from the Building SAFER Communities information pack *(4, 5)*.

5.9.4 Governance arrangements

The project operates a two-tier governance structure. At the national level, a high-level steering group convenes Well Ireland, formerly known as Alcohol Forum Ireland, An Garda Síochána, the Health Service Executive National Alcohol Programme, a representative from each of the seven participating Task Forces and an independent academic expert (Professor Emeritus Thomas Babor); the WHO Regional Office for Europe advises on the project and disseminates its learning *(4, 5)*. At each participating site, the lead Drug and Alcohol Task Force convenes a local inter-agency steering group *(4, 5)*.

5.9.5 Protection from industry interference

Independence from alcohol industry influence is a foundational principle of the project. Alcohol Forum Ireland and the Irish Community Action on Alcohol Network developed the i-Mark initiative in 2022 to support communities and organizations to operate free from alcohol industry funding, partnerships or framing. The i-Mark provides a practical accreditation route and is referenced throughout the project's communication materials *(4, 5)*.

5.9.6 Evaluation architecture

The research and evaluation component is led by an academic partner (Trinity College Dublin), with Professor Babor advising on design. The evaluation has four components, sequenced across the three project years *(4, 5)*.

- **Process evaluation –** assessment of community readiness for change and of the enablers and barriers to successful implementation, using the implementation-science framework of Burke et al. *(25)*. Methods include key-informant interviews, documentary analysis of meeting records and non-participant observation at key meetings *(4, 5, 26)*.

- **Initial needs assessment –** online community survey across all participating sites in Year 1, repeated in Year 3 to permit comparison.

- **Evidence review –** in Year 1, an evidence review provides the basis for development of an outcome-indicator set for community action on alcohol in Ireland.

- **Outcome evaluation –** in Year 3, an outcome evaluation determines whether the programme is having an effect on the target population, using objective measures including local alcohol-related crime figures, hospital emergency-department attendances and other indicators identified by the evidence review.

The project commits to peer-reviewed publication of at least three papers in Year 3 and to submission of evidence to the European Union Drugs Agency (EUDA) Xchange Prevention Registry *(4, 5)*.

5.9.7 Early lessons for other Member States

While outcome data will not be available until 2027, several design lessons from Building SAFER Communities are already transferable:

- Translate before you implement. The decisive design step was the systematic translation of the SAFER package into 15 specific, deliverable community-level actions. Other countries adopting community-level SAFER will need to make a similar translation, calibrated to their legal framework, governance structures and harm profile.

- Build on existing community-coordination structures. Anchoring the project in the existing network of statutory Drug and Alcohol Task Forces – rather than creating a parallel structure – provided immediate convening authority, multisectoral relationships and political legitimacy at each site.

- Plan the evaluation as part of the design. The four-component evaluation architecture, with an academic partner from the outset and a defined publication and registry strategy, ensures that the project will generate transferable evidence rather than only local benefit.

- Make protection from industry influence explicit and visible. The i-Mark initiative gives the protection-from-interference principle an operational form that participants, partners and the public can recognize.

- Connect to the national plan and to the global movement. Linking the project to Ireland's National Drugs Strategy on the one hand, and to WHO and global community-action networks on the other, anchors the work politically and ensures that lessons travel.

5.10 Critical success factors and common pitfalls

Three decades of community-level alcohol action generate a remarkably consistent set of lessons, from the early WHO Healthy Cities work and the WHO Regional Office for Europe Multi-City Action Plan on Alcohol of the 1990s to contemporary multi-country SAFER initiatives in Latin America, Africa, Asia and Europe *(4, 5, 11)*. The points below condense those lessons for community SAFER work, as critical success factors (Table 38) and common pitfalls (Table 39).

5.10.1 Critical success factors

Table 38. Critical success factors for sustained community-level SAFER action

Factor	Why it matters
Sustained political mandate	Community alcohol action plans are voted on by elected councils or assemblies. Political support can take years to build but, once secured, provides a durable platform for action that survives changes of administration.
A named, senior coordinator	A key condition for follow-through. Without a coordinator with adequate seniority, a protected budget and an institutional home in a public health or community-development team, even well-drafted plans tend to stall.
Multisectoral steering group with police and justice	Communities consistently report that the inclusion of police, justice and licensing partners – not only health – is what enables action on availability, advertising and enforcement to be delivered. Health-only steering groups deliver health-only outcomes.
Local data and a published baseline	Communities that publish a community alcohol profile early find it dramatically easier to defend action against opposition. Data are also a precondition for credible evaluation and for contribution to national reporting.
Active management of conflict of interest	Communities that engage with the alcohol industry on policy development frequently see plans diluted. A clear, written, visible COI protocol – and a refusal of industry sponsorship of community-controlled activity – protects the integrity of the work.
Realistic 24-month horizon, with continuity	Two years is enough to deliver structural change but not to demonstrate population consumption shifts. Plans should be designed in 24-month cycles with explicit hand-over to the next cycle.
Connection to national plan and international learning networks	Linking community work to the national SAFER Roadmap, to WHO regional networks and to global community-action platforms anchors the work and accelerates learning. Isolated communities reinvent the wheel slowly.

Source: developed for this handbook, drawing on community alcohol-action experience, WHO Regional Office for Europe city action on alcohol guidance and Building SAFER Communities materials.

5.10.2 Common pitfalls

Table 39. Common pitfalls in community-level SAFER action and how to avoid them

Pitfall	How it shows up	How to avoid
Framing the plan as restrictive	Opponents portray the plan as "prohibitionist" or "nanny state" and pick off individual measures.	Frame the plan in terms of safer communities, healthier childhoods, reduced violence and avoided economic costs – not banned drinking. Use positive imagery and evidence.
Treating alcohol as a niche health issue	Plan is referred only to the health committee; other departments do not engage; momentum stalls.	Route the plan through every relevant committee and tier – planning, transport, education, housing, recreation, justice. Each adopts its own resolutions.
Dependence on short-term project funding	Coordinator post and resources end with the project; activities lapse.	Build alcohol action into core public health and community-strategy budgets from the outset. Use project funds to demonstrate value, not to sustain core delivery.
Engaging the alcohol industry as a partner	Voluntary industry codes substitute for statutory restrictions; the industry shapes the plan to its commercial advantage.	Adopt and apply the COI protocol (Annex 10). Engage hospitality and retail sectors on operational compliance issues but not on policy design.
Overemphasizing education at the expense of structural change	Plan is dominated by leaflets, posters, school sessions and awareness weeks; harm indicators do not move.	Allocate the majority of effort to structural SAFER actions (availability, advertising, enforcement, screening). Use education as a complement, not a substitute.
Skipping evaluation	Plan is delivered but its impact cannot be demonstrated; political support evaporates at the next election.	Build process and outcome evaluation into the plan from day one; budget for an academic partner; commit to publication.
Heroic individuals without succession	A single committed coordinator delivers exceptional results; when they leave, the work collapses.	Document everything; build a deputy and a steering-group identity; institutionalize the work in line departments.
Neglecting children and the digital environment	Plan focuses on physical outlets and adult drinkers; online marketing and youth exposure are unaddressed.	Include explicit youth and digital-marketing actions; partner with schools, youth services and family supports.

Source: developed for this handbook, drawing on community alcohol-action experience, WHO Regional Office for Europe city action on alcohol guidance and Building SAFER Communities materials.

5.11 Conclusion

Community-level implementation translates national SAFER policy into the lived environment in which alcohol is regulated, served, advertised, enforced, prevented and treated. It is the level at which the global ambition of the WHO *Global alcohol action plan 2022–2030* either takes operational form or fails to. The framework set out in this part is intended to give Member States a transferable architecture for that work: the five SAFER intervention areas delivered through the Implement, Monitor and Protect strategies, populated with practical actions matched to the competencies of provincial, municipal and local-council tiers, anchored in the standard five-step pathway, supported by deliberate resource mobilization, and connected to the workplace and institutional guidance set out in Part 6.

The architecture is technical; the work is political. The Building SAFER Communities project in Ireland and the longer history of community alcohol action across all WHO regions show that sustained progress requires patient political work, multisectoral partnership, active protection from commercial interference, and a willingness to share both successes and failures with peer communities. Communities that do this well do not only reduce alcohol harm in their own territories: they generate the evidence and the political momentum that, over time, shape national and global policy.

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PART 6

Workplaces and institutions – walking the talk in SAFER implementation

PART 6

Workplaces and institutions – walking the talk in SAFER implementation

The provisions in this Part are illustrative implementation options for adaptation and review under the applicable legal, regulatory and institutional framework. They do not establish new WHO recommendations or modify the obligations, staff regulations or approved policies of WHO, other United Nations entities or Member States.

6.1 Purpose and rationale

Workplaces are a cross-cutting setting for SAFER implementation. They include national and local governments, public agencies, United Nations entities, multilateral organizations, hospitals, universities, schools, police and emergency services, transport agencies, CSOs and large private-sector employers. Alcohol-related harm in workplace settings affects staff health and safety, productivity, institutional culture, occupational risk, public trust and the credibility of organizations that promote alcohol policies.

A separate chapter is warranted because workplace alcohol policy is not only a community-level issue. It connects occupational health, human resources, event management, procurement, sponsorship, staff welfare, conflict of interest, partnership governance and institutional integrity. It therefore cuts across local, national, regional and global organizations.

6.2 Walking the talk

For governments, public agencies, United Nations entities and public health organizations, workplace alcohol policy is also a matter of "walking the talk". Institutions that advise Member States and communities to implement SAFER should ensure that their own premises, events, procurement practices, partnerships and staff-support systems are consistent with the public health objectives they promote *(1, 2)*. This does not mean punitive or moralizing approaches to staff behaviour. It means creating environments that prevent harm, avoid mixed institutional messages, provide confidential support and protect policy work from commercial influence.

Walking the talk is particularly important where organizations host official events, receive sponsorship offers, procure event or communication services, manage staff welfare programmes, operate field assessments, or engage with partners whose activities may create real, perceived or potential conflicts of interest. The aim is institutional coherence: the organization's internal practices should not undermine its external public health advice.

6.3 Institutional basis and status of this guidance

Workplace alcohol guidance has been discussed among United Nations agencies. No single United Nations system model policy has been agreed, and this chapter does not present one. It sets out WHO implementation guidance drawn from the principles that emerged in those discussions.

The guidance is intended for adaptation by governments, community authorities, United Nations entities, multilateral organizations and other large employers. It should be aligned with applicable national labour law, occupational-health rules, staff regulations, human-resources procedures, privacy requirements, procurement rules and conflict-of-interest policies.

6.4 Scope of workplace alcohol guidance

Workplace alcohol guidance should cover both prevention and protection. Prevention includes staff health, occupational safety, awareness, confidential support, referral pathways and risk management. Protection includes rules on alcohol industry sponsorship, procurement, events, partnerships, donations, research support, communication activities, institutional investments and participation in policy processes.

6.5 Core elements of workplace alcohol guidance

A workplace alcohol guidance note may include the following core elements: a statement of purpose; scope of application; expectations for alcohol-free or alcohol-safe official settings; procedures for occupational-safety-sensitive roles; prevention of and response to alcohol-related harassment and other harm to others; confidential pathways for support and referral; rules on official events and hospitality; procurement and sponsorship safeguards; conflict-of-interest provisions; data protection and confidentiality; training and communication; responsibilities for implementation; and periodic review.

6.6 Events, hospitality, sponsorship and procurement

Official events are a visible expression of institutional norms. Governments, United Nations entities and public health organizations should avoid practices that normalize alcohol consumption in settings where this conflicts with their public health role, staff-welfare responsibilities or duty of care. Where alcohol is served in contexts permitted by the employer's rules and local law, risks should be managed through clear authorization, non-alcoholic alternatives, limits on use of public funds, attention to safety, and avoidance of alcohol branding or promotional activity.

Workplace guidance should exclude alcohol industry sponsorship, in-kind support, branded materials, event support or communication assistance for public health-oriented activities. Procurement rules should also prevent indirect influence through event organizers, communication agencies, research providers, data vendors, training providers or consultants linked to alcohol industry interests *(3)*.

6.7 Staff support, confidentiality and non-stigmatizing pathways

Workplace alcohol guidance should include confidential, non-stigmatizing pathways for staff who may need support for hazardous alcohol use, harmful use or alcohol use disorders. Support may include occupational-health advice, employee assistance mechanisms, referral to health services, reasonable workplace adjustments where appropriate, and safeguards for privacy and dignity. Disciplinary processes, where required for safety or misconduct reasons, should be clearly distinguished from health-support pathways.

6.8 Implementation checklist

Employers adapting this guidance should:

- designate a responsible unit or focal point;
- review existing human-resources, occupational-health, procurement, event and partnership rules;
- identify alcohol-related risk points;
- define rules for official events and premises;
- establish confidential support and referral pathways;
- prohibit alcohol industry sponsorship and promotional support for public health activities;
- include safeguards in procurement and partnership processes;
- train managers and staff;
- document implementation decisions; and
- review the guidance periodically.

6.9 Link with Annex 12

Annex 12 provides a workplace alcohol guidance template for governments, community authorities, United Nations entities, multilateral agencies and other large employers. The annex should be treated as an adaptable template rather than an agreed United Nations system model policy.

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Annexes

The annexes provide worked examples, templates and checklists to support the application of this handbook. Users are encouraged to adapt them to national and community context. Where specific country examples are referenced, they are illustrative only and do not represent final adopted texts.

Annexes 1 to 7 support Parts 1 and 2 of this handbook, and Annex 3 is also relevant to Part 4. Annexes 8 to 11 support Part 5. Annex 12 supports Part 6 on workplaces and institutions. Annex 13 consolidates the fill-in templates used across the country assessment, planning and monitoring sections. Annex 14 provides a short note on the SAFER name and how to carry it across languages.

Annex 1. Example of alcohol policies (illustrative)

Table A1.1 provides an illustrative example of the types of alcohol legislation and policy that are typically identified and characterized in a Country SAFER Landscape Report (Module 1, section 1.1.5.3). The example is composite and should be tailored to each country's legal system.

Table A1.1. Illustrative example of alcohol policies

Instrument	Intervention area	Summary	Strengths	Gaps
National alcohol control act	S A E R	Consolidated statute setting the framework for licensing, sale, advertising and enforcement of alcohol.	Provides a single legal framework; creates a national alcohol policy authority.	Does not explicitly regulate digital/online advertising; outdated penalty schedule.
Road traffic (amendment) act	A	Sets general BAC limit at 0.05% or lower and 0.02% or lower for novice/professional drivers; authorizes random breath testing.	Lower BAC limit for novice and professional drivers if not 0.00%; enforcement powers.	No ignition-interlock programme; inconsistent enforcement across districts.
Excise duty (alcoholic beverages) regulations	R	Specific excise duties on beer, wine, spirits and fortified products.	Specific (rather than ad valorem) duty structure.	No automatic inflation indexation, so real duty levels erode over time; WHO guidance calls for regular adjustment for inflation and income growth.
Broadcasting standards regulations	E	Restrictions on alcohol advertising on public broadcast services between 06:00 and 22:00.	Statutory restrictions with clear enforcement route.	Does not cover social media, sponsorship or influencer marketing.
Primary health care clinical guidelines (AUD chapter)	F	Guidelines on screening, brief intervention and treatment at primary-care level.	Aligned with the WHO Mental Health Gap Action Programme (mhGAP); national coverage.	Limited training; weak referral pathways for specialist care.
Licensing and consumer protection rules	S	Rules on outlet density, opening hours and minimum legal purchase age (18 years).	Tiered licensing categories.	Permissive night-time trading hours; uneven local enforcement.

Annex 2. Example of sectors and stakeholders involved in alcohol policy coordination

Table A2.1 gives an illustrative example of how the institutional and governance landscape can be summarized in a Country SAFER Landscape Report (Module 1, section 1.1.5.4).

Table A2.1. Illustrative example of sectors and stakeholders involved in alcohol policy coordination

Sector/stakeholder	Role in alcohol policy
Ministry of health – NCD department/alcohol, substance use unit	Policy lead; coordinates national alcohol policy strategy; manages screening, brief intervention and treatment guidelines.
Ministry of finance/revenue authority	Designs and administers alcohol excise taxation; enforces tax compliance.
Ministry of transport/road safety agency	Leads drink–driving policy, enforcement and driver education; operates random breath-testing programmes.
Ministry of internal affairs/police	Enforces drink–driving, licensing compliance and public-disorder provisions.
National alcohol licensing board/ authority	Issues and revokes alcohol-retail and production licences; monitors compliance.
Broadcasting regulator	Regulates alcohol advertising on licensed broadcast media.
Consumer protection agency	Oversees labelling, consumer information and product-safety aspects of alcohol retail.
Ministry of education/school health unit	Delivers school-based prevention programmes and life-skills education.
Ministry of social development/ child protection	Addresses alcohol-related family violence, child maltreatment and social-service response.
National bureau of statistics	Provides population, household and economic data used in monitoring and economic assessments.
Office of the attorney-general/state law office	Drafts alcohol-related legislation; provides advisory and litigation support.
Parliament – health, finance and judicial committees	Legislative oversight; adopts and amends alcohol policy statutes.
Academia and research institutions	Research, evaluation, training and analytical support.
CSOs and faith-based organizations	Advocacy, awareness-raising, service delivery and social mobilization.
Professional associations (medical, nursing, public health, bar/legal)	Clinical guidelines; professional training; policy advocacy.
International partners (WHO, UNDP, United Nations Children's Fund, World Bank, donors)	Technical and financial support; alignment with global frameworks.

Annex 3. Example of alcohol industry stakeholder mapping

This illustrative mapping (Table A3.1) shows the categories of alcohol industry stakeholders typically identified during a landscape analysis (Module 1, section 1.1.5.7). Specific company names are omitted; countries should complete the mapping using up-to-date national intelligence and apply the conflict-of-interest principles in Part 4 throughout.

Table A3.1. Illustrative example of alcohol industry stakeholder mapping

Category	Examples of actors	Policy-relevant activities
Transnational alcohol corporations	Major beer, wine and spirits multinationals operating in the country.	Marketing, pricing, sponsorship, lobbying, partnerships with government ministries other than health.
Domestic producers	National breweries, wineries and distilleries.	Marketing, pricing and contributions to trade associations.
Importers and distributors	Wholesale alcohol distribution companies.	Supply-chain positioning; import-tariff advocacy.
Large retailers	Supermarket chains, off-licence chains, hospitality groups.	Retail pricing, promotions, point-of-sale placement.
Informal and unrecorded producers	Home-brewers; traditional alcohol producers; illicit distillers.	Price competition; quality and safety risks; tax evasion.
Industry associations (trade associations)	National alcoholic-beverages association; brewers' association.	Collective advocacy; self-regulation; lobbying of parliamentarians and ministries.
Social-aspects/public-relations organizations (SAPROs)	Industry-funded "responsible drinking" or "education" bodies.	Public-relations campaigns; CSR activities; engagement with education and public health actors.
Advertising, marketing and hospitality partners	Advertising agencies; event organizers; sports and entertainment sponsors.	Marketing amplification; sponsorship; product placement.

Annex 4. Example of potential collaborators for SAFER implementation

This list (Table A4.1) illustrates the kinds of partner organizations that a ministry of health and WHO country office might engage during SAFER implementation (Module 1, section 1.1.5.8). The list is illustrative and non-exhaustive.

Table A4.1. Illustrative example of potential collaborators for SAFER implementation

Sector	Example partners
Government	Line ministries (Health, Finance, Transport, Interior, Trade, Education, Social Development); revenue authority; parliamentary committees; subnational governments.
United Nations and international organizations	WHO (country, regional and headquarters levels); UNDP; United Nations Children's Fund; UN Women; International Labour Organization; World Bank; regional development banks.
Civil society	National alcohol policy alliance; women's and youth organizations; road-safety NGOs; faith-based organizations.
Health-care associations	Medical association; nursing association; public health association; paediatrics society; psychiatry association; family-medicine association.
Academia and research	Public health schools; law faculties; economics departments; policy think tanks; national alcohol/substance-use research centres.
Media	Public-service broadcaster; independent media; health-journalism networks; community radio.
Community and faith organizations	Religious councils; community-based organizations; recovery networks.
Disease and patient associations	Cancer societies; liver-disease associations; cardiovascular societies; mental-health associations; road-injury survivor networks.
Potential donors	Bilateral aid agencies; philanthropic foundations with health portfolios; multilateral funds.

Annex 5. Key-informant interview checklist – institutional and governance arrangements

The following checklist supports section 1.1.5.4 of Module 1. It is designed for semi-structured interviews with government officials, regulators and non-State actors and should be adapted to the role and mandate of the interviewee.

A. Multisectoral coordination

- Is there a coordination mechanism for alcohol policy (committee, task force, technical working group)?
- Who leads it? What is its legal or policy basis?
- Which ministries and non-State actors are represented? How often does it meet?
- How are decisions made and recorded? Are minutes and action logs published?
- Does it interact formally or informally with the alcohol industry? If so, how is this managed (see Part 4)?

B. Decision-making authorities

- Which ministries or agencies have decision-making power on each SAFER intervention?
- How are policy and regulatory decisions taken? What consultation processes apply?
- How are subnational governments involved (cross-reference Part 5)?

C. Implementation capacities

- Which agency licenses and inspects alcohol outlets? What resources does it have?
- What expertise (epidemiology, law, economics, enforcement) is available in-house?
- What monitoring and surveillance systems exist? How are data used (cross-reference Part 2)?
- What communication and public-awareness strategies are in place?
- How do authorities collaborate with NGOs, academia and professional bodies?

D. Resources

- What is the annual budget for alcohol policy activities, by agency?
- Are alcohol taxes earmarked for health or social programmes (cross-reference Part 3)?
- What external funding (donors, technical partners) is used?

E. Gaps and opportunities

- What are the main institutional barriers to effective alcohol policy?
- What opportunities does the respondent identify for strengthening SAFER implementation?
- What examples of industry interference has the respondent observed, if any (cross-reference Part 4)?

Annex 6. Example of a completed SAFER Roadmap (illustrative extract)

Table A6.1 provides an illustrative extract from a SAFER Roadmap (Module 3). It shows, for one selected SAFER intervention, how the Roadmap matrix can be structured to capture objectives, activities, indicators, responsible parties and timelines. In a full Roadmap the same structure is repeated for every selected intervention.

Table A6.1. Illustrative extract from a completed SAFER Roadmap matrix

Objective	Activity	Expected output	Indicator	Baseline	Target	Responsible	Timeline	Budget (US$)
Strengthen drink–driving countermeasures (SAFER intervention A)	Amend Road Traffic Act to reduce general BAC limit from 0.08% to 0.05% and adopt 0.02% for novice/professional drivers	Amendment bill drafted and tabled before parliament	Status of amendment bill	Drafting not started	Bill tabled and enacted	Ministry of transport; attorney-general	Months 1–12	75 000
	Procure and deploy 200 evidential breath-testing devices across districts	Equipment procured, deployed and operational	Number of districts equipped	0	All 45 districts	Ministry of internal affairs/police; ministry of finance	Months 3–18	450 000
	Train 1 500 traffic-police officers on new BAC enforcement and procedures	Trained officer cohort	Number of officers trained	0	1 500	Police Training College	Months 6–18	120 000
	Deliver national mass-media drink–driving campaign	Campaign delivered across radio, television and digital channels	Population reach (%)	–	≥70% of adult population	Ministry of health; road safety agency	Months 9–21	300 000
	Establish a drink–driving surveillance dashboard and publish quarterly reports	Dashboard operational; four quarterly reports published	Number of reports published	0	4	Road Safety Agency; National Bureau of Statistics	Months 6–24	60 000

Annex 7. SAFER key results framework (illustrative)

Table A7.1 illustrates how a country-level SAFER results framework can be structured (Part 2). In a full framework, indicators are populated for each row with baseline values, targets, means of verification and risk/assumption notes, and are aligned with the WHO Global alcohol action plan 2022–2030 and the WHO Global Monitoring Framework for NCDs.

Table A7.1. Illustrative SAFER key results framework

Results level	Statement	Illustrative indicators
Impact	Reduced harmful use of alcohol and related social, health and economic harm in the country.	Alcohol-attributable mortality rate; alcohol-attributable DALYs; APC (litres of pure alcohol, 15+); prevalence of heavy episodic drinking.
Outcome 1	Strengthened policy and legal framework for alcohol policy.	Number of SAFER-aligned laws enacted; coverage of SAFER interventions (scorecard).
Outcome 2	Increased population exposure to effective high-impact alcohol policy measures.	BAC enforcement coverage; marketing-restriction coverage; alcohol tax as % of retail price; minimum-age enforcement rate.
Outcome 3	Strengthened health-system response to alcohol use disorders.	Coverage of SBI in primary care; coverage of AUD treatment services.
Outcome 4	Strengthened protection of alcohol policy from conflict of interest and industry interference.	Existence of inter-departmental COI protocol; number of industry-interference incidents reported and managed.
Outputs	SAFER Roadmap activities delivered as planned (see Annex 6 for activity-level outputs).	Percentage of Roadmap activities completed on schedule; budget execution rate.
Activities/inputs	SAFER Roadmap activities funded, staffed and implemented.	Financial resources mobilized; personnel deployed; technical support provided.

Annex 8. Community alcohol indicator template

This template (Table A8.1) adapts the extended outcome and impact indicators described in section 5.5.4 into a template that the steering group populates at baseline, mid-term review (12 months) and end-of-term (24 months). It is intended to complement – not replace – the alcohol-control module of the WHO City-level monitoring guidance and its 2024 assessment instrument, which together provide the standardized core indicator set on policy interventions. Indicator definitions follow national and GISAH-compatible conventions so that community returns can be aggregated to the national results framework.

Table A8.1. Community alcohol indicator template

Indicator	Definition	Baseline	12 months	24 months	Data source/lead
Alcohol-attributable mortality rate	Deaths from causes wholly or partly attributable to alcohol per 100 000 population				
Alcohol-related hospital admissions	Inpatient admissions with an alcohol-related primary or contributory diagnosis per 100 000 population				
Adult APC (proxy)	Litres of pure alcohol per adult resident per year (sales-based proxy where available)				
Heavy episodic drinking – adults	Percentage of adult population reporting consumption of ≥60 g of pure alcohol on at least one occasion in past 30 days				
Heavy episodic drinking – ages 15–24	Percentage of young people aged 15–24 years reporting heavy episodic drinking in past 30 days				
Alcohol-related violent/ disorder incidents	Number per 1 000 population, including breakdown for night-time economy zones				
Alcohol-related road traffic injuries	Fatal and serious road traffic injuries with alcohol as contributing factor				
Alcohol-related ambulance call-outs and emergency-department attendances	Number per 1 000 population, with weekend/ weekday split				
Outlet density	On-licence and off-licence premises per 1 000 adults, by smallest available area				
Enforcement actions	Number of test-purchasing and licence-compliance actions, with outcomes				
Primary-care SBI training coverage	Percentage of primary-care staff trained in alcohol SBI				
Reported advertising-restriction breaches	Number reported and investigated				
Plan implementation rate	Percentage of plan actions completed on schedule				
Budget executed	Total community-funded budget for alcohol action, executed against allocated				

Annex 9. Community SAFER steering group – terms of reference template

Purpose

To provide leadership, oversight, coordination and accountability for the development, adoption, implementation, monitoring and evaluation of the Community SAFER Plan, in alignment with the WHO SAFER package, the *WHO Global alcohol action plan 2022–2030* and the country's SAFER Roadmap (Module 3 of Part 1).

Membership

The steering group is convened by the community authority and chaired by an elected member or senior official. Standing membership comprises representatives of the agencies listed in Table 35 of this handbook. Members serve in their organizational capacity and are nominated in writing by their organizations. The steering group may co-opt additional members as required.

Functions

- Endorse the Community SAFER Plan and any subsequent amendments.
- Oversee implementation of the plan, including resolving inter-agency obstacles.
- Approve the monitoring and evaluation framework, including the indicator set.
- Receive and consider quarterly progress reports from the community SAFER coordinator.
- Approve communication of plan results to the community authority, media and partner networks.
- Manage conflicts of interest in line with the steering group's COI protocol (Annex 10).
- Authorize representation of the community in national and international learning networks.

Meetings, decision-making and reporting

The steering group meets quarterly. An annual review meeting takes place in person and includes a longer agenda for strategic stocktake. Meeting agendas, papers and minutes are published, subject to data-protection considerations. Quorum is half of the standing membership. Decisions are taken by consensus where possible; where a vote is required, decisions are taken by simple majority of members present. The steering group reports formally to the community authority annually and provides a mid-term (12 months) and end-of-term (24 months) evaluation report.

Annex 10. Conflict-of-interest protocol – illustrative provisions

This annex provides illustrative provisions that communities may adapt to their legal framework. They are designed to align with the protective provisions of the WHO *Global alcohol action plan 2022–2030*, with the principles in Part 4 of this handbook, and with workplace alcohol guidance discussed among United Nations agencies.

- **Declaration.** All steering-group members, expert advisors and officials with substantive responsibility for the plan complete a written declaration of interests on appointment, update it annually and on any material change. Declarations are published where consistent with applicable law and data-protection requirements.

- **Industry exclusion from policy development.** Representatives of the alcohol industry, alcohol industry trade associations, social-aspects/public-relations organizations and bodies receiving substantial alcohol industry funding do not participate in the development of the Community SAFER Plan, in the steering group or in any of its working groups.

- **Recording of contacts.** Any contact between a steering-group member or an official with substantive responsibility for the plan and an alcohol industry actor is recorded in a public log (date, parties, purpose, outcome). Routine regulatory contacts under licensing law are exempted but governed by the standard regulatory-records protocol.

- **Sponsorship and funding.** The community authority does not accept sponsorship, funding or in-kind contributions from the alcohol industry for any event, activity or publication controlled, owned or supported by the authority. Existing arrangements are reviewed and, where in conflict with this provision, terminated at next renewal.

- **Procurement.** Procurement related to the Community SAFER Plan (including evaluation contracts, communications and training providers) may, where consistent with applicable procurement law and policy, exclude bidders with substantive financial relationships with the alcohol industry, unless such relationships are fully disclosed and assessed as immaterial.

- **Whistle-blowing and review.** A confidential route exists for staff and citizens to raise concerns about industry interference. The steering group reviews the operation of the protocol annually and amends it as required.

Annex 11. Selected resources for community-level SAFER implementation

The following resources are recommended starting points for communities preparing to develop or refresh a Community SAFER Plan. The list is illustrative and non-exhaustive.

- The SAFER technical package: five areas of intervention at national and subnational levels. Geneva: World Health Organization; 2019 (https://iris.who.int/handle/10665/330053).

- Global alcohol action plan 2022–2030. Geneva: World Health Organization; 2024 (https://iris.who.int/handle/10665/376939).

- City-level monitoring guidance for the prevention and control of noncommunicable diseases and injuries. Geneva: World Health Organization; 2023 (https://iris.who.int/handle/10665/374874); and assessment instrument. Geneva: World Health Organization; 2024 (https://www.who.int/publications/m/item/city-level-monitoring-guidance-for-the-prevention-and-control-of-noncommunicable-diseases-and-injuries).

- International guide for monitoring alcohol consumption and related harm. Geneva: World Health Organization; 2000 (https://iris.who.int/handle/10665/66529).

- Global status report on alcohol and health and treatment of substance use disorders. Geneva: World Health Organization; 2024 (https://iris.who.int/handle/10665/377960).

- Babor TF, Casswell S, Graham K, Huckle T, Livingston M, Österberg E, et al. Alcohol: no ordinary commodity – research and public policy. Third edition. Oxford: Oxford University Press; 2022.

- Healthy Cities – global movement and regional networks [website]. Geneva: World Health Organization (https://www.who.int/teams/health-promotion/enhanced-wellbeing/ninth-global-conference/healthy-cities).

- Alcohol Forum Ireland, Irish Community Action on Alcohol Network. Building SAFER Communities: preventing and reducing alcohol harm – information pack for participating communities (2024–2026). Letterkenny: Alcohol Forum Ireland; 2023 (https://alcoholforum.org/wp-content/uploads/2024/03/Building-SAFER-Communities-Preventing-Reducing-Alcohol-Harm.pdf).

- Vital Strategies. RESET Alcohol [website] (https://www.vitalstrategies.org/alcohol-policy/).

- Guidelines for city action on alcohol. Copenhagen: WHO Regional Office for Europe; 1998 (EUR/ICP/LVNG 03 03 01; https://iris.who.int/handle/10665/108102).

Annex 12. Workplace alcohol guidance template for governments, community authorities, United Nations entities and other large employers

The provisions below are illustrative implementation options for adaptation and review under the applicable legal, regulatory and institutional framework. They do not establish new WHO recommendations or modify the obligations, staff regulations or approved policies of WHO, other United Nations entities or Member States.

This annex provides a workplace alcohol guidance template that governments, community authorities, United Nations entities, multilateral agencies and other large employers can adapt to their legal framework, labour-relations context, occupational-health arrangements and conflict-of-interest rules. It supports Part 6 of this handbook and should be treated as an adaptable template rather than as an agreed UN system model policy. It addresses both the employer's duty of care towards staff and others affected by staff alcohol use, and the protection of institutional decision-making from alcohol industry influence: employers that hold themselves to the standards they promote strengthen both the policy and their own credibility.

Purpose

To prevent and reduce alcohol-related harm to the employer's staff and to others affected by staff alcohol use, including accidents, injuries and harassment; to provide non-stigmatizing support for staff with hazardous alcohol use or alcohol use disorders; to maintain a safe, healthy and respectful working environment, consistent with the employer's duty of care to its staff and to the people it serves; and to protect the integrity, independence and effectiveness of the employer's policies and programmes from undue influence by the alcohol industry and related commercial interests.

Scope

This policy applies to all staff, consultants, secondees, interns and elected representatives of [the community authority/agency]; to all premises owned or operated by the authority; to work-related travel and to driving or operating vehicles or machinery in the course of duty; to work-related social occasions, wherever held; and to all official functions, events, partnerships, sponsorships, procurement and investment decisions taken by or on behalf of the authority.

1. Knowledge and awareness

- Inform all personnel and leaders about alcohol-related harm, including harm to others such as accidents, injuries and harassment; about the support available under this policy; about the importance of protecting policy and programme development from industry interests; and about common industry tactics (including front and affiliated bodies).

- Ensure programmes and partnerships are consistent with the WHO *Global strategy to reduce the harmful use of alcohol*, the *Global alcohol action plan 2022–2030* and SDG 3.5, within the authority's mandate.

2. Events, premises and hospitality

- On the authority's premises, do not serve or allow alcohol during working hours, except where cultural, functional or diplomatic protocol requires it and a senior official authorizes an exception.

- For official functions, events and receptions: as a rule, do not use public funds to purchase alcohol. Any protocol or custom exception requires prior written approval and itemized reporting.

- Where alcohol is present as an exception or from other sources: participation is voluntary and no one is pressured to drink; choosing not to drink carries no professional or social disadvantage; alcohol-free options are equally prominent and of equal quality; service is controlled (reasonable limits, no self-service, ends before close); unlimited or promotional offers are not allowed; food is provided; safe transport options are communicated and drink–driving is actively discouraged; and alcohol industry sponsorship, branding or in-kind donations are not accepted.

3. Impairment, safety and fitness for duty

- Require all personnel to be fit for duty and unimpaired by alcohol during working hours, when on call, and when representing the authority or providing services to the public; managers address suspected impairment promptly, prioritizing safety and a supportive, non-disciplinary first response where no misconduct or harm to others has occurred.

- For safety-critical roles and tasks, apply a zero-alcohol rule during duty and, where applicable law and occupational-health policy allow, for a defined period before duty. The rule should be consistent with, or stricter than, national law and occupational-safety standards. Safety-critical roles and tasks include driving or operating vehicles, machinery or equipment, clinical care, emergency response and policing.

- Record and review alcohol-related accidents, injuries and near misses through existing occupational-safety systems; after any incident, offer support to those affected and use the findings to strengthen prevention.

- Expect managers, senior officials and elected representatives to model the standards in this policy, including at official functions and events.

4. Harassment, misconduct and harm to others

- Make clear that alcohol-related harassment, including sexual harassment, and alcohol-related violence, intimidation and other misconduct – at the workplace, at work-related events or during work-related travel – are addressed under the authority's codes of conduct and disciplinary procedures, and that intoxication is never a mitigating circumstance.

- Provide safe, confidential channels for reporting alcohol-related harassment and safety concerns, protect those who report from retaliation, and act on reports promptly.

- Offer support to staff affected by another person's alcohol use, including referral to occupational-health, counselling or employee-assistance services.

- Distinguish support from accountability: seeking help for one's own alcohol use is never penalized, and support is available alongside – not instead of – accountability for harm caused to others.

5. Support for staff with hazardous alcohol use or alcohol use disorders

- Provide confidential access to screening, brief intervention and referral to treatment for hazardous alcohol use and alcohol use disorders, covered where possible through the applicable health-insurance or occupational-health arrangements.

- Promote a supportive, non-stigmatizing environment and adapt work arrangements where feasible for staff in treatment or recovery.

- Treat alcohol use disorders as health conditions: safeguard confidentiality within occupational-health and data-protection rules, protect staff who seek help from discrimination in employment decisions, and apply normal sick-leave and return-to-work arrangements.

- Encourage early, voluntary help-seeking before health, safety or performance is affected, including self-referral routes that do not involve line management.

6. Interactions, partnerships and funding

- Do not enter partnerships, joint programmes, memoranda of understanding, sponsorships or other voluntary arrangements with the alcohol industry or its representatives where this would create a real or perceived conflict with public health goals.

- Where strictly necessary, limit interaction with the alcohol industry to dialogue and information exchange for legitimate purposes; do not enter joint statements, contracts or use of the authority's name, logo or emblem by the industry; correct misrepresentation promptly.

- Decline direct or indirect financial or in-kind contributions from the alcohol industry (including via intermediaries) where a real or perceived conflict exists.

- Do not engage in partnerships or sponsorships involving non-alcoholic brand extensions that use the same or confusingly similar branding, logos or imagery as alcoholic beverages.

7. Conflicts of interest, procurement and participation

- Maintain a conflicts-of-interest policy covering staff, consultants, secondees and elected representatives; require potential donors and partners to disclose income sources and any direct or indirect links to the alcohol industry; conduct due diligence and decline support where a real, perceived or potential conflict exists.

- Do not award contracts to tenderers with material conflicts of interest related to alcohol policy objectives.

- Exclude individuals employed by, funded by, or providing services to the alcohol industry from committees, advisory groups, governance roles or assessment teams where a real or perceived conflict exists.

- Adopt a staff code of conduct for any strictly necessary dealings with the alcohol industry, consistent with the Standards of Conduct for the International Civil Service or equivalent national standards. Do not accept payments, gifts, services, hospitality or research funding from the alcohol industry, whether monetary or in-kind.

8. Investments

- Avoid holdings in alcohol industry companies or organizations furthering alcohol industry interests; where such holdings exist within the authority's control, consider divestment where consistent with applicable investment policies and fiduciary obligations.

- Do not award or recognize alcohol industry entities for activities described as "socially responsible".

9. Governance, transparency and review

- Designate focal points to advise on this policy, register interactions with industry actors, and review conflict-of-interest and due-diligence assessments.

- Where a limited, mandate-critical exception is strictly necessary (for example, a one-off technical briefing), require prior written approval by the relevant division head or higher and document the safeguards in place (no industry funding or branding, no promotion, neutral framing).

- Maintain an internal register of approved exceptions and interactions; publish summaries where appropriate and lawful.

- Share information on alcohol industry interference and countermeasures through national and international coordination mechanisms (in the UN system, through UNIATF; in community work, through the steering group, the WHO country office and applicable regional networks).

- Review and update measures periodically for consistency with national and international guidance and WHO technical advice, covering the staff health, safety and support provisions as well as the integrity provisions of this policy.

10. Reporting

- Share the endorsed policy, where appropriate, with the relevant national or international coordination body (for UN agencies, UNIATF; for community authorities, the national SAFER coordination structure).

- Report annually on implementation, including challenges, lessons learned and examples of good practice.

Annex 13. Templates for country assessment, planning and monitoring

This annex consolidates the fill-in templates and the worked example used when preparing country SAFER landscape reports, joint SAFER assessments, SAFER roadmaps and monitoring plans. Each is cross-referenced at the relevant point in the main text.

Table A13.1. Template for analysing alcohol laws and policies

Analysis template for the legal and policy review; one row per law or policy instrument, covering status, enforcement and gaps. Annex 1 provides worked examples.

Legal or policy measure	Description	Strengths and gaps	Evidence of effectiveness
[For example, Liquor Licensing Act]	[Summary of scope and purpose]	[Summary of strengths and gaps]	[Evidence base]
...	...	...	...

Table A13.2. Template for stakeholder mapping and political drivers

Mapping template for stakeholders and political drivers; updated through the life of the Landscape Report and reused in later modules.

Stakeholder category	Activities (ongoing and previous)	Impact
Government ministries, departments and agencies	• Alcohol policy activities and initiatives. • Partnerships and collaborations with non-State actors. • Participation in international alcohol policy platforms.	Impact of government action on alcohol policy.
Local, regional and global CSOs, professional bodies, academia	• Advocacy, awareness-raising, service-delivery and research activities. • Engagement of academia in research and policy analysis.	Impact of non-State actors on alcohol policy.
Lawmakers and parliamentary committees	• Identification of relevant lawmakers and committees. • Key political and economic priorities and incentives.	Impact of lawmakers on alcohol legislation.
Local and international alcohol industry actors	• Market share and ownership structure. • Trade associations, SAPROs and lobbying groups. • Lobbying, partnerships with government, CSR initiatives, marketing strategies.	Impact of industry activity on alcohol policy-making and implementation.

Table A13.3. Stakeholder-mapping template

Stakeholder-mapping template for Joint Assessment preparation; identifies the actors to be engaged during the assessment.

Stakeholder group	Organization	Role in alcohol policy/SAFER interventions
Government MDAs	Ministry of health; ministry of finance/treasury; revenue authority; ministry of communication; ministry of trade; alcohol-licensing authority; national bureau of statistics; law enforcement; state law office/attorney-general; parliament (committees and caucuses); subnational governments.	
UN agencies and key development partners	Relevant UN agencies; World Bank; bilateral donors; foreign development organizations.	
Local NGOs and faith-based organizations		
Professional bodies		
Research institutions and academia		
Media		
Professionals and subject-matter experts		
Alcohol industry – informational only, governed by the Part 4 COI principles		

Table A13.4. Assessment-plan template

Assessment-plan template; prepared at the start of the planning phase, normally three to six months before the assessment start date, and reissued as planning evolves.

Assessment activity	Key tasks	Responsible party
Assessment coordination	Overall coordination; chair of coordination meetings.	
Liaison with government	Liaison with ministries; management of government protocols; management of inception meetings.	
Programme development	Assessment schedule and timeline; meeting objectives and agendas; design of sessions, workshops and interviews.	
Stakeholder mapping	Identification of stakeholders; meeting invitations; scheduling of interviews.	
Logistics	Venue and conferencing services; information technology and audiovisual support; transportation and accommodation.	

Assessment activity	Key tasks	Responsible party
Materials preparation	Presentations, handouts, participant packs, name badges, logistics documents.	
Session rapporteurs	Assignment of rapporteurs for each session.	
Reporting	Meeting and session reports; compilation of assessment report.	

Table A13.5. Example assessment programme

Worked example of a five-day assessment programme; adapt to the country context.

Date	Timing	Activity	SAFER lead	Attendees	Venue
Day 1	Evening	Arrival at hotel	Local consultant	International and national assessment team	Hotel
Day 1	Evening	Reception and working dinner	Local consultant	International and national assessment team; local stakeholders	Hotel
Day 2	Morning	Opening session	WHO	All team members	Conference room
Day 2	Afternoon	Meeting with ministry of health	WHO/SAFER partners	Chief of Health; NCD lead; Alcohol Unit	Ministry of health
Day 3	Morning	Meeting with ministry of transport	WHO/SAFER partners	Controlled-substance unit; road-safety team	Ministry of transport
Day 3	Morning (parallel)	Meeting with CSOs	Local consultant	Faith-based organizations; CSOs	Conference room
Day 3	Afternoon	Group workshop	WHO/SAFER partners	CSOs; international organizations	Conference room
Day 4	Full day	Field visits (primary care; licensing authority)	WHO/SAFER partners	Health, law-enforcement and licensing staff	Field sites
Day 5	Morning	Closing workshop; consensus-building	Ministry of health/ WHO	All team members and lead stakeholders	Conference room
Day 5	Afternoon	Press briefing and departure briefing	Ministry of health	Media; team members	Ministry of health

Note: The programme is a template only; in practice every assessment adapts this structure substantially to the country context and to the political moment.

Table A13.6. Stakeholder mapping and analysis of political drivers

Analysis matrix for stakeholder mapping and political drivers at the Roadmap stage.

Stakeholder group	Role and influence	Opportunities to support SAFER	Possibility of opposition	Engagement strategy
Government MDAs				
Legislative bodies/assemblies				
Civil-society and faith-based organizations				
Professional bodies				
Academia and research institutions				
Alcohol policy champions				
Alcohol industry – informational only, governed by Part 4				

Table A13.7. Planning matrix for monitoring

Planning matrix for monitoring: responsibilities, data flows, sources, frequency, analysis and reporting.

Expected results (outcomes and outputs)	Indicators (with targets)	M&E event and data-collection methods	Time schedule and frequency	Responsibilities	Means of verification	Resources	Risks

Source: adapted from: Handbook on planning, monitoring and evaluating for development results. New York: United Nations Development Programme; 2009 (https://digitallibrary.un.org/record/671515).

Annex 14. The SAFER name and how to carry it across languages

A short note for translators, communicators and designers.

About the name

SAFER is two things at once. It is an acronym for the five evidence-based, cost-effective measures that any country can use to reduce the harm done by alcohol:

- **S** Strengthen restrictions on alcohol availability;
- **A** Advance and enforce drink–driving countermeasures;
- **F** Facilitate access to screening, brief interventions and treatment;
- **E** Enforce bans or comprehensive restrictions on alcohol advertising, sponsorship and promotion;
- **R** Raise alcohol prices through excise taxes and pricing policies.

It is also a word. In English, safer is what these measures make people: safer on the roads, safer at home and in the community, and safer from disease and injury. The promise is built into the name, because the goal and the word are the same.

Carrying the name into other languages

This double meaning is part of SAFER's strength in English, and it is the one element that translation cannot easily preserve. In other languages the five interventions translate well, but their initials no longer spell the word, and the five letters on their own carry no meaning. The aim is therefore not to translate the name but to re-create its effect: transcreation rather than translation. One principle guides this work: keep the anchor, build the bridge.

Keep the anchor. SAFER stays unchanged as a single global wordmark in every language and script, in the way a logo does, so that each national or community effort is recognized as part of one worldwide initiative to reduce alcohol harm, led by WHO with its international partners. The SAFER visual identifier is used in line with WHO branding guidance.

Build the bridge. Beside the anchor, a local layer restores the sense of being safer. Depending on the language and the resources available, it can take one of three forms:

1. a short local tagline that conveys the promise of being safer;
2. the five-letter structure kept as a memory aid, with each letter introducing the local name of an intervention where the language allows; or
3. a locally created word or phrase that is itself a promise of safety, with SAFER retained as the technical sub-brand.

Non-Latin scripts

Where a language uses a non-Latin script, the five-letter mnemonic cannot carry across and the letters cannot be read as a word. SAFER then works best as a Latin-script logo, a visual anchor, transliterated into the local script where that helps it to be spoken and recognized, with the meaning carried by the local tagline or name.

Whichever form is chosen

Keep the five interventions clearly distinguished and presented in a consistent sequence, so that the structure survives even where the acronym cannot. Use the agreed local rendering consistently across all materials and record it in the national or community communication plan so that all partners use the same form. Develop and test every adaptation with native speakers and with language services before it is used.

For more information contact:

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